

Title

Author

MINDFULNESS-BASED SOBRIETY

A Clinician's Treatment Guide *for* Addiction Recovery Using
RELAPSE PREVENTION THERAPY,
ACCEPTANCE & COMMITMENT THERAPY
& MOTIVATIONAL INTERVIEWING

Includes downloadable client worksheets & group facilitation materials

NICK TURNER, MSW
PHIL WELCHES, PHD
SANDRA CONTI, MS

COMPREHENSIVE CLINICAL CURRICULA FOR OPEN GROUP THERAPY IN OUTPATIENT & RESIDENTIAL PROGRAMS

“In this book, Turner, Welches, and Conti have made a much-needed addition to the substance abuse treatment field. Integrating several of the most important new approaches to helping people change, the authors have written a concise, lucid, and practical book about their evidence-based approach. *Mindfulness-Based Sobriety* details an approach that is not only effective, but one that is profoundly respectful and compassionate toward those who suffer from addiction.”

—**Michael Maslar, PsyD**, director of mindfulness and behavior therapies at The Family Institute at Northwestern University

“The authors have done a thoughtful job of integrating motivational interviewing, acceptance and commitment therapy, and relapse prevention into a well-designed curriculum for outpatient and residential groups. … This is what can happen when knowledge and compassion come together.”

—**Stan McCracken, PhD**, senior lecturer at the University of Chicago, School of Social Service Administration

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Summary: "Mindfulness-Based Sobriety presents a breakthrough, integrative approach to addiction recovery for clinicians who treat clients recovering from substance abuse and addiction. The book combines relapse prevention therapy, acceptance and commitment therapy (ACT), and motivational interviewing to help clients conquer substance abuse by identifying their own values, strengthening their motivation, and tackling other mental health problems that may lie at the root of their addiction. The book also puts a strong emphasis on relapse prevention, so that clinicians can help clients stay on the path to sobriety"-- Provided by publisher.

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We wish to express our thanks to the many Gateway Foundation clients and clinicians whose enthusiastic participation and valuable feedback helped to refine the MBS model. Further acknowledgments go to the Gateway administrative and support staff, who embraced and supported the initiative.

Introduction

This clinical manual and curricula began as part of a clinical improvement initiative by the Gateway Foundation, which sought to better fulfill the mission of reducing substance abuse and co-occurring mental health problems. A range of research-supported practices was reviewed, with particular attention paid to the following factors:

- Engaging clients in therapy
- Eliciting and clarifying pro-health values and goals
- Enhancing motivation toward value-based lifestyle change
- Addressing, through strategic planning and skill development, the multiple factors that may assist a person in achieving and maintaining sobriety

The product of our efforts was the mindfulness-based sobriety (MBS) model, which integrates three evidence-based practices: acceptance and commitment therapy (ACT), motivational interviewing (MI), and relapse prevention therapy (RPT). The MBS model is a collaborative approach to open-group therapy that is intended to help individuals with substance use problems achieve sobriety through enhancing awareness, accepting experience, and clarifying values. To the extent that individuals are aware of their situations and to the extent that their goals and actions are grounded in personal values, they are less likely to engage in self-defeating behaviors—that is, behaviors that diminish their sense of well-being, self-worth, and personal meaning. In MBS, sobriety is approached in the service of value-based living.

Pilot Project

The outpatient curriculum presented in this manual was piloted for eighteen months in a Gateway adult intensive outpatient program (IOP) in Aurora, Illinois. The residential model was developed and piloted separately, as a result of the success achieved with the IOP curriculum. Throughout the pilot period, session formats, content, information sheets, and handouts were refined based on client feedback and clinician experience.

During the final year of the outpatient pilot project (April 1, 2011, through March 31, 2012), significant improvements were noted in client satisfaction, engagement, retention, and successful completion of treatment (compared to the previous year).

Data include:

- 473 clients received services in the pilot program.
- 300 of the 473 clients (63.4 percent) completed treatment successfully using statewide norms; this can be compared to a statewide IOP successful completion rate average of 49.2 percent (8,671 clients) for fiscal year 2011 (the most recent full year for which statewide data was available at the time).

Using the national standard Press Ganey Behavioral Health Satisfaction Survey, during the final quarter of the pilot (January 1, 2012, through March 31, 2012), this program, compared with 76 centers across the country, scored as follows:

- Overall Assessment: 98.6 percent (99th percentile)
- Likelihood of Recommending: 98.8 percent (95th to 99th percentile)
- If (Needed to and) Had Choice, Would Return: 100 percent (99th percentile)
- Overall Rating of Care: 98.8 percent (99th percentile)
- Felt Ready to Be Discharged: 95.0 percent (95th to 99th percentile)

- Information Regarding Your Treatment: 98.5 percent (99th percentile)
- Included in Treatment Decisions: 96.4 percent (90th to 95th percentile)

Evidence-Based Practices

The approach espoused in this manual is consistent with current research on addictions and mental health treatment. As noted previously, it represents an integration of ACT, MI, and RPT, all of which are listed in the National Registry of Evidence-Based Programs and Practices (NREPP). Additionally, the model presented in this manual promotes client-therapist collaboration, emphasizing empathy and therapeutic alliance, referenced in NREPP under "Evidence-Based Therapy Relationships." Finally, there is a plethora of research supporting the effectiveness of mindfulness in ameliorating both addiction and mental health problems. Mindfulness is central to the MBS approach and is incorporated in all treatment sessions. For those interested in reviewing research on therapeutic components contained in MBS, please see appendix H, "Research Support."

Organization of the Book

This book is organized into two parts. Part 1 provides the underpinnings of the therapy model and approach, while part 2 provides curricula with session outlines, descriptions, facilitator guides, and client handouts for intensive outpatient and residential levels of care.

Part 1, "Therapeutic Foundation and Approach," includes three chapters. In chapter 1, the therapeutic model is presented, including the value and characteristics of the therapeutic relationship and the overarching theoretical perspective and therapeutic approach. An overview of contributing models is provided: ACT, MI, and RPT. This is followed by a discussion of MBS modifications of traditional RPT. Chapter 1 ends with a review of complementary treatment models that can be used in conjunction with the MBS curricula.

Chapter 2 contextualizes MBS in a continuum of care and references this with American Society of Addiction Medicine (ASAM) Patient Placement Criteria. In chapter 3, general therapeutic principles and practical facilitation issues are the focus. These include understanding characteristics of the open-group format, orienting new members to the group, addressing stages-of-change issues, prioritizing group tasks, processing in group therapy, personalizing the approach, and learning awareness areas and tips for the practitioner.

Part 2, "Mindfulness-Based Sobriety Curricula," consists of two chapters. Chapter 4 presents the mindfulness-based sobriety intensive outpatient (IOP) curriculum, while chapter 5 presents the residential curriculum. Each of these chapters includes level-of-care (outpatient and residential) treatment tasks, a general outline of a three-hour session model, and an overview of rotating session topics. This is followed by session outlines, session descriptions (with instructions), and all facilitation materials and client handouts to be used in the sessions.

Finally, this book includes eight appendices. Appendices A, B, and C provide facilitator instructions for key components of the MBS model that are used in multiple sessions:

Appendix A, "Mindfulness and Urge Surfing," provides descriptions and instructions for the practice of mindfulness, including mindfulness to help cope with substance cravings and urges ("urge surfing").

Appendix B, "Overview of the MBS Model and Principles," provides a general description of the MBS model that is briefly reviewed in each group therapy session.

Appendix C, "Value-Based Living Presentation," provides a description of and instructions for the value-based living presentation, which is included in multiple sessions.

Appendix D, "High-Risk Events and Scenarios," is a client handout that provides a list of experiences and situations to consider when assessing relapse risk and developing strategies to achieve and maintain sobriety. This handout is used in both outpatient and residential sessions.

The remaining appendices offer additional information for areas that some readers may wish to further explore:

Appendix E, "Glossary of Terms," provides definitions of key terms used in this book.

Appendix F, "Generations of Cognitive Behavioral Therapy," describes differences among behavioral

therapy, second-generation cognitive behavioral therapy, and contemporary third-generation cognitive behavioral (contextual behavioral) therapy.

Appendix G, "Recommended Readings," is a list of books providing introductions and overviews of the key elements in MBS: acceptance and commitment therapy, motivational interviewing, relapse prevention therapy, empathy and therapeutic alliance, and mindfulness.

Appendix H, "Research Support," is a list of articles providing research support of the key elements in MBS: acceptance and commitment therapy and other contextual behavioral approaches, motivational interviewing, relapse prevention therapy, empathy and therapeutic alliance, and mindfulness.

Use of Pronouns and References to Those in Treatment

In writing this book, the authors attempted to distribute use of feminine and masculine pronouns in a relatively equal manner. The chapter on intensive outpatient treatment uses feminine pronouns nearly exclusively when referring to those in treatment and when referring to clinicians. In a similar way, the chapter on residential treatment uses male pronouns. In earlier chapters, pronouns referencing both genders are used. In regard to those in treatment, the terms "individuals," "persons," "group members," "participants," and "clients" are used interchangeably.

Suggested Approach to Reading This Book

While it may be tempting for some readers to skip over part 1 and go directly to part 2 to read the chapters that specifically address the outpatient or residential level of care in which they work, we recommend that you read all of part 1. This will provide theoretical and therapeutic foundations that serve to guide practice in the MBS model, as well as tools with which to work flexibly in managing novel situations that may occur.

We hope that you and your clients find value in this book and these curricula, and we appreciate any feedback that you may have.

PART 1

THERAPEUTIC FOUNDATION AND APPROACH

Chapter 1

Therapeutic Model

In my early professional years, I was asking the question, "How can I treat or cure or change this person?" Now I would phrase the question in this way: "How can I provide a relationship which this person may use for his own personal growth?"

—Carl Rogers

Therapy occurs in a context that intentionally and explicitly promotes change. The individual (person in context) is understood as someone who will undergo change, and the context is understood as the therapeutic framework within which change is facilitated. In mindfulness-based sobriety (MBS), relational aspects of the person's context include the therapist and therapy group members. The client is in relationships with both the therapist and the therapy group.

Therapeutic Alliance

The decision to enter into a change-promoting relationship is a mutual agreement between the individual and the therapist. It may be preceded by:

- The person presenting on his life situation and possibly on desired change
- The therapist presenting on the therapeutic process
- Collaborative discussion on whether or not a working relationship can be established, negotiation regarding parameters of the relationship and process, and general agreement on how to proceed

Note: In MBS, the therapy agreement between the individual and therapist entails mutual participation in a therapeutic process. More frequently than not, individuals who enter therapy are undecided or ambivalent about change. Instead of requiring commitment to change as a precondition for entering treatment, the MBS therapeutic repertoire includes exploration, elicitation, and clarification of individuals' values, as well as related desires, abilities, reasons, and needs to change. This person-centered, motivational interviewing approach can help to resolve ambivalence and achieve commitment to change.

The individual's experience of being in therapy is central to therapeutic engagement and outcomes. In MBS, the individual's self-agency is respected. It is recognized that real and lasting therapeutic change can occur only through decisions and commitments coming from the individual; therapeutic change cannot be externally imposed. Accordingly, attempts to coerce or persuade are not employed. "Therapy" approaches that attempt to direct or control outcomes—for example, using criticism, confrontation, and judgments—tend to create a struggle for control of the client's life direction, his sense of self-agency. Such battles preclude an effective therapeutic alliance and can deadlock the therapy process. From the point of initial contact through development and continuation of the therapeutic alliance, respect for client self-agency, or autonomy, is understood.

Therapeutic Frame

In MBS, individuals learn to enhance awareness, accept their experiences and situations, clarify values, and live in value-informed ways. Awareness refers to a nondefensive openness to experience. In MBS, awareness is enhanced in two ways: mindfulness practice and self-reflection. In mindfulness, one is aware of the present—one's range of experience, including sensations, emotions, and thoughts—while not attempting to control the experience itself. The person practices just observing: neither "clinging to" nor avoiding aspects of experience. Self-reflection is a topic-focused process of self-exploration that includes personal values; taking account of one's situation; reviewing one's thoughts, feelings, and actions; and exploring options and priorities. Individuals are encouraged to accept, rather than struggle with, their experiences and situations while grounding their motivation and life direction in personal

values.

Helping individuals enhance awareness of personal values is essential in the MBS therapeutic process. When we discuss values, it is important to clarify that MBS does not adhere to a "moral model" that regards people who are in need of help as having "moral deficits" and needing to adapt to and comply with externally recommended sets of values. Instead, the values that are essential to therapeutic change are seen as intrinsic (within the person) and accessible through self-reflection. Accordingly, the therapeutic process includes awareness enhancement. Value awareness assists in goal setting and life planning, and it helps to motivate and guide one's actions in meaningful ways. Thus, MBS helps the person shift from actions that are motivated primarily by feelings (including emotions, physical discomfort, and impulses) to a life course that is motivated by values: what is fundamentally important and meaningful. Often, the "decision point" involves a choice between immediate relief and a deeper and more sustaining sense of life meaning and satisfaction.

In the case of addiction recovery—and in the case of a wide range of mental and emotional disturbances—nothing changes unless the person becomes conscious of and lets go of behaviors that are in the service of avoiding awareness. In the case of addiction, substance use and the related lifestyle may constitute avoidance of unpleasant life circumstances. Avoidance encourages more avoidance. Why? Because it works, at least temporarily, and we are "wired" in large part for temporary results. To get the "bigger picture," or longer-range perspective, we must be with the immediate experience (however unpleasant) without reacting, and we must develop and maintain a perspective on what's really important and what we need to do to make it happen.

Commitment to value-informed living, versus avoidance, has a prerequisite: awareness of, acceptance of, and openness to one's situation. Urges to impede awareness must be recognized, experienced, and accepted rather than acted on. This includes efforts to impede sobriety. Practicing awareness (sobriety) may be seen as being in the service of value-based living.

Contributing Models

MBS incorporates contributions from acceptance and commitment therapy (ACT), motivational interviewing (MI), and relapse prevention therapy (RPT) toward reducing substance use and mental health problems. All three models raise awareness, clarify values, enhance motivation, and elicit commitment for healthy lifestyle changes. MBS does not so much require a shift from one model to another; rather it is a single, integrated process that is informed by all three therapies.

Having become integral in both ACT and RPT, mindfulness is receiving growing recognition for its efficacy in addiction and mental health therapy. It is a practice and process of being in the present. One simply observes—or is aware of—what is going on in the here and now. Mindfulness helps to enhance awareness, improve focus, and develop more grounded and encompassing life perspectives. In addiction treatment, it can help the person to face adverse life experiences, including high-risk events, cravings, and urges, without relapsing.

Acceptance and Commitment Therapy

Acceptance and commitment therapy, or ACT (pronounced "act"), is part of the third generation of cognitive behavioral therapy (CBT) and is a participant in the Association of Contextual Behavioral Science. The name "acceptance and commitment therapy" is descriptive of the approach.

From the client's perspective, the ACT therapeutic process is twofold: accepting one's experience and situation while committing to value-informed living. Unlike the second generation of CBT, ACT does not require eliminating or replacing "problematic" thoughts, as such efforts may only draw additional attention to the unwanted thoughts. Instead of attempting to "fix" what is believed to be "broken," ACT encourages individuals to accept that which is present, including thoughts and feelings, while committing to value-informed action. In order to further explain ACT as a therapeutic approach, we will highlight the interrelated core processes below and how they have been interwoven into the MBS approach.

HUMAN SUFFERING: PSYCHOLOGICAL RIGIDITY

From an ACT perspective, much of human suffering is caused by what is referred to as "psychological rigidity." This occurs when one becomes entangled with his thinking and avoids experiences in ways that compromise value-informed living. The aforementioned behavioral pattern is believed to, at one time, have

increased our chances of survival as a species (Wilson, 2008). For early humans, most, if not all, of their time was dedicated to survival needs, such as finding food, shelter, and water and developing means for protection. The "fight, flight, or freeze" instinct is believed to have developed out of our need to take quick action in service of survival. For example, if we encountered a lion and stopped to think about how to respond, we would quickly become food for the lion. However, if we felt frightened by the lion we encountered and instinctually killed it, or ran away from it, or played dead (fight, flight, or freeze, respectively), we were able to survive.

As humans developed, our lives became more complex. Our fulfillment in life is now based less on survival (we now have grocery stores, fast-food restaurants, and varying degrees of shelter and transportation) and more on values and goals (what we want our lives to be about). While eradicating unwanted emotions by effectively addressing threats (fight, flight, or freeze) increased chances of survival, using the same approach in the context of values may interfere with a purposeful existence. In other words, our "survival" instincts are now seemingly engaged or provoked by emotions related to valued/non–life-threatening situations. Consequently, a person might instinctively avoid experiences that could potentially benefit him in the long term. Life now requires us to feel and move toward (not away from, as in the case of the lion) certain difficult experiences. Examples include applying for jobs (while risking rejection), asking someone on a date (while risking embarrassment and rejection), expressing one's true feelings (while risking being vulnerable), and experiencing an alcohol or drug craving (without acting on it). The following case example will be used throughout this section as a means of illustrating and explaining the principles and model.

Dan is a forty-two-year-old male who is entering outpatient treatment after completing residential services due to his ongoing struggle with alcoholism and depression. In recent years he has been experiencing intense feelings of sadness and loneliness related to being unmarried and lacking satisfying relationships in his personal life. During his intake appointment, he made statements such as "It's impossible to find friends," "I am unlikable," "I can't trust anyone," and "Nothing will ever change." When asked how he spent his free time, Dan commented, "I go to work, and I come home and drink. I don't have any friends; what's the point of leaving the house?"

In conceptualizing Dan's case from an ACT perspective, he seems to be attached to the idea of himself as an "unlikable" person, which is often the result of someone getting caught up in the content of his thinking. Additionally, in attaching to the content of his thoughts, Dan has developed a hopeless perspective on the future, which does not leave room for change or growth. Due to being an "unlikable" person, Dan has avoided situations in which he would have a chance to form new relationships and overcome his current position in life. Such actions include isolating in his house and drinking to numb the emotional pain.

Additionally, over time Dan has developed avoidant behavior patterns, which provide temporary relief but have seemingly increased his suffering in the long term. The six core processes of psychological rigidity will now be explored in detail. These core processes are experiential avoidance, inflexible attention, attachment to a conceptualized self, cognitive fusion, disruption of chosen values, and inaction or impulsivity (Hayes, Strosahl, & Wilson, 2012).

Experiential avoidance. Whether conscious or not, we tend to avoid things that we anticipate will involve uncomfortable or unwanted internal experiences (feelings, thoughts, emotions, sensations, and so on). This process is referred to as *experiential avoidance*.

In survival contexts, emotions that signal threats can be eliminated by successfully eliminating the threats (such as through fight, flight, or freeze). However, a similar "eliminate the unwanted emotion" approach can be counterproductive in non–survival-based contexts, such as purposeful living. In fact, purposeful, value-based living typically requires that one experience a degree of emotional discomfort while attempting to accomplish goals.

Returning to the above case example, Dan eventually revealed to his counselor that he had experienced a series of personal failures when he was in his early thirties. This included his wife leaving him for his best friend. His drinking increased at that time, which contributed to problems in employment, health, and friendships. He spoke extensively about "not being able to trust anyone" and "being better off alone anyway." Without risking the experience of feeling hurt and vulnerable (experiential avoidance), Dan has spent years in isolation, which seemingly has hindered him from establishing and maintaining healthy relationships.

The opposite of experiential avoidance is acceptance (or experiential presence), which will be discussed later in this section.

Inflexible attention. Inflexible attention can be understood as the instinctual tendency of the human mind to focus on the past or the anticipated future, while ignoring or devaluing what is happening in the present moment.

In Dan's case, he appears to be overly attached to the past ("I can't trust anyone" and "Things will never

change"), which prevents him from moving forward and making changes. In fusing with his mind's insatiable need to worry, Dan has unintentionally increased his suffering and hindered himself from achieving a value-based future.

The opposite of inflexible attention is flexible attention to the present moment, or mindfulness, which will be discussed later in this section.

Attachment to a conceptualized self. When someone becomes overly attached to a conceptualized sense of self, this can hinder him from being the person he actually wants to be. For example, Dan seems to be attached to the idea that he is an "unlikable person." Other examples include when individuals are overly attached to diagnoses; for example, "I'm bipolar" or "I'm an addict." If a person truly is that label or concept, how much room does he have to behave in a flexible and value-based manner? Additional examples of indicators of when someone is attached to a conceptualized self include phrases such as "That's just the way I am" and "I am not the type of person who does something like that."

The opposite of attachment to a conceptualized self is self as context, which will be reviewed later in this section.

Cognitive fusion. *Cognitive fusion*, or simply "fusion," can be defined as the state of being overly attached to and controlled by internal experiences. Again, from a mindfulness and ACT perspective, what we think and how we feel are not the problem; it is how we respond to our thoughts and feelings that can lead to difficulties. When we are overly attached to, or fused with, internal experiences, thoughts feel like truths, and emotions feel as if they will never go away. Our actions are then based on how we feel and what we are thinking, rather than on how we want to be as people. This can further distance us from the lives we want to live. In Dan's case, he is overly attached to, or fused with, the self-concept and belief "I am unlikable." Additionally, when he is fused with his thoughts about the past and future (*I can't trust anyone* and *Things will never change*) and his emotions (depression, loneliness, and anxiety), he behaves in a way that is not conducive to achieving his larger life goals—that is, he drinks and isolates himself.

The opposite of fusion is defusion, which will be reviewed later in this section.

Disruption of chosen values. In order to remain in contact with our values, we must risk experiencing uncomfortable internal experiences. In the case example, Dan would need to risk rejection, vulnerability, and failure in order to eventually establish relationships that are fulfilling and meaningful. Again, if he is caught up in the content of his thinking, Dan continues to base his actions on his emotions (depressed, anxious, lonely, and so on), which has caused him to behave in ways that have disrupted his ability to contact his values and goals. Although such behaviors as drinking or avoiding social gatherings have provided him with temporary relief, they have taken him further away from what he truly wants in life. At this point, Dan's values are limited to not feeling, or numbing out. However, when we look below the surface, Dan seems to truly value relationships and companionship, which he has avoided for many years.

The opposite of disruption of chosen values is chosen values, which will be discussed later in this section.

Inaction or impulsivity. Lastly, behaviors related to inaction or impulsivity can result in short-term relief or gratification but can leave us feeling empty in the long term. As highlighted above, Dan's actions, or lack thereof, have only provided him with short-term results (temporary relief from uncomfortable internal experiences) and have done very little to increase his quality of life. As time goes on, enough impulsivity and inaction can limit our ability to take effective steps in a valued direction toward our goals.

The opposite of inaction or impulsivity is committed action, which will be discussed later in this section.

HEALING AND GROWTH: PSYCHOLOGICAL FLEXIBILITY

The main goal in ACT is to help individuals develop a sense of openness and psychological flexibility. As defined by Jason Luoma, Steven Hayes, and Robyn Walser (2007), psychological flexibility is the ability to contact the present moment more fully as a conscious human being and, based on what the situation affords, to change or persist in behavior in order to serve valued ends. As mentioned above, psychological rigidity (the opposite of psychological flexibility) can lead a person to suffer more and engage less in life. Through psychological flexibility, one can gain a sense of choice and purpose in decision making and obtain an ongoing sense of fulfillment (regardless of how he thinks and feels at any given time).

In ACT and MBS, psychological flexibility is cultivated and developed through a variety of methods, including the use of metaphors, mindfulness meditation, and other experiential exercises. We will now explore

Dan's case further through the six core processes of psychological flexibility. These processes include acceptance, flexible attention to the present moment, self as context, defusion, chosen values, and committed action (Hayes et al., 2012).

Acceptance. In ACT, individuals are encouraged to work toward accepting and opening up to what life has to offer: the comfortable, uncomfortable, and everything in between. By mindfully observing and "making room for" one's life experiences and related internal responses, one can avoid engaging in an ongoing and unproductive struggle to control or eliminate them. In this sense, the person approaches his internal and external experiences in life with a genuine sense of curiosity and compassion. In Dan's case, through developing a sense of acceptance, he may be able to improve his quality of life by abandoning his internal struggles.

The "Quicksand metaphor" is useful in illustrating acceptance from an ACT perspective (Luoma et al., 2007). If one were to find himself sinking in a patch of quicksand, he would intuitively begin to squirm and struggle in an attempt to escape and avoid drowning. Unfortunately, in the case of quicksand, our instinctive actions cause us to sink faster, actually increasing our chances of drowning. In order to escape the quicksand, one must spread out and let as much of his body contact the sand as possible, which allows the body to float on the surface. A person can use a similar approach to life and internal experiences. One can spread out and relax into his experiences (internal and external), rather than struggle, which might eventually result in "drowning."

Examples of acceptance-based exercises within the MBS curricula include mindfulness meditation exercises, defusing from the addiction, and "dirty" versus "clean" pain. Additional acceptance exercises and interventions can be found in other seminal ACT texts (Hayes et al., 2012).

Flexible attention to the present moment: Mindfulness. Like a child who continuously wanders away from a parent in a crowded place only to be repeatedly snatched up and brought back into custody, our minds tend to wander to the past and anticipated future. If allowed to roam freely, the mind, like the wandering child, can become lost and overwhelmed, finding itself far from home (the present moment) and missing a sense of safety and peace. Although thoughts of the past and future are not inherently "dangerous" or "bad," people can become so fixated or fused with them that they lose their ability to focus on and act in the here and now. Another way of looking at it is that through becoming present and aware, one can actively participate in life, as opposed to ruminating on the sidelines.

In Dan's case, he tends to fuse with the past and ruminate on the inevitable infidelity and betrayals that will come about through "letting people in." Prior to social gatherings, he often finds himself thinking about his ex-wife and former best friend, and he then looks for any excuse to avoid leaving his house. This usually results in his drinking in isolation. In situations where Dan finds himself becoming fused with the past and the "hopeless" future, he could contact the present moment by expanding his awareness to his breath, his feet on the floor, and any other sensations available to him. Such practices could help to loosen the mind's grip and create the space for him to act in a valued manner (go out with friends, avoid drinking, establish new relationships, and so on). For further explanation of mindfulness skills and practice, see appendix A of this book.

Self as context. Self as context highlights the idea that we are not our internal experiences. Although we may "contain" things such as emotions, thoughts, memories, and sensations, they do not have to define and make up who we are and how we act as people. A useful metaphor to explain self as context is the "Chessboard metaphor" (Luoma et al., 2007). Much of the time, people spend their lives engaged in a metaphorical chess game. They attempt to attach to the "good" pieces (pleasant and rewarding experiences) and to defeat the "bad" pieces (unwanted internal and external experiences) in service of living a happy and fulfilling life. Unfortunately the key word in the previous sentence is "attempt," due to the fact that the game is rigged and largely unwinnable.

In our attempts to defeat the "bad" pieces, we often blindly create more suffering for ourselves, which can lead us to fight harder only to suffer more, with the end result being defeat. An example would be when Dan avoids social situations and drinks in isolation. Although he temporarily wins the battle (numbs his feelings and avoids facing potential rejection), he ultimately loses the war (he feels worse the next day and is even more depressed and lonely over time). If one can simply "be the chessboard," he does not need to engage in the battle and is free to live his life, regardless of what he is thinking or feeling. Again, self as context, or the "observing self," can be developed through various exercises and activities, including mindfulness meditation. Self as context is discussed further in other seminal ACT texts (Luoma et al., 2007).

Defusion. As discussed earlier, the opposite of fusion (becoming overly attached to internal experiences) is defusion. If one is fused with the content of his thinking, he tends to believe thoughts to be truths, facts, and

commands to follow. In early forms of CBT, individuals were assisted in challenging or restructuring thinking patterns or "errors" in service of developing healthier or positive ways of thinking (and feeling). ACT differs from those earlier forms of CBT in that it encourages individuals to respond to their internal experiences in an open and flexible manner. Instead of challenging or replacing the thoughts and feelings, a person can simply acknowledge and observe them from a defused perspective. Specific defusion skills are included in the MBS curricula (see chapter 4, session 3, "Defusing from the Addiction"). Additional defusion exercises can be found in other seminal ACT texts (Luoma et al., 2007).

As mentioned above, Dan often fuses with the content of his mind, including the belief that he is "unlikable." He also tends to fuse with his emotions (loneliness, depression, anxiety), which results in his drinking to numb the emotional pain. Dan could benefit from utilizing defusion skills such as "naming the addiction," which are included in the MBS curricula. In this exercise, Dan would name his mind or addiction in service of awareness and defusion. In moments where his mind feeds him thoughts such as *I'm unlikable*, Dan could respond simply by saying *Hello, mind, thanks for your input*. He could also learn to interact with his thoughts in a different way by simply saying *I am having the thought that I am unlikable* instead of *I'm unlikeable*. In taking this approach, Dan is able to defuse from the content of his thinking and choose to act in a way that is in service of his values and goals (rather than getting caught up in the struggle of arguing with or changing the content of his current thinking). The subtle difference in approach can create space for change and growth.

Chosen values. ACT provides a focus on values: what's especially important to the person in a greater life perspective. Values may inform one's actions and provide guidance. Constructing value-based or value-informed goals can provide concrete and temporal structure, which carries the potential to interrupt behaviors such as substance use. Values and goals are addressed and developed throughout both IOP and residential MBS curricula.

In Dan's case, values and goals work would be very important. Through receiving help with identifying his personal values (companionship, honesty, growth, respect, loyalty) and goals (building a support network and finding a significant other), Dan may be able to gain a deeper sense of fulfillment in life. In turn, he may also experience a decrease in substance abuse due to his having more to lose and a lot more to gain in life and relationships.

Committed action. Lastly, committed action involves a person's ability to act in a value-consistent manner, regardless of what he is thinking or how he is feeling. An example with Dan would be attending a social gathering—consistent with his deeper values (companionship, support, and sobriety)—even though he may feel anxious and afraid. The main goal in committed action is to achieve value-related fulfillment through expanding one's behavioral repertoire.

ACT IN MBS

We believe that ACT has influenced much of CBT toward transcending its second-generation model and potentially embracing an integrated or transtheoretical one. The fundamental underpinnings of ACT are similar to some humanistic, existential, and Eastern approaches. In Medard Boss's *daseinsanalysis*, resolution of psychological impasses is made possible through openness and acceptance: "Man's freedom consists in becoming ready for accepting and letting be all that is" (Boss, 1982/1963, pp. 47–48). In Morita therapy (Morita, 1998/1928), based on Zen psychological principles, one is encouraged to live in pursuit of what is personally meaningful and, in the process, to be aware of the automatic nature of thoughts and feelings. What these approaches have in common with ACT is this principle: be mindful of what presents in our experience—thoughts, feelings, and sensations—but do not attach to them or allow them to be the "driver." Instead, one's actions are informed by personal values.

ACT is used in the MBS curricula to promote acceptance through mindfulness, values clarification, motivation enhancement, and the setting of value-based goals.

Professional training in ACT is encouraged. Recommended ACT readings are listed in appendix G of this book. For additional information on ACT and training resources, visit the website of the Association for Contextual Behavioral Science: www.contextualscience.org.

Motivational Interviewing

When we are making changes in life, many of us feel ambivalent—that is, we feel two ways about it. Some of us spend years in a state of indecision before choosing a direction to take. Furthermore, if we are pressured to

change by a loved one or a legal party, for example, we tend to take the "stay the same" side of the ambivalence. Put metaphorically, humans tend to pick up the rope and engage in a tug-of-war with whatever or whoever is trying to make them change. Both sides tug at the rope in hopes of winning the battle, while providing each other with reasons to change and not change.

As practitioners, it is our job to avoid picking up the rope and engaging in the tug-of-war with the people we are trying to help. This is where motivational interviewing (MI) comes into play (Miller & Rollnick, 2013, p. 29):

> *Motivational interviewing is a collaborative, goal-oriented style of communication with particular attention to the language of change. It is designed to strengthen personal motivation for commitment to a specific goal by eliciting and exploring the person's own reasons for change within an atmosphere of acceptance and compassion.*

MI evolved from the humanistic-existential tradition, particularly from Carl Rogers's person-centered therapy. MI is a way of being with the client. The MI approach engages clients wherever they are on the readiness-to-change continuum and helps them process their ambivalence regarding behavioral health or lifestyle change.

SPIRIT

The spirit of MI—partnership, acceptance, compassion, and evocation—is prioritized over technique (Miller & Rollnick, 2013). In MBS, the practitioner is encouraged to embody the spirit of MI both inside and outside of interactions with clients, including when conceptualizing and speaking about client cases with other clinical staff. Through embodying the spirit of MI, the practitioner will be able to maintain and enhance the therapeutic alliance and create an environment where a person can make her own decisions about change. The following case example will be used throughout this section, as a means of illustrating and explaining the principles and model.

Amy is a twenty-four-year-old single mother. She was referred to residential treatment by child protective services after repeatedly testing positive for cocaine. Amy is raising her two children by herself, because the father of her children was abusive and involved in illegal activities. She has a history of experiencing anxiety and depressive symptoms from an early age. During her intake, Amy made statements such as "I don't need to be here; I'm doing this for legal reasons and for my kids," "Cocaine makes me feel better after a stressful day at work," and "I haven't found anything that works better to give me energy for the things I need to do."

There are four vital aspects of spirit, and they are explained below.

Partnership. The first aspect in the spirit of MI is partnership. MI is collaboration between the practitioner and the person in front of her. Though she may possess wisdom and great advice, the practitioner is not the expert on the client's life. The client is the expert on herself. Clinical attempts by an "expert" to coerce, convince, or confront the client into changing are unlikely to succeed and risk the client's reacting in defense of her self-agency. The MI and MBS practitioner sees herself as a companion for the client on a journey of decision making. A useful metaphor may be comparing an MI session to "dancing" rather than "wrestling" (Miller & Rollnick, 2013). MI is a fluid process in which the practitioner does not attempt to overpower or defeat an opponent.

In Amy's case, she does not appear to be committed to discontinuing her use of cocaine. In her first group, Amy made her feelings very clear during the check-in by stating, "I am forced to be here, and I am missing work, which is going to cost me in rent and grocery money." The non-MI practitioner could easily get caught up in providing Amy with reasons why she needed to be in treatment, potentially spending twenty minutes or more of group engaged in a tug-of-war.

Instead, the person facilitating the group adopted the spirit of MI and partnered with Amy by replying, "I'm sorry to hear that you are missing work to be here. I would be frustrated too, especially if I were a parent and needed money for food and rent. What do you need from us in order to help you feel supported?" In this example, the group facilitator did not engage in a power struggle, was empathetic, and fostered a sense of partnership between Amy and the group.

Acceptance. The second aspect of MI spirit, acceptance, features four specific qualities: absolute worth, accurate empathy, autonomy support, and affirmation. The MI or MBS practitioner creates a space for the group members to feel accepted and comfortable enough to explore and potentially make life-enhancing changes.

Absolute worth involves the practitioner's acknowledging and respecting the inherent worth or ability within someone. Although many of the people we work with present with challenges or in a "compromised" state, it does

not mean they do not have the ability to heal, grow, and change. If we are partnering with clients on a journey toward change and take on a skeptical or pessimistic mind-set in regard to their ability to change, they are less likely to persevere and make it to their destination. However, if the practitioner remains a supportive and hopeful partner, the client may be more likely to continue moving forward on the sometimes-uncomfortable journey toward change.

The second quality of acceptance is *accurate empathy*. This involves the practitioner's putting forth a genuine effort toward trying to understand the person's experience. This does not mean identification (actually going through what the person has) or sympathy (feeling sorry or sad for the person). The practitioner truly listens to the client with open ears, mind, and heart and reflects the information back to her in a nonjudgmental manner. A common trap practitioners fall into is making the assumption that the person "must develop insight" or a "more accurate view on life." In attaching to judgments and assumptions about clients, we compromise our ability to truly understand them and their experiences in life.

The third quality of acceptance is *autonomy support*. The MI and MBS practitioner approaches clinical work from a standpoint that the person and only the person can make decisions and carry out actions. Accordingly, the practitioner does not try to make the client's decisions or impose solutions. Rather, the practitioner facilitates a collaborative partnership between him- or herself and the client through empathy and use of communication skills, including reflections, open questions, and affirmations of client strengths. The counselor's collaborative approach and related respect for the client's autonomy facilitate a therapeutic alliance in which the client finds little need to defend herself against the practitioner.

It is important to note that in MI, discord in the therapeutic relationship—for example, client talk that favors not changing ("sustain talk")—is not regarded as "resistance" or as a client personality trait. Rather, it occurs in, and is a product of, an interpersonal context. Consequently, the therapist's experience of discord is an indication that that client might be experiencing an infringement on, or invalidation of, her autonomy. This is a "cue" for the therapist to make an adjustment in approach. In MI, the counselor refrains from arguing or confronting. Instead, the counselor respects the client's views and autonomy while helping the client to explore all sides of the situation.

The last quality of acceptance is *affirmation*, which involves the practitioner's seeking and acknowledging the client's inner abilities and strengths. This is an important aspect of the spirit of MI in that it encourages the practitioner to look for what is there (a strength based approach) rather than what is not there or what needs to be there (a problem focused one). It is important to distinguish between being affirming and praising, especially when you are practicing from an MI approach. Through affirming strengths, the clinician is highlighting the client's own inherent abilities and helping her to feel empowered and capable. In contrast, providing someone with appraisals or approval can, at times, seem condescending and, in certain situations, may foster a need for individuals to seek external (as opposed to internal) validation.

Acceptance and its related components are especially important when working with clients like Amy. In a group setting, Amy can be thanked for arriving on time and encouraged to share her own ideas about life and recovery (acceptance, absolute worth, and affirmation). When she questions sobriety and the treatment process, she can be met with acceptance and openness (autonomy support), and her strengths can be affirmed. (She reported during her intake that she was there for her children, which demonstrates her value of being a good parent.) Practitioners should take a nonjudgmental stance by remaining empathetic and validating Amy's struggles.

Compassion. Compassion, the third aspect of MI spirit, involves commitment and behavior on the part of the therapist to promote the welfare of the other person. Compassion, in MI, is not an emotion, but the therapist's consistent actions supporting the best interests of the client. It is through compassion that the clinician can legitimately deserve a client's trust.

Amy came to treatment with an attitude that others regarded as "rude and demanding." She dismissed the idea that treatment might benefit her. The clinical mind-set, or what is referred to in MI as "heart-set," was to assist Amy in exploring her options and to support her in making her own informed decisions. In spite of Amy's presentation, the counselor dedicated herself to supporting Amy's best interests while respecting Amy's autonomy.

Evocation. The fourth aspect of spirit is evocation. In contrast to a problem-focused or deficit approach to treatment, MI encourages practitioners to evoke and bring about that which is already present (the person's own reasons for and ways of approaching the change process). This is done through the use of reflections, open-ended questions, and exploration, as opposed to overreliance on lectures and didactic interventions. Although the MBS model involves some lecture and client education, each session features exercises that are exploratory and evocative in nature. This is in service of drawing out the client's own ideas on life and change. (For an example, see chapter 4, session 7, "Motivation," in the IOP curriculum.) Thus, in MBS, any didactic information presentation should be

done tentatively, encouraging clients to draw from it what makes sense to them.

In Amy's case, the practitioner can help her explore both sides of her ambivalence, including her reasons to stay the same and her reasons to change. Also, during MBS groups, Amy would be encouraged to identify values and goals, as well as lifestyle changes that she would like to make. Through contacting one's values, discrepancies between behavior and values may be brought to awareness, which can bring about reasons for change. An example with Amy would be her identifying her desire to be a loving parent to her children as a value and goal, even though she initially refused to comply with stipulations made by child protective services. In allowing Amy to connect the dots herself and make a decision through exploring her values and goals, the practitioner is likely to be far more effective than if she confronted Amy directly.

MI IN PRACTICE

In individual work, MI begins with a process of engaging the person in a counseling relationship, followed by a focus on reasons the person came in for therapy. A collaborative therapeutic alliance is formed in which the counselor helps the individual to reflect on what is important to her, how she wants her life to unfold, and her current choices and behaviors in lieu of her values and life goals. This is all done with a respectful tone and an appreciation of the client's autonomous role; it is only the client who can decide what she prefers and how she wants to proceed with her life. In MI, no attempts are made to impose solutions or argue in favor of change.

Where there are discrepancies between current behaviors and important goals—that is, where the person's current behaviors do not seem to lead toward her desired state—she may become aware of conflicting motivations. On one side, there is motivation to continue the status quo, and on the other, motivation to change. With increased awareness of this discrepancy comes heightened ambivalence, which can be very uncomfortable. The counselor does not attempt to resolve the ambivalence. The solution comes from the client. In many cases, as the person recognizes that her current behavior, such as substance use, risks interfering with her important goals, she verbalizes desire, ability, reasons, or needs to make changes. In MI, this is called "change talk." Change talk may be followed or accompanied by "commitment talk": client statements that reflect the fact that she will take action. Once the client is committed to change, action planning begins. The counselor's role throughout the MI process is one of facilitation.

MI IN MBS

In this manual, the use of MI provides both philosophical guidance and curricula content. In MBS, MI is integrated into group treatment through a pervasive spirit and through exercises that can improve self-efficacy, evoke and clarify personal values, and enhance motivation to change.

As discussed above, developing discrepancy tends to heighten ambivalence about change. For a person who is less motivated to change, ambivalence is a step forward; it raises change as a consideration. However, for a person who has already decided on change, ambivalence may represent a step backward, a reconsideration of her prior decision to change. Since therapy groups are composed of individuals with varying degrees of motivation, use of MI must be done in ways that do not increase ambivalence for those who are already motivated. MBS group therapy seeks to develop discrepancy in two general ways: through selected group-therapy exercises, and during the brief individual-therapy work that sometimes occurs during group sessions. In regard to individual work, the facilitator, with permission from the individual group member, may point out disparities between the person's desired and current states. For instance, the client may value being a good parent and providing economic security for her children while continuing to use cocaine. Another person's desired state may include earning a college degree, yet she has difficulty getting started and staying focused due to excessive marijuana use. Still another, who values her personal freedom and wants to get probation off her back, remains on supervision due to toxicology reports that are positive for stimulants.

In MBS, prior to providing individual feedback, the facilitator follows the MI process of "ask-provide-ask." That is, the clinician first asks the client if it's okay to provide observational feedback on the client's goals and behaviors. If the client says yes, which is common, the clinician provides the feedback in a tentative manner, by including a qualifier such as "This is just one perspective, and it may or may not be entirely accurate, but I'll throw it out there, and you can see whether or not it fits for you." The clinician can then reflect both sides of the discrepancy: the values and goals on one side, and the current behavior on the other. Following this, the clinician asks the client how this might fit for her, if at all. This process (ask-provide-ask) respects the client's autonomy, avoids the trap of establishing the therapist as the expert on the client's life, and increases the likelihood that the client will respond nondefensively.

With that said, it is also important to not focus too much attention or time on any particular individual in the group. Balance is the key. The facilitator should not compromise the focus and engagement of the larger group.

During group exercises, MBS focuses strongly on values clarification and enhancement. By focusing on values, discrepancy seems to naturally develop for individuals whose current behaviors are inconsistent with their values. For those whose current behaviors are consistent with their values, discrepancy does not typically occur. Instead, the focus on values may serve to strengthen their grounding in their values and reinforce their value-consistent behaviors. Thus, values clarification and enhancement potentially increase motivation and commitment for individuals in all levels of readiness to change without significant adverse risks.

For more on use of MI in MBS, please refer to the chapter 3 section "Addressing Stages-of-Change Issues in an Open-Group Format."

Professional training in MI is encouraged. Recommended MI readings are listed in appendix G of this book. For additional information on MI and training resources, visit the *Motivational Interviewing* website: www.motivationalinterview.org.

Relapse Prevention Therapy

Anyone who has worked for any time in the addiction treatment field can attest to the common occurrence of individuals achieving sobriety and then relapsing. It is a source of frustration for many clinical staff. Moreover, clients typically experience relapse as evidence of personal failure and inadequacy. Unfortunately, such "conclusions" only make matters worse, and it seems that relapse begets relapse.

Noted psychologist Alan Marlatt (1978) conducted qualitative research in which he interviewed alcohol-dependent males who had achieved sobriety in residential treatment and then relapsed after discharge. Findings from this research led Dr. Marlatt to develop a taxonomy of common high-risk situations and a clinical approach to address the comprehensive range of factors that contribute to relapse. This clinical approach was called *relapse prevention therapy* (RPT). Marlatt's RPT purports to be the first cognitive behavioral model of relapse prevention.

Fundamental theoretical assumptions of RPT include:

- Substance-use–related behaviors, like all behaviors (adaptive and maladaptive), are learned.
- For most substance-dependent individuals, quitting alcohol or other drugs is usually not as difficult as maintaining sobriety.
- "A lapse becomes more likely when a client is faced with substance-related cues in a high-risk situation and is beginning to feel unable to cope effectively (low self-efficacy) without resuming the addictive habit" (Marlatt, Parks, & Witkiewitz, 2002, p. 5).

The above quote suggests the clinical importance of understanding and addressing that which progresses toward exposure to high-risk situations, as well as the person's self-efficacy and coping skills. The road toward exposure to high-risk situations often begins with what Marlatt referred to as "lifestyle imbalance" or "global issues." Lifestyle imbalance occurs when a person has achieved substance abstinence but has not made changes in other lifestyle areas, such as diet, exercise, relationships, life interests, and so-called positive addictions. Imbalance may be accompanied or followed by cravings or urges to use and a sense of deserving to use, sometimes referred to in RPT as "I owe myself." Implicit is the expectancy that using would somehow make things better or make one feel better. This is referred to in RPT as "positive outcome expectancy," and it may not be justified. To some extent, the person's cravings and urges may be less than fully conscious. Accompanying these unconscious processes are rationalization, denial, and what Marlatt called "apparently irrelevant decisions" (AIDs). AIDs occur as a chain of decisions that move—sometimes gradually, sometimes as "rapidly escalating scenarios" (Marlatt et al., 2002, p. 14)—toward exposure to high-risk situations.

HIGH-RISK SITUATIONS

In RPT, a high-risk situation is defined as "any experience, emotion, setting, thought, or context that presents an increased risk for a person to engage in some transgressive behavior" (Witkiewitz & Marlatt, 2007, p. 5).

While RPT recognizes individual differences in regard to high-risk situations, Marlatt's research led to a

taxonomy that included common or general categories that warrant individualized assessment. These categories are outlined in the table below:

1. Intrapersonal Determinants
    1. Coping with negative emotional states
        1. Coping with anger or frustration
        2. Coping with other negative emotional states
    2. Coping with negative physical-physiological states
        1. Coping with physical states associated with prior substance use
        2. Coping with other negative physical states
    3. Enhancement of positive emotional states, such as celebrations and other special occasions
    4. Testing personal control
    5. Giving in to temptations or urges
        1. In the presence of substance cues
        2. In the absence of substance cues
2. Interpersonal-Environmental Determinants
    1. Coping with interpersonal conflict
        1. Coping with frustration or anger
        2. Coping with other interpersonal conflict
    2. Social pressure
        1. Direct social pressure
        2. Indirect social pressure
    3. Enhancement of positive emotional states (in primary interpersonal setting)

RELAPSE PREVENTION STRATEGIES

Understanding the “slippery slope” toward high-risk situations provides the clinician with opportunities to intervene.

Per RPT, clinical interventions can help to increase day-to-day lifestyle changes, the so-called global issues, or lifestyle balance. These lifestyle changes may include “positive addictions” and indulgences, such as enjoying a healthy diet, exercise, massage, yoga, meditation, recreation, social relationships, and spiritual endeavors.

High-risk situations for the individual must be assessed. Strategies are developed to avoid risky situations and to escape situations as needed. In RPT, the client and clinician may collaborate in a process of analyzing choices the person might make that could lead to a high-risk situation, drawing so-called relapse road maps. Such analyses and accompanying psychoeducation may enhance insight by increasing the person’s awareness of “tricks” his mind sometimes plays, including rationalization, denial, and “positive outcome expectancies.” In RPT, these tricks are regarded as cognitive distortions, and they may underlie an individual’s chain of AIDs that lead toward high-risk situation exposure. Through proactive relapse analyses, the person may develop the ability to identify and address relapse warning signals.

Skill building occurs to help the person better cope with substance-use cues and the desire to use. Where cues cannot be avoided or are not avoided, imagery may help to manage urges. In applying a mindfulness technique to the experience of substance-related cravings and urges, Marlatt coined the term “urge surfing.” The urge is like a

wave—rising, cresting, and then subsiding—a process that tends to repeat. Being mindful of this experience, observing and not reacting, the person learns that cravings and urges will come and go and that substance use is not necessary to eliminate them. Other RPT skill-building approaches may include self-monitoring of urges and cravings and the person's responses to them, including cue identification and avoidance strategies, drug refusal skills, relaxation training, and stress management.

In RPT, cognitive restructuring may be employed to modify a variety of cognitive distortions. Such distortions might include positive outcome expectancies, for example, when the client expects that using would be a more positive experience than it actually would be. Conversely, when clients have unfavorable beliefs about sobriety (such as associating sobriety with "conformity" or "uncoolness"), cognitive restructuring might help clients adopt alternative cognitive associations. (*Note:* As will be presented later, MBS does not employ cognitive restructuring.)

Lapse management plans may be developed so that in the event of a lapse, the person does not proceed into a full-blown relapse. Lapse management plans may include a contract to limit use and may also include reminder cards with emergency coping instructions. In cases where a lapse has occurred—that is, substance use that does not constitute a full return to the preabstinence use pattern—an individual may experience what, in RPT, is called an *abstinence violation effect* (AVE) (Marlatt & Witkiewitz, 2005). AVE often involves self-blame, shame, and a decrease in self-efficacy. RPT uses cognitive restructuring to define the lapse as a "mistake," not a "failure." It is regarded as an opportunity to solve the problem of what happened, improve the relapse prevention plan, and further develop needed coping skills.

RPT assists the client in enhancing motivation to change through values exploration and the use of a decisional matrix that evaluates the costs and benefits of lifestyle change versus the status quo. RPT attempts to improve self-efficacy by providing positive feedback on the person's strengths, progress in treatment, and skill development. The change process is defined as one of learning and skill building.

Professional training in RPT is encouraged. Recommended RPT readings are listed in appendix G of this book.

MBS Approach to Relapse Prevention: Sobriety in the Service of Value-Based Living

Mindfulness-based sobriety is consistent with principles of contextual behavioral science and the third-generation cognitive behavioral therapies that fall in that camp. Third-generation CBT differs from second-generation CBT in fundamental ways. One way has to do with the attitude toward thoughts or beliefs. In second-generation CBT, treatment efforts were made to replace beliefs that were considered irrational, inaccurate, or problematic. Techniques referred to as cognitive restructuring and thought stopping were used on that basis. In contrast, third-generation CBT regards such efforts as potentially counterproductive. According to third-generation CBT, efforts to eliminate or replace particular thoughts may increase the attention paid to the same thoughts. Consequently, "fusion," or struggle with the undesired thoughts, may occur. In the third-generation approach, thoughts are to be accepted and, at the same time, taken in perspective: thoughts are simply thoughts, not reality.

MBS incorporates much of the RPT model, including a focus on lifestyle balance, high-risk situations, "subconscious" cravings, positive outcome expectancies, and abstinence violation effects. However, MBS alters some of the RPT terminology and differs from RPT in some aspects of its approach to relapse prevention. These differences in approach, which are described below, are primarily related to MBS's being grounded in a contextual behavioral paradigm. Integrating key elements of RPT with ACT, in particular, could not be accomplished without some conversion of RPT to a contextual behavioral frame.

Terminology and Meaning

The MBS curricula have modified some of the terminology commonly used in RPT while still employing the basic concepts. These terminological changes result primarily from client feedback and clinician experience. As previously mentioned, "apparently irrelevant decisions" has the acronym "AIDs," which also refers to a serious medical disease. For some of our clients, the acronym AIDS is loaded with personal meaning that is unintended in the RPT model. While MBS does not use the RPT acronym AIDS, MBS does recognize that "subconsciousness" decision-making processes can lead to lapses. MBS refers to these processes as "prelapse behaviors."

Some clients reported experiencing the term "abstinence violation effect" (AVE) as too "technical." In therapy, we refer to aspects of the postuse experience—such as guilt, shame, and self-blame—without using a specific term.

While RPT categorizes high-risk situations in two broad areas, referred to as intrapersonal determinants and interpersonal determinants, it should be noted that RPT understands that it is the person's response to a situation, not the situation itself, that determines lapse or relapse (Larimer, Palmer, & Marlatt, 1999). To avoid possible misinterpretation and confusion, MBS does not use the term "determinant," because for some people, this term may imply a lack of personal agency, as if one's lapse or relapse were caused by factors beyond the person's control (such as emotions, physical discomfort, cravings or urges, conflict with others, social pressure, and pleasant experiences with others). For our purposes, we use the term "challenge" rather than "determinant." We emphasize that the person can remain committed to value-based living and pursue value-based goals even while experiencing difficulties. However challenging a situation may be, there is inherent within it some degree of choice or, to use an existential term, "situated freedom." Additionally, MBS does not use "positive" and "negative" in reference to emotional or physical states, because these terms, for some individuals, may imply judgment or nonacceptance.

Cognitive distortion and cognitive restructuring are sometimes used in RPT and frequently used in second-generation CBT. They reflect clinical conceptualizations and represent therapeutic approaches that may be inconsistent with third-generation CBT. The terms "cognitive distortion" and "cognitive restructuring" are not used in MBS. However, cognitive processes and thought patterns are approached in the contextual behavioral manner described below.

"Cognitive distortion" is a term that is commonly used in second-generation CBT, essentially implying that a particular belief is inaccurate vis-à-vis reality—that is, that the belief is a distortion of reality. Our approach, which is more consistent with third-generation CBT, as well as some humanistic-existential approaches, emphasizes that cognitions (or beliefs) of any kind are not and cannot be synonymous with reality. We regard cognitions (including beliefs) as "tools" or, more specifically, constructs that are generalized from direct experience or less-direct learning processes (reading, lectures, and so on). Beliefs are intended to serve functions related to navigating oneself in and through one's life in the world. This being the case, "breadth of perspective" and "functionality of belief," rather than "cognitive distortion," are the salient issues.

Similarly, MBS does not use the term or the technique of cognitive restructuring. We do recognize that clients sometimes have beliefs that they associate with distressful feelings, beliefs that they try to avoid experiencing. Unfortunately, attempting to modify, or rid ourselves of, unwanted beliefs often results in greater focus on and attachment to the same beliefs. MBS adopts a contextual behavioral approach wherein problems are seen as occurring when we assume that our beliefs are direct representations of reality and when we become fused with those beliefs.

Our approach, which begins with awareness, observation, and acceptance of internal experiences, allows the opportunity to acknowledge the relativity of beliefs: that beliefs serve functions and are not synonymous with reality. In cases where the person's inner language has included self-degradation, he can now see that the "negative self-talk" does not represent reality but is a self-made construct. Functionality of beliefs can be explored in regard to the history of the beliefs and their behavioral implications and consequences. This process, which may open one to alternative perspectives, can be quite liberating and empowering.

High-Risk Events and Scenarios

While influenced and inspired by RPT's high-risk categories, MBS understands high-risk events (past occurrences) and high-risk scenarios (postulated future occurrences) in a contextual manner: experience in situation, that is, the individual's personal experience in the situation where it occurred or is anticipated. Both polarities, experience and situation, are considered in analyzing an event or assessing an anticipated scenario. Accordingly, MBS offers a noninclusive list of categories for both experiences and situations that may represent personal challenges, especially as related to sobriety.

EXPERIENCES:

- Emotions: Undesired or desired emotions such as joy, sorrow, hope, despair, courage, anger, frustration, desire, aversion, guilt, shame, love, hate, sadness, loneliness, fear, happiness
- Thoughts, including self-critical or judgmental, glamorizing substance use, worry or rumination, confidence related, other beliefs

- Sensations:
  - Physical discomfort accompanying withdrawal from drugs or alcohol, including cravings and urges
  - Pain: Chronic or acute
  - Physical sensations one might associate with an oncoming panic attack
  - Other sensations
- Memories:
  - Trauma related
  - Pleasant memories, such as those associated with prior use
- Visualizations, imaginations, and forms of fantasizing
- Behavioral tendencies:
  - Impulses and compulsions
  - Other behavioral tendencies
- Experience of intent (that is, intending to do something):
  - Intending to get rid of or decrease unwanted feelings
  - Intending to test one's ability to drink or use drugs in a controlled manner
  - Intending to self-reward through substance use ("I deserve this")
  - Other intent

SITUATIONS:

- Social:
  - Conflicts
  - Peer pressure
  - Interactions, including "medicating" social anxiety and enhancing intimacy
  - Celebrations and entertainment, such as weddings, sporting events, graduations, music concerts
  - Isolation
  - Other social settings
- Places, such as where alcohol or other drugs are prevalent or can be obtained
- Time related, such as holiday, anniversary, time of year, stage of life, other time-related factors
- Other situations

EXPERIENCE-IN-SITUATION EXAMPLES:

- Feeling anxious at a party where one then indulges in smoking marijuana
- Feeling happy at a wedding preceding drinking alcohol
- Anxiety regarding an upcoming social event and taking nonprescribed benzodiazepines
- Reminiscing about old times and then snorting cocaine
- Feeling lonely and using heroin
- Remembering being attacked while watching a provocative movie and using amphetamines soon after the movie's end

Experience-in-situation possibilities seem nearly endless, and scenarios need to be individualized. As much as possible, we use the person's own words while processing to elicit details and clarify nuances. The purpose is for the person to understand his risks well enough to develop strategies and skills in the service of sobriety maintenance.

Analyzing past relapses can be very helpful in understanding experiences and situations that have increased, and may continue to increase, relapse risk. In MBS this analysis is a collaborative client-counselor process toward yielding descriptions. It is typically initiated by requests for descriptions or by open requests, such as:

- "Tell me what happened."
- "Please describe the situation."
- "Tell me about your experience leading up to and during the event."

Future scenarios can be imagined:

- "What kinds of experiences are challenging for you? In what situations might they occur?"
- "What kinds of situations are most risky for you in terms of maintaining your sobriety?"
- "What do you see coming up that might present challenges?"

This process, facilitated by the counselor's reflections and requests for clarification, allows the person to respond and self-reflect freely in forming experience-in-situation descriptions. Finally, the descriptive ("free-association") process may be followed by reviewing a list of experience and situation examples, which the person can use as prompts to promote further remembering or reflecting.

Relapse event analysis can be helpful in refining one's relapse prevention plan and in identifying needed skill development. It is also important to analyze successes in challenging situations, because this can help to clarify what worked while boosting self-efficacy. Additionally, writing in a journal can be helpful in recording and reflecting on challenges that turned into successes or other learning experiences.

SKILL ENHANCEMENT AND SOBRIETY PLANNING

Skill enhancement and sobriety planning in MBS considers both polarities of the experience-in-situation scenario. In MBS, the therapeutic process includes growth in one's capacity to be experientially present and nonreactive in an increasingly broader range of situations. The improved presence capacity (mindfulness), which facilitates safe and effective situation exposure, expands options and opportunities in regard to one's pursuit of value-based goals and activities.

The experience polarity. In regard to experience, the approach is twofold: identifying experiences that are personally challenging and frequently avoided, and increasing skills of being present while nonreactive during challenging experiences. In regard to challenging experiences that are avoided, of particular importance are what is avoided, how the person avoids, and the consequences of avoiding. That which is avoided often includes areas where the person experiences or anticipates intense anxiety, emotional pain, distressing thoughts, or threats to his sense of self-worth. There is a broad range of possibilities in regard to how the person avoids. This range includes external-geographical avoidance of situations in which these experiences are more likely to occur—such as people,

places, and things—and it also includes "internal" psychological processes, such as self-distraction and levels of experiential dissociation. Substance use may "assist" avoidance efforts by aiding in distraction or through chemical numbing. Consequences of avoiding typically include a "benefit" and a "cost." The benefit may be immediate relief from "distress," while the cost may be a compromise in regard to longer term quality of life; for example:

- Sheldon avoids intimacy in relationships, because anticipated intimacy "brings up" memories associated with emotional pain. While consequences may include immediate relief, rewards of interpersonal closeness are forgone.

- Lucy drinks alcohol to "not think about" failing to live up to self-expectations. While consequences may include immediate relief, the substance use delays her pursuing value-informed goals.

The second aspect of the therapy approach involves increasing skills of being present while nonreactive during challenging experiences. Mindfulness practice is central to this approach. In MBS, key foci relate to that which had been avoided through prior substance use and that which, when avoided, has serious impact on quality of life.

- Through mindfulness practice, Sheldon progressively increases his capacity to "be with" uncomfortable aspects of his experience, which furthers his ability to be in intimate situations.

- Through mindfulness practice, Lucy becomes able to experience feelings of inadequacy without reacting, while pursuing important life aspirations (value-based goals).

Situation exposure and therapeutic avoidance. Ideally, one might have the capacity to be fully mindful of any experience that presents in any kind of situation. However, few of us accomplish this. An objective of MBS is for the person to have skills to be present and nonreactive in a broad range of situations, particularly where doing so is in the service of value-based living.

Types of situations will vary for the person in terms of level of risk and level of value. MBS uses a continuum measure, the "Situation Rating Scale and Action Plan," to assess both risk severity and personal value in regard to particular situations. Thus, MBS considers these two dimensions, risk and value, in the decisional process. Strategies for addressing risky situations fall in two general categories: avoiding the situation (provided that it can be avoided) and coping within the situation. Whether or not to avoid or cope within a situation may be advised by the following considerations:

- Situations prompting higher risk of using weigh on the side of avoidance.

- Higher importance (value-related) situations weigh on the side of coping within the situation.

For "high-risk/low-value" situations, strategies may be developed to avoid them. For "low-risk/high-value" situations, there may be no need to avoid; approach or involvement may be consistent with value-based living. Complexities may arise in the middle ground. For instance, strategizing how to address high-risk/high-value situations can be a tension-laden process. When decisions are made to cope within (rather than avoid) these situations, well-developed coping strategies are usually indicated. They may include enhanced coping skills, social supports, and situation-specific strategic planning. One example of a high-risk/high-value situation might be a niece's wedding where there is champagne on the table, others are drinking and feeling jubilant, and the person is expected to lead a toast. Preparation for coping in this situation might involve accompaniment by a sober friend, presence of nonalcoholic sparkling cider, well-rehearsed alcohol-refusal skills, and a convenient exit strategy (should one be needed).

As time goes on and the person's mindfulness and grounding in his values have developed, prior risky situations may become less risky. Circumstances involving moderate-to-high values that had been avoided may be successively approached. Of course, for situations that offer little or no value, there may be no reason to engage.

MORE NOTES ON EXPERIENCE IN SITUATION:

- We cannot directly choose our experience.

- There are some situations we cannot choose, such as family of origin, some medical problems, time of year, and so on.

- We can choose some situations, such as friends, peers, residence, school, work, leisure or recreational sites, and other places.
- Choosing any particular situation usually has both benefits and costs, and giving up something is not always easy. Not choosing also has both benefits and costs.
- Sometimes giving up things is easier than we think; sometimes it is not.
- All exposure is fundamentally experiential exposure.
- Nonreactive experiential exposure can enhance mindful maturity.
- One can be in a situation and not be experientially exposed to it, that is, experientially present and mindful. Avoidance of experience can be "accomplished" by such processes as self-initiated distraction and dissociation.
- The crucial issue is always what one does.
- We can choose how to respond or act.
- We can choose to be present and mindful.

Clinical Interventions That Complement Mindfulness-Based Sobriety

Use of an MBS curriculum may be accompanied by use of other clinical curricula and practices. In general, MBS-compatible approaches are those that can be delivered in ways that are consistent with the spirit aspect of motivational interviewing and contextual behaviorism.

CLINICAL SERVICES COMPATIBLE WITH AND COMPLEMENTARY TO MBS:

- Individual motivational interviewing sessions, as needed
- Individual psychotherapy, as needed
- Family counseling
- Multifamily psychoeducation
- *The Twelve Step Facilitation Outpatient Program* (Nowinski, 2006)
- *Helping Women Recover: A Program for Treating Addiction* (Covington, 2008)
- Group interventions to help with co-occurring mental health problems
- Treatment of co-occurring trauma-related issues:
    - *Acceptance and Commitment Therapy for the Treatment of Post-Traumatic Stress Disorder and Trauma-Related Problems* (Walser & Westrup, 2007)
    - *Beyond Trauma* (Covington, 2003)
    - Dialectical behavior therapy skill building in *Skills Training Manual for Treating Borderline Personality Disorder* (Linehan, 1993)

Conclusions

This chapter presented the general therapeutic frame and approach of MBS, including the nature and importance of the therapeutic relationship; the overarching principles of awareness, values, and commitment; the primary

contributing models—acceptance and commitment therapy, motivational interviewing, relapse prevention therapy, and mindfulness—the concept of sobriety in the service of value-based living; and complementary clinical interventions and curricula.

MBS therapy occurs within a therapeutic alliance. The person's therapeutic relationship is with the therapist and the group. The therapist employs a fundamental person-centered, collaborative approach, respecting the autonomy of the person as his own "change agent." Awareness of one's life situation is improved, personal values are clarified, motivation is enhanced, and commitment to change is elicited through reflection, mindfulness meditation, and guided curriculum exercises. Strategies to help cope with sobriety challenges are developed and refined during the recovery process. One particular area of clinical interest is that of events and scenarios that hold a high risk for relapse. To analyze high-risk events and scenarios, the clinician and client collaborate in reviewing prior lapses and possible future scenarios in terms of the client's experience-in-situation, as well as the client's response options. Sobriety planning includes strengthening mindfulness skills and troubleshooting in regard to risky situations. Situations are rated in terms of risk level and value level, and strategies are developed to minimize risk and maximize value.

Chapter 2

Mindfulness-Based Sobriety in a Continuum of Care

Go ahead and be the best imperfect person you can be and get started on those things you want to accomplish before you die.

—Shoma Morita

Addiction treatment occurs across a continuum of care, consisting of early intervention; outpatient treatment; intensive outpatient treatment; partial hospitalization; low-, medium-, or high-intensity residential treatment; medically monitored intensive inpatient treatment; or medically managed intensive inpatient treatment (Mee-Lee, 2001). The individual enters treatment into the level of service that meets his unique set of needs and progresses through less-restrictive levels as cravings and biomedical and co-occurring mental health issues become manageable; motivation is enhanced; relapse prevention plans and skills are strengthened; and recovery supports are sufficiently in place.

MBS Residential and Intensive Outpatient Curricula

The MBS model in this book provides curricula for two levels of care: intensive outpatient (IOP) and residential (either medium or high intensity). The primary treatment focus is on factors that contribute to the need for the particular level of care within which the person is placed. In the MBS residential curriculum, the balance of concern is on what needs to be accomplished prior to the person's "stepping down" to an outpatient level of care. Since sobriety is usually the norm in a controlled residential environment, relapse prevention planning is typically done in anticipation of discharge. The MBS IOP curriculum focuses on the person's more immediate life-in-community situation, including experiences and events that happened on the way to the group and the previous night at home, at work, at school, and so on. Thus, in IOP, relapse prevention planning is done in the context of current and ongoing sobriety challenges: what worked and what didn't work so well.

Partial-Hospitalization Programs

In the service continuum, the partial-hospitalization program (PHP) level of care lies between residential and IOP levels. While IOP provides clients with 9 to 19 hours of services per week, PHP provides 20 or more hours of clinically intensive services.

Although MBS does not have a specific curriculum for the PHP level, either the MBS residential or the MBS IOP curriculum may be used, depending on the circumstances. In general, the MBS residential curriculum may be more suitable for a PHP that is accompanied by some type of sober-living environment, such as a recovery home or sober-living residence. Alternatively, the MBS IOP curriculum may be more suitable for a PHP without a formal sober-living arrangement.

Mindfulness-Based Sobriety and ASAM Patient Placement Criteria

The *ASAM* (American Society of Addiction Medicine) *Patient Placement Criteria for the Treatment of Substance-Related Disorders* (ASAM PPC-2R) (Mee-Lee, 2001) is the standard level-of-care placement tool in addiction treatment. While the ASAM PPC-2R is a tool of utilization management, it can also be used to help organize clinical thinking and treatment planning. The ASAM PPC-2Ris composed of six dimensions, and the person is rated in terms of level-of-care appropriateness on each dimension:

1. Acute Intoxication and Withdrawal
2. Biomedical Conditions and Complications

3. Emotional, Behavioral, or Cognitive Conditions and Complications
4. Readiness to Change
5. Relapse, Continued Use, or Continued Problem Potential
6. Recovery Environment

Please refer to the ASAM PPC-2R for specific level-of-care characteristics and criteria. A fundamental principle is that individuals should be admitted to the least restrictive level of care that allows them to succeed in treatment. Accordingly, in any level of care, treatment focuses primarily on factors that represent barriers to the person's transitioning to a lower level of care. The MBS curricula address therapy issues that can be organized within ASAM dimensions in ways that are described below.

Dimension 1: Acute Intoxication and Withdrawal

Addiction medicine interventions that are not directly part of the MBS curricula may be indicated to assure physical safety in managing complicated withdrawal symptoms and reducing the intensity of cravings. MBS provides psychological help to address cravings, urges, and other discomfort associated with acute and postacute withdrawal. Mindfulness, urge surfing, value-based living, and motivation enhancement may be especially helpful for the person who is learning to cope with cravings without using.

Dimension 2: Biomedical Conditions and Complications

Biomedical interventions may be indicated in regard to pain management and to treat medical problems that may affect one's physical well-being, self-esteem, and life outlook.

In addition to primary medical interventions, MBS therapeutic tools such as mindfulness, urge surfing, value grounding, and motivation enhancement may help the individual cope with biomedical and other unpleasant physical states without abusing substances.

Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications

This ASAM dimension is generally considered the one that takes into account a range of problems related to mental health that commonly occur in addiction treatment populations. In MBS, the focus is not on particular diagnoses. Instead, the concern relates to the person's way of responding to his experiences of emotional, behavioral, or cognitive natures. Of particular interest in substance abuse treatment is the extent to which the person attempts to avoid internal experiences (distressing feelings, behavioral impulses, and thoughts) through substance use and through other means that diminish individual meaning, purpose, and quality of life.

EMOTIONAL

Individuals sometimes use substances to avoid undesired emotions—such as anger, frustration, anxiety, fear, sadness, guilt, shame, regret, embarrassment, and humiliation—or to enhance pleasurable ones, such as happiness, joy, a sense of strength, companionship, and celebratory feelings.

As stated previously, ACT, RPT, and other mindful approaches typically relate to existing "negative" emotional states, such as anxiety and sadness, with an attitude of observation and acceptance, without "fusion" (that is, without merging with or clinging to the emotion). According to ACT, attempts to eliminate or escape from unwanted emotions may be futile, because such attempts may increase focus and attention on the emotions. Mindfulness, an acceptance-based attitude, can help the individual be with, rather than struggle with, unwanted emotions, thoughts, and behavioral urges while not being driven or guided by them. Value-based living provides the inner guidance toward meaningful living and improved quality of life.

BEHAVIORAL

Behavioral factors include patterns of behavior or actions that may put the person at risk for relapse (depending on the severity and the frequency at which he engages in the behavior). Included in this dimension are thoughts, impulses, and compulsions related to acting in a value-inconsistent, socially inappropriate, or illegal manner (for example, behaviors that are violent or dangerous to self or others, personally or socially unacceptable sexual activities, behaviors that victimize others, and so on). Although compulsions, impulses, and urges can include biophysiological, psychological, and social factors, how one behaviorally responds to an urge to act is a crucial therapeutic concern. In the end, addiction recovery all comes down to what the person does. Sobriety is a product of one's behavior.

COGNITIVE

For the purposes here, *cognition* is defined as a mental event. Thoughts, feelings, and perceptions are among the commonly experienced cognitions. In addiction and mental health treatment, a person's way of relating to cognitions influences therapy outcomes, either favorably or not. In general, attempts to either avoid thoughts or "fuse" with them perpetuate a problematic status quo, while acceptance without fusion leads to therapeutic progress.

Attempts to avoid experience can progressively limit the individual's field of operation and range of options. This may be evident when the person avoids types of situations in which panic attacks have occurred. As additional situations seem to be "panic prone," the field to be avoided increases until the person potentially finds himself in a state of agoraphobia. The scenario is similar with both phobias and trauma-laden stimuli. Acts of avoiding anxiety-associated objects typically result in immediate relief, thereby reinforcing the phobia-avoidant behavior; yet the longer-term result is a more limited range of operation. Stimuli associated with traumatic events may be kept at a distance, because they risk prompting frightening experiences. Trauma-related avoidant behaviors can be physical (not going to places, meeting with people, or participating in activities) or cognitive (internal attempts to avoid thoughts or feelings, such as by mental distraction or dissociation). Further, traumatic reexperiencing may be avoided or dampened by substance-induced experiential numbing. Whatever the context and however the form, avoidant behavior may diminish opportunities to develop and improve coping skills, thereby sacrificing long-term growth and development.

Fusion with cognitions can present another set of clinical issues. For example, fusion with thoughts that are self-critical in content is often associated with depression and anxiety, while fusion with suspicious thoughts may be accompanied by fear, paranoia, or anger. Cognitive processes involving worry and rumination may be fueled by one's fusion with a sense of incompleteness, dread, or unsettledness and, over time, may be biopsychologically exhausting. Merger ("fusion") with thoughts limits conceptual flexibility and options.

Finally, cognitions that affect substance use may also occur in the form of beliefs that present substance use in favorable ways. This includes glamorizing substance use, where a person might associate substance users with "positive" qualities, such as being "cool" or "popular." Seeking relief from or simply fusing with these cognitive experiences through substance use is a common relapse risk factor, because it tends to perpetuate patterns of substance abuse and dependence.

Mindfulness provides an alternative to both avoidance and fusion. Mindfulness approaches typically advocate that one relate to cognitive events through observation, acceptance, and nonattachment. The person neither avoids nor fuses with the cognition. The individual can be with unwanted thoughts while not identifying with or being driven by them. Instead, personal values provide the motivation and guidance toward meaningful living.

Dimension 4: Readiness to Change

Motivation and readiness to change are essential to recovery. Individuals who have clarified and are grounded in their personal values are more likely to express desire, ability, reasons, need, and commitment to change. Accordingly, exploration and clarification of personal values, commonly practiced in both MI and ACT, are central to MBS.

Supplemental MI through individual counseling may be indicated for some individuals.

Dimension 5: Relapse, Continued Use, or Continued Problem Potential

This dimension considers a broad range of factors that might contribute to relapse risk, including inability to

recognize “triggers,” or prompts to use; craving intensity; difficulty coping with cravings; tendency to seek immediate gratification; difficulty coping with pressures from others to use; and insufficient motivation to remain in treatment at the clinically indicated level of care.

MBS addresses these concerns through awareness raising, motivation enhancement, skill development (including mindfulness, coping with cravings and impulses, and drug refusal and assertiveness skills), strategic sobriety planning, and mindful-perspectives development to help the individual achieve and maintain sobriety. A “Situation Rating Scale and Action Plan” (IOP program, chapter 4, session 4) helps the individual assess both risk severity and value level (importance) of risk for relapse of situations; this provides context and guidance for sobriety planning.

Finally, grounding oneself in personal values can be especially helpful for people who tend to be motivated by external controls (others) rather than by personal values.

Dimension 6: Recovery Environment

This dimension considers a broad range of environmental and social network factors, including home, neighborhood, work, school, family, friends, and peer supports. Increasingly, one’s social sphere includes technology-assisted networks, which may involve popular social networking websites, texting, e-mailing, and accessing online contacts and products.

There are two general therapy approaches in regard to recovery environment. The first is to help the person better strategize and cope with his situation. The second is to attempt to directly influence the environment. MBS focuses on the former. It offers sobriety planning and coping-skill enhancement, both of which help the person to better survive and succeed in challenging environments. MBS also guides reflection and processing in areas of value-based living, motivation enhancement, and enrichment of quality of life.

MBS does not try to directly affect the individual’s recovery environment.

Note: Although MBS does not intervene directly in the individual’s recovery environment, therapy elements such as values clarification, perspectival changes, and motivation enhancement may facilitate processes through which changes in one’s recovery environment may occur: for example, a person’s choice to associate with different people, change jobs (leaving one where substances were used at work), relocate residence, connect or reconnect with positive acquaintances, engage in healthy social situations, and pursue value-based ambitions. Engaging in family counseling and developing positive peer supports (such as 12-step fellowship facilitation) are important approaches that attempt to directly improve the recovery environment. Using family counseling and social network enhancement in conjunction with MBS may add breadth and depth to the clinical package.

Conclusions

In addiction treatment there are levels of care that range from early intervention to medically managed intensive inpatient treatment. In between are levels that include outpatient and residential programs. MBS provides two curricula: intensive outpatient and residential. Either of these curricula may be used for a partial-hospitalization program (PHP), depending on the particulars of the PHP.

A general principle is that clients should be placed in the least restrictive level of care that suffices toward achieving and maintaining sobriety. Informed by the “least-restrictive” principle, MBS focuses its curricula on the factors that contribute to the person’s need for the particular level of care in which he or she is placed. At the residential level, the question becomes, What will it take such that the person can transition to a lower level of care, outpatient, and have a reasonable chance of maintaining his newly achieved sobriety? Residential treatment occurs in a controlled environment where one has some protection from high-risk situations, including peer pressure to use and immediate access to drugs. Primary treatment challenges in residential treatment typically include retaining the person in treatment, coping with cravings in postacute withdrawal, enhancing motivation to change, and developing sobriety plans that include strategies to avoid or cope with the risky scenarios one may face upon returning to the community at large.

In IOP, the MBS model focuses on what is going on in the person’s more immediate life, for example, on the way to the group or the previous night at home, at work, at school, and so on. Sobriety planning is refined through ongoing experiences of what worked and what needs improvement. While MBS IOP focuses on achieving and maintaining sobriety, its ultimate goal is the person’s independent self-maintenance of sobriety, that is, continued

sobriety in the absence of formal treatment.

Chapter 3

Therapeutic Principles and Facilitation

It is the very pursuit of happiness that thwarts happiness.… Happiness cannot be pursued; it must ensue.

—Viktor Frankl

Use of curricula in group treatment provides structure for the therapeutic process. It helps assure that individuals in addiction recovery are exposed to important principles and skill development, it prompts clinicians to address key topics, it provides a common language for clinical communication, and it contributes toward seamless coverage in the event of a group facilitator's absence. Additionally, training on curricula provides clinicians with knowledge and skills in specific concepts and strategies.

Open-Group Curricula

More often than not, treatment curricula are designed to be implemented sequentially—that is, beginning with session 1 and then proceeding in order such that the individual's knowledge base and skills develop progressively. Knowledge and skills acquired in earlier sessions make the platform for knowledge and skills addressed in later ones. Typically, there is a defined number of sessions, concluding with a termination session. Sequential approaches lend themselves to individual counseling. They are also used in "closed-group" formats, where all clients begin at the same time with session 1 and progress as a group through a series of sessions, each building on the ones that came before. There are well-designed, evidence-based sequential models available, and there is certainly value in sequential approaches when and where they can be implemented. Unfortunately, economic and other practical realities make those opportunities rare. Treatment programs commonly adapt evidence-based curricula from closed-group formats for use in open groups. There are seldom any guidelines in sequential curricula to inform such adaptation. Consequently, evidence-based curricula are sometimes modified and implemented in ways that are inconsistent with the manner that gave evidence of their effectiveness.

While adapting closed-group models to open-group formats may sacrifice design advantages of the particular sequential curriculum, it also risks failing to recognize and use strengths inherent in open-group approaches.

Mindfulness-based sobriety is a collaborative approach to open-group therapy that is intended to help individuals with substance-use problems achieve and maintain sobriety by enhancing awareness, accepting experience, and committing to value-based living.

Strengths of Open-Group Approaches

Group therapy in either open or closed design offers opportunities to improve peer relations and supports, enhance social skills, obtain feedback from others, provide feedback, observe others in change processes, and practice new skills in role-play scenarios.

MBS was designed as an open-group curricula. Use of open-group formats may offer both operational and clinical advantages. Operationally, open groups offer scheduling efficiency and economic benefits. New clients can join groups at any juncture. Vacancies in group membership do not have to continue until the next cycle begins for the closed group. The prospective client can more easily access treatment without having to approach a different treatment provider.

Clinically, open groups may better lend themselves to treatment engagement by allowing the person to begin treatment when he or she is motivated to do so. With a closed-group model, what can you tell the person who has to wait for the next group cycle to begin? What are the person's options? Continue the status quo, attempt sobriety on your own, or seek help elsewhere. By the time a new closed group begins, the person's motivation for treatment may have waned.

Open groups may better lend themselves to individualized therapy. There is a range within the client pool in terms of readiness and preparedness for treatment ("starting points") and in regard to treatment needs. Some clients are entering treatment for the first time, while others have prior treatment experience (sometimes with periods of sobriety success). Clients also vary in terms of severity of addiction, co-occurring issues (mental health and medical), quality of life, coping skill sets, and social supports. Consequently, a closed group of individuals starting at the same time, undergoing the same curriculum at the same pace, and then concluding at the same time may lack flexibility in accounting for participants' individual strengths, problems, and needs. In open groups, clients' lengths of stay vary. Some will need repeated, reinforced exposure to therapy content and skill practice, and additional in-treatment life experience accompanied by refined sobriety planning. Others will need less time in treatment.

At any point in time, open groups consist of a set of individuals who have been in the group for varying lengths of time, some more experienced with therapy and some newer to the process. This allows new members to learn from the experiences of others who may have faced similar challenges and who have experienced some success. This can be a source of hope and can help to enhance the newer person's self-efficacy: "If he succeeded, then I can too." Conversely, the person who is more experienced with therapy can perceive in the newer member a reflection of how he was in the beginning stages of therapy and therefore gain a sense of accomplishment. Additionally, clients who are experienced at being in therapy can take client-leader roles, which further develop their knowledge, skills, and confidence.

Finally, there may be advantages to the nonlinearity inherent in open-group approaches. The MBS curricula present in each session a holistic perspective of the model. Specific session topics are presented within the larger framework, and topics overlap to reinforce assimilation of knowledge and response fluidity.

Orienting New Members to the MBS Open Group

Since clients may come into the MBS group at any juncture, it is important that they have sufficient orientation to the model. This can be accomplished in two ways, and it is recommended that both take place:

- A brief overview of the MBS model is offered in each group, providing contextual grounding in which each topic can be understood (see appendix B).
- A new client may be oriented through an individual session in which the counselor provides an overview of fundamental aspects of the MBS model.

Addressing Stages-of-Change Issues in an Open-Group Format

In 1983, Prochaska and DiClemente introduced the Transtheoretical Model of Change (Prochaska & DiClemente, 1984), which included readiness-to-change levels:

- *Precontemplation:* The person is not considering making a change.
- *Contemplation:* The person is considering making a change but has not decided to do so.
- *Preparation:* The person has decided to make a change and is preparing an approach.
- *Action:* The person is actively addressing his problem.
- *Maintenance:* The person has achieved and is now in the process of maintaining sobriety.

Since then, the term "stagewise treatment" has been used to refer to attempts to match the treatment approach with the individual's readiness-to-change level. Accordingly, recovery-skill development (of drug refusal skills, for instance) is sometimes considered appropriate for clients in the "action" stage of change—that is, for clients who are motivated and are taking steps to change. Conversely, these same approaches are often considered inappropriate for clients who are in the precontemplation or contemplation stage of readiness, that is, for clients who have not yet decided to change. The thinking is something like this: "Why teach drug refusal skills to someone who is not motivated to stop using? The focus should be on enhancing motivation." Some who advocate "stagewise treatment"

adopt this premise.

While the logic of the above premise is apparent, this curriculum assumes a more nuanced understanding of the issue and adopts a modified approach. MBS takes into account that motivation to change can be influenced by self-efficacy. For instance, a person who lacks confidence in his ability to succeed in making a change may lack motivation for that very reason. After all, why would someone be motivated to attempt something that he didn't believe he could accomplish? From the person's perspective, attempting the unachievable would be an exercise in frustration, resulting in failure and feelings of inadequacy. This curriculum holds the perspective that skill development can help to increase one's confidence and thereby enhance motivation to change. A secondary point here is that skill development may provide resources that can be used at a later time, if and when the person becomes motivated.

In regard to individuals who are already motivated to change, would they benefit from motivation enhancement therapies? The answer to this depends on the particular methods used. Attempts to enhance motivation by cost-benefit analyses may run the risk of evoking benefits for not changing along with the benefits of changing. For some, this may help to strengthen motivation to change, while others may be more attracted to the "status quo" side of the decisional balance. Given the diversity within any group and the risk that some may respond by lowering motivation, MBS does not use cost-benefit or decisional balance exercises with the therapy group.

In MBS, motivation enhancement is conducted in two general ways: through individual work in the group session and through group exercises. In individual work, motivation may be increased by direct attempts to develop discrepancy between how the person wants his life to become and his current set of behaviors. In group exercises, motivation may increase and commitment may be strengthened through values clarification and enhancement. Please refer back to "MI in MBS" in the "Motivational Interviewing" section of chapter 1 for a description of motivation enhancement in MBS group therapy.

General Facilitation Issues

The purpose of this section is to provide guidance on facilitating processes and addressing issues that are common in intensive outpatient and residential group therapy sessions.

Flexibility of Focus

Session outlines and descriptions for MBS group therapy are presented in chapters 4 (IOP) and 5 (residential). While clinicians will generally follow these session plans, variations may occur when they are clinically warranted. Each session includes an initial check-in where group members discuss recent challenges and successes they have experienced. This check-in may also be used to discuss issues related to group dynamics. Still, significant issues may emerge at times after the check-in period or with needs that extend beyond the time allotted for the check-in. When this happens, clinical judgment should be used. The MBS position is to prioritize current issues and situations in the clients' lives. Accordingly, the topic of the day might be shortened or suspended to address immediate concerns. It should be acknowledged that a deviation from the plan is an exception, not the rule. Continued deviations from session plans may signify problems with group dynamics or group facilitation.

In addition to unexpected significant issues (as mentioned above), the authors recognize that there may be occasions when the topic content, and related group exercises and discussion, requires more time than is indicated in the session outlines and descriptions. When this occurs, the facilitator will again need to make a clinical judgment call. If the apparent value of the topic being presented and discussed is high, the facilitator may need to condense or omit a subsequent topic in the session to allow the necessary time. In some cases, content in a single session may be spread over two sessions. As facilitators become increasingly familiar and adept with the MBS model, clinically indicated modifications will occur naturally.

Processing in Group Therapy

In chapters 4 and 5, where there are descriptions of IOP and residential sessions, references will be made to the facilitator processing various matters with the group. They include analyzing recent events; troubleshooting challenges; and assessing and improving viability of relapse prevention plans, therapeutic discord in the group, perceptions, comprehension, life strategies, emotions, and responses to life challenges.

Processing occurs in a context established by the facilitator: the meeting place and time; session structure and content; and an accepting, growth-oriented therapeutic undertone. The facilitator's approach should be one that elicits nonjudgmental, therapeutic discussion. Open questions and the use of reflections in a nonjudgmental atmosphere help group members to feel safe in their mutual vulnerability, as they reflect on and disclose personal information.

To establish the therapeutic atmosphere, the facilitator functions as a role model in respecting the questions and ideas of group members. There are no "stupid" questions and no "wrong" ideas. In processing a group member's past actions, the facilitator should seek openings to affirm good-faith efforts and strengths, even in situations in which the person may have fallen short of his values or goals. "Shortcomings" are "grist for the mill"—learning opportunities, not personal failures. The objective is always to promote sobriety through understanding and appreciating the challenges involved, respecting autonomy, providing steadfast support, and recognizing the person's ability to succeed in value-based living.

Personalizing the Approach

It has been our experience that facilitators who know their clients tend to be more engaging than those who do not. Being able to ask members about the outcomes of significant (or seemingly insignificant) events that happen outside of treatment—for example, being able to ask a client about the outcome of his son's baseball game or the wedding he attended the previous weekend— demonstrates to clients that the facilitator genuinely cares and is invested in their well-being. Also, knowing the client's interests, passions, and hobbies can be an important factor in engaging him in treatment and establishing rapport. An example of this would be involving clients in brief conversations about their favorite genres of music or movies or their favorite sports teams before, after, or during group (when relevant). Although we are mindful of the challenges of high caseloads and turnover, attempts to get to know clients may contribute toward engagement, retention, and successful completion.

Another important factor is the ability of the facilitator to be creative and bring the clients' experiences to the material. The facilitator can do this through summarizing, broadening, and generalizing "real-life" experiences presented by group members so that the whole group can benefit. This can also be accomplished by linking group members' experiences (with permission) to topics and concepts that were presented in the current or previous sessions. Finally, the facilitator should conduct "temperature checks" on a regular basis. Checking in with the clients in regard to comprehension, pace, and relevancy can be important for engagement and member success. The facilitator can do this by asking questions such as "Does this make sense?" and "Any questions?" The facilitator may also invite the members to ask questions in order to encourage curiosity, honest exploration, and feedback on the members' behalf.

Some Awareness Areas and Tips

If an individual doesn't want to talk or disclose something to the group, the facilitator initially can respectfully ask if the group member would like to discuss the reasons for not wanting to talk (don't push; respect the person's decision to remain quiet). If the person appears to be in distress or if the silence continues through multiple sessions, the facilitator or another clinician should meet with the group member individually.

When a client lapses, the clinician can help the person process and learn from the experience.

LAPSE MANAGEMENT:

- Lapses are commonly accompanied by intense feelings of guilt or shame, and the client may be inclined to move toward a full relapse. One way to address this issue is to describe the lapse as a "slip," "mistake," or "learning experience," rather than as a "failure."
- What was the situation in which the lapse occurred?
    - Had the client identified it in the past as a "risky situation"? Was it a "blind spot?" Are there other blind spots?
    - What is the "risk/value" ratio of this type of situation? If it's high risk/low value, what might be done in the future to avoid such situations? If it's high value, what might be done to prepare and

better cope with it?

- What possible decisions may have led to the person encountering the risky situation?

- Coping strategies may be reviewed and modified.
- If motivation was a significant contributing factor, motivational interviewing may be considered.
- The client's expectations about substance use can be explored:
  - What were the expectations?
  - Did the outcome of substance use result in the pleasant experience that the client expected?
- The clinician can review with the client immediate versus delayed effects.

When a client experiences urges, what can a clinician do?

- Normalize cravings: Cravings are normal. They come and they go. They do not indicate signs of weakness or ineffectiveness. It is how we respond to urges and cravings that makes the difference.
- Teach and practice "urge surfing" (see appendix A).

When a client has been in a risky situation and didn't use:

- The clinician should first affirm the person's success in not using. Many clinicians fall into the trap of scolding someone for having been in a risky situation. This is counterproductive and may result in a missed opportunity to affirm the use of skills gained in treatment.
- How did it happen that the client was exposed to a risky situation? What possible decisions may have led to the exposure?
- What coping skills worked for the client?

The clinician should seize opportunities to:

- Enhance motivation using a motivational interviewing approach.
- Build the client's self-efficacy: View the change process as skills building. Help the client reflect on prior and current successes.
- Facilitate client skill development.

Conclusions

This chapter addressed therapeutic principles and general facilitation issues. The MBS curricula are designed for open groups: clients may enter the group at any session, provided that they are appropriate for the level of care and provided that there is an opening in the group. Consequently, sessions are designed to take this into account, and curricula attempt to best use advantages of the open group:

- Clients join the group when they are motivated and ready.
- Length of stay in open groups may be individualized.
- There is a seniority range in the open group, from new members to more-experienced members. This provides multiple perspectives on the course of treatment, reflecting progress and inspiring hope.
- There is a nonlinearity inherent in open-group approaches that may be used to help promote a holistic perspective of recovery.

Group session outlines are presented in MBS curricula and should be followed with infrequent exceptions. However, when significant issues emerge, clinical judgment should be used. The “default” position is to prioritize the current issues and situations in the clients’ lives.

The group composition may be diverse in regard to readiness to change: some are more sobriety motivated than others. MBS curricula include therapy content that can be applied across a broad readiness continuum. For example, skill-building interventions, traditionally considered appropriate for more-motivated individuals (in the “action” stage of change), are implemented in ways that may improve readiness to change for less-motivated persons. Skill acquisition may serve to improve motivation as a by-product of boosting confidence.

For individuals in different stages of the change process, motivation may be enhanced in MBS groups through values clarification exercises. Additionally, the facilitator may engage in brief, individual MI-based interactions within a group session as needed. MBS recommends individual MI counseling sessions (outside of group) where motivational issues are paramount and require more time than can be devoted to a single individual in the group format.

Caution should be taken, however, in regard to using some aspects of motivation enhancement therapy for individuals who are already motivated and committed to change. For instance, developing discrepancy about sobriety should generally be avoided, because this might reintroduce “ambivalence about changing” for a person who had already achieved sufficient motivation and commitment to change.

This chapter provided guidance on facilitating processes and addressing issues that are common in intensive outpatient and residential group therapy sessions, including check-ins, motivational interviewing spirit, knowing one’s clients, and integrating clients’ experiences into session materials. Awareness cues and counseling tips were presented to address lapse management; coping with cravings or urges to use; debriefing after exposure to risky situations; and integrating into treatment sessions motivation enhancement, self-efficacy strengthening, and skill building.

PART 2

MINDFULNESS-BASED SOBRIETY CURRICULA

Chapter 4

Mindfulness-Based Sobriety: Intensive Outpatient (IOP) Curriculum

Accept your feelings. Know your purpose. And do what needs to be done.

—Shoma Morita

MBS outpatient groups are designed to be comprehensive and flexible due to the diverse nature of the client population. Clients may enter outpatient treatment directly or as a step down from a higher level of care. Those who are stepping down from higher levels of care typically have a period of sobriety obtained in a highly structured residential environment. Clients sometimes report that they experience life after residential treatment as where "the rubber meets the road." Planning and skill development that occurred in the residential setting may have had, to some extent, a "hypothetical" quality. Once clients are back in the "real world," they may be faced with more direct sobriety challenges, such as having easier access to their substance of choice. In contrast, individuals entering directly into outpatient treatment may be in the early stages of sobriety. Those who are directly entering outpatient treatment may have unique challenges due to the fact that they have not spent any time removed from their living environments, daily habits, and routines (although this is not meant to diminish the types of challenges experienced in residential treatment).

In each outpatient session, there is a focus on what needs to happen (motivation enhancement, strategic planning, and skill development) in the service of sobriety both during and after treatment. MBS sessions help the individual develop:

- A life direction that is value informed.
- Plans and skills for staying on the path and for self-assessing and self-correcting whenever they veer away from value-based living.
- Sobriety strategies that they can review and modify as necessary. This includes analyzing exposure to and management of risky situations. In the face of challenges, what worked? What didn't work so well? What worked partially or for a period of time? Values and motivation may be enhanced; goals may be revised; coping skills may be assessed, reviewed, rehearsed, and further developed; and sobriety plans may be revised.

Open Group Therapy: Helping Clients in Different Places

The client population in any particular IOP open group typically is composed of individuals whose lengths of stay will vary. Considering this, it is important that clinicians are cognizant of curriculum exposure variance and the need to keep redundancy to a minimum. However, what may, on the surface, appear to be repetition in the curriculum content may also represent opportunities for rereviewing information and strategies, repracticing skills, broadening and deepening perspectives, and enhancing motivation. For instance, IOP clients with more seniority in the group may continue to refine their plans, values, skills, and approach. Clients with less treatment experience may come to see such individuals as symbols of hope, while more senior clients may view newer clients as evidence of their own progress.

Additionally, since clients may come into the MBS group at any juncture, it is important that they have sufficient orientation to the model. A brief overview of the overarching context (the MBS model) is presented in each session, and the session-specific topic is framed within this overarching context. The overview serves multiple purposes. For new and not-new clients, the overview provides a context within which the topic of the day can make sense. It provides or reinforces the "big picture" and reminds or prompts clients to be attuned to potential blind

spots. More experienced clients can be involved in presenting the overview of the model. Client orientation to the model can be enhanced further with one individual session, during which the fundamental aspects of the MBS model are reviewed.

SESSION SETTING AND MATERIALS:

Room Arrangement: For MBS sessions, a circular or semicircular arrangement of chairs is recommended, and the presence of a table, which may be experienced as an interpersonal barrier, is discouraged.

Facilitator Guides and Client Handouts: In order to conduct MBS sessions, the clinician will need facilitator guides and client handouts. These items are located in two places: the guides and handouts that are used in multiple sessions can be found in appendices A, B, C, and D, while the guides and handouts that are specific to individual session topics are located after the session descriptions. Additionally, facilitator guides and client handouts can be downloaded directly from www.newharbinger.com/28531. See the back of the book for more information.

Whiteboard or Flip Chart and Clipboards: During most sessions, the facilitator will write information on a whiteboard or flip chart. Additionally, clients will need a surface to write on, for which clipboards may suffice.

Check-In: Intensive Outpatient Model

Each of the ten IOP sessions begins with a two-tiered check-in. This part of the session may last anywhere from fifteen to forty-five minutes (or more), depending on the issues presented by group members.

During the first tier, the facilitator will begin by engaging each group member in a brief dialogue (name, sobriety date, something the group member did mindfully outside of the session). If left to take the floor during the initial moments of group, certain members may "take over," resulting in the other members of the group disengaging (becoming distracted, disinterested, drowsy, and so on) from the very beginning. In our own experiences with facilitating groups, we have found that engaging group members during initial moments of group is essential in obtaining and sustaining their attention for the duration of the three hours.

During the second tier, the facilitator will open up the floor to general issues that may need to be processed or shared with the group (high-risk situations, goals, successes, and so on). If relevant and useful material is presented during the second tier of the check-in, the facilitator can help the group members process each issue through modeling empathetic listening, presenting supportive feedback (with permission), and linking and summarizing material that is presented. However, if the issues are irrelevant or inappropriate (war stories, glamorizing, irrelevant storytelling, and so on), the facilitator must be able to redirect and move the group along in a polite, prompt, and assertive manner. We have found that if members become frustrated or disinterested early on, it is a challenge to reengage them as the session progresses.

Three-Hour IOP Session Outline

As presented in this manual, there are ten IOP sessions, each lasting for three hours. Each session is divided into three segments (each approximately 45 minutes to an hour in duration, although this will vary).

PART 1:

- Staff and client introductions and check-in (two-tiered check-in) (15 to 45 minutes)
    - Initial check-in: Name, sobriety date, and something the client did mindfully outside of group)
    - Follow-up questions:
        - "Any risky situations? How did you cope?"
        - "Any successes or goals met?"
- Introduction to mindfulness (10 to 15 minutes) (see appendix A)

- Brief mindfulness experience
- Open discussion of experiences
- Questions or comments

- Brief review of MBS model (5 to 10 minutes) (see appendix B)
- Introduction to topic of the day (10 to 15 minutes)

(*Break, 10 minutes*)

PART 2:

- Topic of the day (45 to 60 minutes)

(*Break, 10 minutes*)

PART 3:

- Topic of the day, continued (20 to 30 minutes)
- Debriefing and review (10 to 20 minutes)
  - Summary statements by clinician group leader
  - Elicitation of feedback from clients on today's group
    - "What are you walking away with?"
    - General feedback: "What made sense? What didn't make sense or was not so useful? What do you need in future groups?" (The purpose is to engage clients and elicit specific feedback for tailoring MBS to their specific needs. Additionally, the clinician may tie clients' needs self-assessments to relapse prevention and future topics.)
    - Values and goals: "What steps have you taken toward your values and goals in recent days or weeks?"
    - Random question (favorite movies, books, music, and so on)
  - Reminder to practice MBS skills (practice between sessions)
- Brief closing mindfulness experience (10 to 15 minutes) (see appendix A)
- Clinician announces topic for next group

IOP GROUP TOPIC ROTATION:

1. Exploring values
2. Exploring and setting goals
3. Defusing from the addiction
4. Value-based avoidance
5. Building recovery skills, part 1
6. Building recovery skills, part 2

7. Motivation
8. “Re-mindfulness”
9. Quality of life: Value-based living
10. Relationships

The following pages are organized by session topics. They contain session descriptions, session outlines, facilitator’s guides, and client handouts.

Session 1: Exploring Values

SESSION OBJECTIVES

Group members will:

1. Identify personal values and things in their lives that are important to them.
2. Learn the relationship and distinction between values and goals.
3. Practice and enhance mindfulness skills.
4. Further their understanding of comprehensive sobriety planning through the MBS model.

SESSION OUTLINE

1. Staff and client introductions and check-in (15 to 45 minutes)
2. Introduction to mindfulness (10 to 15 minutes)
3. Brief review of MBS model (5 to 10 minutes)
4. Topic introduction: “Important person” exercise (10 to 15 minutes)
   Break (10 minutes)
5. Introduction to values and goals (20 to 30 minutes)
6. Experiential values exercises (20 to 30 minutes)
   Break (10 minutes)
7. “What’s Important” worksheet (20 to 30 minutes)
8. Debriefing/review (10 to 20 minutes)
9. Brief closing mindfulness experience (10 to 15 minutes)
10. Topic for next group

SESSION MATERIALS

Session materials, including facilitator guides and client handouts, can be found at the end of the “Session Description.”

FACILITATOR GUIDES:

- Script for Experiential Exercise 1

- Script for Experiential Exercise 2
- Value-Based Living: Ideas to Keep in Mind (Facilitator Copy)

CLIENT HANDOUTS:

- Value-Based Living: Ideas to Keep in Mind
- Valued Experiences
- What's Important Worksheet

SESSION DESCRIPTION

1. Staff and client introductions and check-in (two-tiered check-in) (15 to 45 minutes). The two-tiered check-in is designed to allow each person in the group to speak and offer input prior to opening up the floor to larger issues that may need to be processed (such as high-risk situations, goals, successes, and so on).

The facilitator begins the session by asking all group members to answer the first tier of questions. Once all members have answered the first tier of questions, the facilitator encourages members to address the second tier. This latter tier may or may not involve the participation of the entire group.

INITIAL CHECK-IN:

- First Tier:
  - Name, sobriety date, and something clients did mindfully outside of group
- Second Tier:
  - "Any risky situations? How did you cope?"
  - "Any successes or goals met?"

2. Introduction to mindfulness (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

- Brief mindfulness experience
- Open discussion of experiences
- Questions and comments

3. Brief review of MBS model (5 to 10 minutes). The facilitator provides a brief review of comprehensive sobriety planning by referring to the MBS model. For an overview of the MBS model, the facilitator should refer to appendix B.

Experienced group members may assist in this review process.

4. Topic introduction: "Important person" exercise (10 to 15 minutes). This exercise is designed to help group members to make contact with their personal values through recalling people who are or have been important or influential in their lives. The facilitator should begin the exercise by instructing group members to think of someone who is important to them (dead, alive, friend, relative, stranger, and so on). Then she should ask group members to think about what they would want that person to say about them on being asked, "What type of person is (name of group member)?"

The group members are then instructed to go around the circle, sharing their answers out loud. The first person will say her answer, and the person to the left will repeat what she said and then give her own answer. (This procedure will continue as the group members share.) This encourages group members to listen mindfully to what others are saying and helps them stay engaged in the exercise. The facilitator has the option of participating in the exercise, as a way of establishing rapport and demonstrating collaborative spirit. As the group members repeat what

the person before them said, the facilitator identifies each group member's values and writes them on the board. (An example might be a person saying that her child is important to her and that she wants her child to say that she is a caring parent; the value might be "caring parent").

Once everyone in the group has had a turn to speak, the facilitator engages the group in a dialogue about how the exercise and group members' identified values relate to their recovery. The facilitator then speaks about the potential of making valued decisions every moment of every day. For instance, when she is struggling or experiencing moments of weakness, a group member can ask herself, *If I were a caring parent, what actions would I take in this moment?* It is important to note that in moments of struggle and weakness, the person may fall short of full adherence to her personal values and goals. The facilitator should emphasize that this does not make her a "bad" or a "weak" person, and she should not be met with criticism or judgment. This exercise is simply designed to help group members connect with personal values, which they can utilize to cope with risky situations and difficult moments.

(Break, 10 minutes)

5. Introduction to values and goals (20 to 30 minutes). The introduction to values and goals has two exercises: "Living vs. Existing" and "Defining Values and Goals / Qualities of Values and Goals."

Exercise 1: Living vs. Existing

The facilitator writes the following quote on the board: "If valuing has been put aside to avoid hurt, a much greater hurt is created through not living your life" (Luoma et al., 2007, p. 137).

The facilitator then processes the meaning and message behind the quote with the group members. From an ACT standpoint, human suffering, although universal and more common than most individuals would like to admit, is amplified due to experiential avoidance and remoteness from core values. An example would be individuals who value companionship and intimacy but do not pursue relationships in order to avoid loss, possible rejection, and hurt. The more a person avoids what she truly values in life, the more she suffers. Taken to an extreme, one may experience "just existing" rather than actually "living." A question to pose to the group is, "Are you willing to feel emotions (even ones you may experience as 'negative' or 'uncomfortable') if it means that by doing so, you can live a more fulfilling life?" The skill to be learned is that of being able to make contact with one's values and goals during moments of weakness or suffering.

Exercise 2: Defining Values and Goals/Qualities of Values and Goals

For this exercise, the facilitator refers to the "Value-Based Living: Ideas to Keep in Mind (Facilitator Copy)" resource. The facilitator writes each of the "Important Points" on the board and reads each explanation out loud. The facilitator then repeats the same process with the "Qualities of Effective Values." Using the client handout "Value-Based Living: Ideas to Keep in Mind," group members are asked to take notes on the "Important Points" and "Qualities of Effective Values." Note taking can be optional for those who have attended the session previously or for those who have difficulty reading or writing. The facilitator then assesses group members' comprehension.

Additionally, the facilitator asks group members to provide personal examples or responses based on their experiences with values and goals. As an example, a goal of completing school (destination) may require steps (valued actions) that include getting enough sleep, using self-care, attending class, completing homework, and studying daily.

6. Experiential values exercises (20 to 30 minutes). Prior to beginning the guided experiential exercises, the facilitator passes out the "Valued Experiences" worksheets. The facilitator instructs group members to place the worksheets underneath their chairs without reading the questions and filling out the answers.

Next, the facilitator guides the members in the first experiential exercise (using the "Script for Experiential Exercise 1").

Note: For some group members (such as those with unresolved trauma histories), the following exercise may be intense or uncomfortable. Accordingly, when giving the instructions, the facilitator informs the group: "If this exercise becomes too intense, you may choose to just fill out the questions on the worksheet without participating in

the visualizations."

After the first exercise has been completed, the facilitator instructs the members to fill out the first set of questions on the "Valued Experiences" worksheet. Once group members complete the first set of questions (no more than five to ten minutes should be allowed), the facilitator instructs them to place the worksheets back underneath their chairs. The facilitator repeats the same process for the second set of questions. After both exercises are completed, the facilitator leads the group in a discussion about their answers and experiences during the exercises. The discussion should include both the intellectual and emotional (feelings, emotions, sensations) experiences of the group members.

The experiential exercises are designed to encourage group members to make deeper contact with their core values. What we have found is that many individuals enter treatment after years of abusing substances and engaging in impulsive and unhealthy behaviors. This has created a divide between their core values and their actions. Through making contact with what is truly meaningful to them, group members can hopefully begin to make value-based behavior decisions, instead of choices based on impulse or a need for instant relief. The facilitator should be prepared to process the exercises with the group members, because some may have strong emotional reactions.

(Break, 10 minutes)

7. "What's Important" worksheet (20 to 30 minutes). The facilitator should begin this section of the session by passing out the "What's Important" worksheet. The group members are then instructed to complete the questions either individually or in small groups, whichever the facilitator prefers. This is followed by a discussion with the larger group.

8. Debriefing/review (10 to 20 minutes). The debriefing/review section of the session serves multiple purposes:

- To summarize the session process, content, and experience in ways that help group members remember and assimilate important information from the session
- To address any pressing unfinished business
- To elicit feedback from group members regarding the quality of the group, including what would be helpful in future sessions
- To provide focus on what's next: skill practice prior to the next session

OUTLINE OF DEBRIEFING AND REVIEW:

- Summary statements by the clinician–group leader.
- The facilitator writes the following questions on the board and elicits feedback from group members on today's group.
    - "What are you walking away with?"
    - General feedback: "What made sense? What didn't make sense or was not so useful? What do you need in future groups?" (The purpose is to engage clients and elicit specific feedback so as to tailor MBS to their specific needs. Additionally, the clinician may tie clients' needs self-assessments to relapse prevention and future topics.)
    - Values and goals: "What steps have you taken toward your values and goals in recent days or weeks?"
    - Random question (favorite movies, books, music, and so on).
- Reminder to practice MBS skills (practice mindfulness and other skills between sessions).

9. Brief closing mindfulness experience (10 to 15 minutes). The facilitator should refer to appendix A for instructions.

10. Topic for next group. The facilitator will inform group members of the topic for the next session: "Exploring

and Setting Goals": "In the next session, we will identify and further explore values, goals, and things in our lives that are important to us, and we will link our values (what's important) and goals."

Script for Experiential Exercise 1

Close your eyes or gently gaze at the floor.

Take a few normal breaths and sit with what is present (feelings, thoughts, emotions, sensations, pain, and so on) for a moment.Your mind may wander, which is totally normal; just observe your thoughts as if they were text moving across a television or computer screen. … Let them pass and fall away. (Wait thirty seconds to a minute.)

Now, as you feel comfortable, I want you to consider a moving or powerful moment in your life, one that you consider inspiring, life changing, and emotional. Regardless of the type of experience you had (whether it was of happiness, sadness, anger, triumph, or fulfillment). Again, it doesn't matter if the judging part of your brain deems it "negative" or "bad." Try to revisit in your mind's eye as many details as possible (who was there, what was said, the feelings you felt). I'm going to let you sit with that for a moment before I continue. (Wait one to two minutes.)

See if you can focus on some of the emotions you were feeling that day, both outside and inside of you. Continue to notice things that pop up. Distracting thoughts? Difficult emotions? A desire not to think about what I am asking you to think about? You may not feel anything at all. This is totally normal. There is no wrong way to do this exercise. Just observe whatever is there and gently bring your focus back to that event or moment. (Wait one to two minutes.)

Now slowly return to the room. Continue to breathe. Maybe stretch a bit and ground yourself as you bring yourself back to the group.

Script for Experiential Exercise 2

Close your eyes or gently gaze at the floor.

Take a few normal breaths and sit with what is present (feelings, thoughts, emotions, sensations, pain, and so on) for a moment.Your mind may wander, which is totally normal; just observe your thoughts as if they were text moving across a television or computer screen. (Wait thirty seconds to a minute.)

This time I want you to think about a person you look up to or respect. Whether that person is a loved one, friend, ex–significant other—dead or alive—doesn't matter. Just choose someone who means a lot to you. (Wait thirty seconds.)

I now want you to picture yourself writing a letter or e-mail to that person five years from now, updating him or her on how you are doing, including your current whereabouts, job, relationships, schooling, triumphs, successes, struggles, and so on. Your mind may wander or become distracted; remember to gently return it to the exercise—the letter you are writing and whom you are writing it to. Consider what you would want to tell this person about yourself. I'm going to let you sit with that for a moment before I continue. (Wait one to two minutes.)

Now, for a moment I want you to picture this person reading the letter or e-mail. What does this person look like? What facial expression is this person wearing? What is the person feeling inside as he or she reads your letter? Again, your mind may wander or become distracted; remember to gently return it to the exercise and the image of the person reading your letter. (Wait thirty seconds to one minute.)

Now slowly return to the room. Continue to breathe. Maybe stretch a bit and ground yourself as you bring yourself back to the group.

Value-Based Living: Ideas to Keep in Mind

Important Points

1. Goals are the *destination.* Values help you choose and stay on the *path*.
2. Goals will not be accomplished every day of someone's life. However, valued decisions can be made every moment of every day. Even small decisions can enhance or impede someone's progress toward larger goals.
3. Values can be carried with you. Goals can be completed, accomplished, or finished.
4. This point is an extension of point 1. Again, you can always head in a valued direction, whether or not you ultimately reach your destination, that is, whether or not you achieve your goal. It is important to note and recognize that some goals may never be reached or accomplished, but one can always work toward them.
5. Values require *commitment* and *action*.
6. Living a value-based life can be difficult at times and requires us to feel emotions that are often labeled as "negative" or "uncomfortable." This includes doing things even when we don't feel like it. Therefore, if someone attempts to live a value-based life, it requires a deeper commitment, because she may encounter moments of struggle and doubt. Again, pose the question, *Am I willing to feel my emotions and continue moving forward, if it means I can live a more fulfilling life?*
7. Actions are based on *values* rather than on *feelings* or *emotions*.
8. Both inside and outside of treatment, from time to time you may encounter individuals who make statements such as "I just want to be happy." Although this is understandable and a normal human desire, one cannot be happy or content all the time. Life requires us to struggle and suffer at times, which is the idea behind this point. We cannot live a valued life without taking action. In contrast, the alternative of valuing only feeling states may result in an empty chase for feeling good most, if not all, the time.
9. Research has demonstrated that *self-chosen* life directions are the ones that work; they result in greater *life fulfillment*.
10. This point is to emphasize the importance of group members establishing their own values and goals, as opposed to taking on the values and goals of outside parties.

Value-Based Living: Ideas to Keep in Mind (cont'd)

Qualities of Effective Values

Vitality

When someone is able to stay in contact with her values and goals, she tends to feel more alive. (Examples can be elicited and given by the facilitator.) An example that we often hear in our groups is when parents talk about how it feels to play with their children and spend quality time with loved ones.

Choice

Living in a value-consistent way allows a person to be flexible in responding to situations encountered on a daily basis, instead of just reacting in ways that are automatic, habitual, and impulsive. An example of reacting is when someone habitually expresses anger with verbal and physical aggression, or when someone reacts to substance cravings by "compulsive" use.

Present Focused

Present focus has two components. One is in regard to the destination the person is pursuing, while another has to do with the present in its own right. In other words, while values inform the path one takes, they also inform how one is in the here and now. For instance, one's values may guide a person on being fair and compassionate to others whom she encounters in the here and now (not just in the future).

Willing Vulnerability

Again, living a value-based life may require individuals to be vulnerable from time to time. If a person is unwilling to feel her emotions, particularly the experience of vulnerability, she may continue to engage in behaviors that are avoidant and impulsive.

Value-Based Living: Ideas to Keep in Mind (cont'd)

Take Note…

1. Goals are the _____. Values help you choose and stay on the _____.
2. Values can be _____.
3. Goals can be _____, _____, or _____.
4. Values require _____ and _____.
5. Actions are based on _____ rather than on _____.
6. Research has demonstrated that _____ life directions are the ones that work; they result in greater _____.

Qualities of Effective Values:

_____
_____
_____
_____
_____
_____
_____
_____
_____
_____

Valued Experiences

1. What was the moving moment in your life? Briefly describe it:

   _____
   _____
   _____
   _____

   When you thought of that moment, what did it feel like inside? What emotions came up? What did your body feel like (your arms, stomach, shoulders, and so on)? What thoughts came up?

   _____
   _____
   _____
   _____

   If someone saw you in that moment, how would the person describe you (facial expressions, body language)?

_____
_____
_____
_____

What are some things you noticed while participating in this exercise? For example, did you become distracted, have judgments, feel bored, have difficult feelings, and so on?

_____
_____
_____
_____

2. To whom did you write the letter or e-mail? Describe this person and why he or she is important to you:

_____
_____
_____
_____
_____
_____

If you were to write this person a letter or e-mail as if it were the future, what would it say? It may not be the life you are leading today; it may be what you are working toward. What are some of the things you would touch on in this letter?

_____
_____
_____
_____
_____
_____

What are some things you noticed while participating in this exercise? For example, did you become distracted, have judgments, feel bored, have difficult feelings, and so on?

_____
_____
_____
_____
_____
_____

What's Important Worksheet

What's important or personally meaningful to me?

_____
_____
_____
_____

Activities I am currently engaged in that are consistent with what's important and meaningful:

_____
_____
_____

_____

Activities I am currently engaged in that are inconsistent with what's important and meaningful:

_____
_____
_____
_____

Activities I might engage in that would be consistent with what's important and meaningful:

_____
_____
_____
_____

Session 2: Exploring and Setting Goals

SESSION OBJECTIVES

Group members will:

1. Identify personal values, goals, and things in their lives that are important to them.
2. Link their values and goals.
3. Practice and enhance mindfulness skills.
4. Further their understanding of comprehensive sobriety planning through the MBS model.

SESSION OUTLINE

1. Staff and client introductions and check-in (15 to 45 minutes)
2. Introduction to mindfulness (10 to 15 minutes)
3. Brief review of the MBS model (5 to 10 minutes)
4. "Important People, Places, and Things" exercise (10 to 15 minutes)
   (Break, 10 minutes)
5. Introduction to values and goals (5 to 10 minutes)
6. a. Establishing values and goals (15 to 20 minutes)
   (Break, 10 minutes)
   b. Establishing values and goals, continued (20 to 30 minutes)
7. Debriefing/review (10 to 20 minutes)
8. Brief closing mindfulness experience (10 to 15 minutes)
9. Topic for next group

SESSION MATERIALS

Session materials, including facilitator guides and client handouts, can be found at the end of the "Session Description."

FACILITATOR GUIDE:

- Linking Values and Goals (Facilitator Copy)

CLIENT HANDOUTS:

- Linking Values and Goals
- Examples of Values
- Establishing Values and Goals

SESSION DESCRIPTION

1. Staff and client introductions and check-in (two-tiered check-in) (15 to 45 minutes). The two-tiered check-in is designed to allow each person in the group to speak and offer input prior to opening up the floor to larger issues that may need to be processed (such as high-risk situations, goals, successes, and so on).

The facilitator begins the session by asking all group members to answer the first tier of questions. Once all members have done so, the facilitator encourages members to address the second tier. This latter tier may or may not involve the participation of the entire group.

INITIAL CHECK-IN:

- First Tier:
  - Name, sobriety date, and something clients did mindfully outside of group
- Second Tier:
  - "Any risky situations? How did you cope?"
  - "Any successes or goals met?"

2. Introduction to mindfulness (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

- Brief mindfulness experience
- Open discussion of experiences
- Questions and comments

3. Brief review of MBS model (5 to 10 minutes). The facilitator provides a brief review of comprehensive sobriety planning by referring to the MBS model. For an overview of the MBS model, the facilitator should refer to appendix B.

Experienced group members may assist in this review process.

4. Important people, places, and things exercise (10 to 15 minutes). This exercise is similar to the values exercise that occurs at the end of the first hour in session 1. It is designed to help the group members recall three things that are important in their lives while also making contact with their values. The facilitator begins the exercise by instructing the group members to think of three things that are important to them (a person, a place, and a miscellaneous thing). The group members are then instructed to go around the circle sharing their answers out loud. The first person will say her answer, and the person to her left will repeat what she said and then give her own answers (this procedure will continue as the clients share). This encourages the clients to listen mindfully to what other group members are saying, and keeps them engaged in the exercise. The facilitator has the option to participate in this exercise as a way of establishing rapport and demonstrating a collaborative spirit. As the group members repeat what the person before them said, the facilitator can pick out values or related goals and write them on the board (examples would be learning, the clients' children, sobriety, a religious text, clients' guitars, and their jobs).

Once everyone in the group has had a turn to speak, the facilitator can engage the group members in a dialogue about how the exercise and how values and goals relate to recovery and life in general. The facilitator can then speak to the group about making valued decisions every moment of every day in their lives. In continuing the previous example, the group member could ask herself in a moment of struggle or weakness (especially with alcohol or drug cravings), *If I were a caring parent, what actions would I take in this moment?* It is important to note that if someone continues to use substances, it does not make her a bad or a weak person. This exercise is simply designed to teach a skill that someone could use in order to connect with her values in a risky situation or difficult moment.

(Break, 10 minutes)

5. Introduction to values and goals (5 to 10 minutes). The introduction to values and goals has two exercises: "Living vs. Existing" and "Linking Values and Goals."

Exercise 1: Living vs. Existing

The facilitator writes the following quote on the board: "If valuing has been put aside to avoid hurt, a much greater hurt is created through not living your life" (Luoma et al., 2007, p. 137).

Living vs. Existing: While part of this session may function as a review of the previous session, it can also be presented as a stand-alone session. If time allows, revisiting the above quote may be important in order to emphasize living life in service of values and goals. The facilitator should write the quote on the board and process the meaning and message behind it with the group members. From an ACT standpoint, human suffering, although universal and more common than most individuals would like to admit, is amplified due to experiential avoidance and remoteness from core values. An example of this would be those individuals who value companionship and intimacy, but do not pursue relationships in order to avoid loss, possible rejection, and hurt. The more a person avoids what she truly values in life, the more she suffers and is just "existing" rather than actually "living." Another question to pose to the group is, "Are you willing to feel your emotions (even the ones that humans typically label as "negative" or "uncomfortable") if it means that you can live a more fulfilling life? The skill to learn would be to be able to ask yourself during moments of weakness or suffering, 'If I were my ideal self, how would I act in this moment?'"

Exercise 2: Linking Values and Goals

For this exercise, the facilitator refers to the "Linking Values and Goals (Facilitator Copy)" resource. The facilitator writes each of the "Important Points" on the board and reads each explanation out loud. The facilitator then repeats the same process with the "Qualities of Effective Goals." Using the client handout "Linking Values and Goals," group members are asked to take notes on the "Important Points" and "Qualities of Effective Goals." Note taking can be optional for clients who have attended the session previously or for those who have difficulty reading or writing. The facilitator then assesses group members' comprehension. Additionally, the facilitator asks group members to provide personal examples or responses based on their experiences with values and goals.

6a. Establishing values and goals (15 to 20 minutes). This activity is designed to help group members sharpen their focus and link their values with their goals. The facilitator begins the activity by passing out two client handouts: "Examples of Values" and "Establishing Values and Goals." The facilitator instructs the group to take the exercise a step at a time in order to avoid confusion. Next, the facilitator instructs the group members, using the "Establishing Values and Goals" handout, to circle three values domains that they would like to work on in their lives outside of group.

Next, group members should write their chosen domains on the blank "Domain" lines below the box. After they have completed this step in the process, the facilitator should instruct clients to look over the "Examples of Values" worksheet in order to pick out values that match their chosen domains. Group members should then write the values on the "Value" line, below the "Domain" lines on the "Establishing Values and Goals" handout. There may be values that work for multiple domains, or members may choose to list values that are not listed on the "Examples of Values" handout, both of which should be allowed. Finally, group members are then instructed to develop goals related to their chosen domains and values that embody the goal qualities discussed earlier in the session (specific and measurable, practical and accomplishable, and something they can commit to).

(Break, 10 minutes)

6b. Establishing values and goals, continued (20 to 30 minutes). Once group members are finished with the worksheet, the facilitator can have them share their answers in small groups. As part of the small groups, members can provide constructive feedback or affirmations related to their answers (asking questions such as, "Could you make that goal a bit more specific?"). The facilitator then leads a full group discussion, during which each member can share one answer (domain, values, and related goals).

7. Debriefing/review (10 to 20 minutes). The debriefing and review section of the session serves multiple purposes:

- To summarize the session process, content, and experience in ways that help group members remember and assimilate important information from the session
- To address any pressing unfinished business
- To elicit feedback from group members regarding the quality of the group, including what would be helpful in future sessions
- To provide focus on what's next: skill practice prior to the next session

OUTLINE OF DEBRIEFING AND REVIEW:

- Summary statements by clinician–group leader.
- The facilitator writes the following questions on the board and elicits feedback from group members on today's group.
    - "What are you walking away with?"
    - General feedback: "What made sense? What didn't make sense or was not so useful? What do you need in future groups?" (The purpose is to engage clients and elicit specific feedback so as to tailor MBS to their specific needs. Additionally, the clinician may tie clients' needs self-assessments to relapse prevention and future topics.)
    - Values and goals: "What steps have you taken toward your values and goals in recent days or weeks?"
    - Random question (favorite movie, book, music, and so on).
- Reminder to practice MBS skills (practice mindfulness and other skills between sessions).

8. Brief closing mindfulness experience (10 to 15 minutes). The facilitator should refer to appendix A for instructions.

9. Topic for next group. The facilitator will inform group members of the topic for the next session: "Defusing from the Addiction": "In the next session, we will further our understanding of the valued-living process—that is, living our lives based on what's important to us—and we will learn about and practice defusion skills. 'Defusion from addiction' simply means learning to become less attached to it."

Linking Values and Goals

Important Points

1. Living a valued life and working toward your goals requires *flexibility*, which may require giving up behaviors that no longer work for you.
2. This point is an extension of the "choice" characteristic of values in session 1. Again, living within your values and goals allows you to choose how to respond to situations you encounter on a daily basis, instead of just reacting in ways that are automatic, habitual,

and impulsive. An example is when someone becomes angry and always verbally and physically fights, or when someone always gives in when experiencing a drug or alcohol craving.

3. It also requires you to *observe* and *respond* instead of just *reacting*.
4. This point is an extension of point 1.
5. You have a choice to approach life with *an open mind*.
6. This point further reinforces the qualities of flexibility and willing vulnerability in decision making. If a person is open to what life has to offer, she is better able to approach life in a flexible manner, without increased struggle and avoidance. However, if a person is not open to experiencing the unavoidable trials and tribulations that life may bring, she often bases her decisions on avoiding discomfort.

Qualities of Effective Goals

Specific and Measurable

Increased self-efficacy is an important factor in someone's ability to cope with high-risk situations and life in general. If a person only establishes goals that are broad and vague, she may never accomplish them, which may in turn negatively affect her confidence and self-worth. Establishing goals that are specific and measureable allows her to feel accomplished, which may help her to gain momentum ("I finished this goal. What's next?").

Practical and Accomplishable

We often encounter individuals who set unrealistic goals, which places them at risk for failure. Goals that are unrealistic at the outset can be broken down into smaller and more achievable steps, which may increase a person's chances for success. An example would be someone who wants a higher-paying job but does not have the degree to move forward in her field of choice. This person would need to then work toward researching schools that would allow her to obtain a higher education. Another common example is goals related to feeling states, such as "I just want to be happy."

Committed To

It is important to establish goals that you can commit to both privately (within yourself) and publicly (to friends, family, counselors, sponsors, and so on).

Linking Values and Goals

Take Note…

1. Living a valued life and working toward your goals requires _____, which may require giving up behaviors that no longer work for you.
2. It also requires you to _____ instead of just _____.
3. You have a choice to approach life with _____.

Qualities of Effective Goals:

_____

_____

_____
_____
_____
_____
_____
_____
_____
_____
_____

Examples of Values

Achievement

Adventure

Affection (Love and Caring)

Authenticity

Being productive

Challenges

Change and variety

Close relationships

Community

Competition

Cooperation

Creativity

Curiosity

Discipline

Effectiveness

Excitement

Family

Financial security

Freedom

Friends

Genuineness

Health

Helping others

Honesty

Humor/Wit

Independence

Intimacy

Involvement

Knowledge

Leadership

Loyalty

Peace

Personal growth

Pets

Physically active

Pleasure/Fun

Power and authority

Privacy

Recovery

Reputation

Security

Self-Respect

Sophistication

Spirituality

Stability

Establishing Values and Goals

Domains

Community

Education

Family

Friends

Giving Back

Health

Occupation/Work

Religion

Social

Spirituality

Wellness

1. Domain: _____
   Value: _____
   Goal: _____
   Goal: _____
2. Domain: _____
   Value: _____
   Goal: _____
   Goal: _____
3. Domain: _____
   Value: _____
   Goal: _____
   Goal: _____

Session 3: Defusing from the Addiction

SESSION OBJECTIVES

Group members will:

1. Further their understanding of (or be introduced to) the “Value-Based Living” process.
2. Learn and practice “defusion” skills.
3. Practice and enhance mindfulness skills.

4. Further their understanding of comprehensive sobriety planning through the MBS model.

SESSION OUTLINE

1. Staff and client introductions and check-in (15 to 45 minutes)
2. Introduction to mindfulness (10 to 15 minutes)
3. Brief review of MBS model (5 to 10 minutes)
4. Value-based living presentation (10 to 15 minutes)
   (Break, 10 minutes)
5. Defusion vs. fusion (10 to 20 minutes)
6. Defusing from the addiction (30 to 40 minutes)
   (Break, 10 minutes)
7. Once upon a relapse (20 to 30 minutes)
8. Debriefing/review (10 to 20 minutes)
9. Brief closing mindfulness experience (10 to 15 minutes)
10. Topic for next group

SESSION MATERIALS

Session materials, including facilitator guides and client handouts, can be found at the end of the "Session Description."

FACILITATOR GUIDES:

- Value-Based Living Presentation (see appendix C)

CLIENT HANDOUTS:

- Defusing from the Addiction
- Once upon a Relapse

SESSION DESCRIPTION

1. Staff and client introductions and check-in (two-tiered check-in) (15 to 45 minutes). The two-tiered check-in is designed to allow each person in the group to speak and offer input prior to opening up the floor to larger issues that may need to be processed (such as high-risk situations, goals, successes, and so on).

The facilitator begins the session by asking all group members to answer the first tier of questions. Once all members have done so, the facilitator encourages members to address the second tier. This latter tier may or may not involve the participation of the entire group.

INITIAL CHECK-IN:

- First Tier:
  - Name, sobriety date, and something clients did mindfully outside of group
- Second Tier:

- "Any risky situations? How did you cope?"
- "Any successes or goals met?"

2. Introduction to mindfulness (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

- Brief mindfulness experience
- Open discussion of experiences
- Questions and comments

3. Brief review of MBS model (5 to 10 minutes). The facilitator provides a brief review of comprehensive sobriety planning by referring to the MBS model. For an overview of the MBS model, the facilitator should refer to appendix B.

Experienced group members may assist in this review process.

4. Value-based living presentation (10 to 15 minutes). The facilitator should refer to appendix C for a script and instructions.

(Break, 10 minutes)

5. Defusion vs. fusion (10 to 20 minutes). In this section of the session, the facilitator provides group members with a description of the ACT concepts of "defusion" and "fusion." This description is followed by questions and answers, as well as open discussion on how "defusion" and "fusion" relate to group members' addictions and other life issues.

EXPLANATION AND DISCUSSION:

- From an ACT perspective, our internal experiences (thoughts, feelings, sensations, and so on) are not problems in and of themselves. As previously mentioned, many of the processes linked to our internal experiences helped us get to where we are as a society today. If we had not acted on instinct as early humans, we would not have survived as a species. With that said, what can be problems and increase our suffering are the ways in which we respond to our internal experiences as modern humans. Becoming "fused" with or overly attached to instinctual or automatic ways of thinking and reacting can limit our ability to act flexibly and to live within our values and goals.

- In distinguishing defusion from fusion, the facilitator can provide the following example to the group:

  A person fusing with the thought or belief *I'm broken* may say to herself, *No one will ever accept me. My life will never change.* These thoughts may also be linked to feelings of hopelessness and depression, which the person may also fuse with. In becoming fused with the self-statement and emotions, the person is much more likely to engage in behaviors that strengthen that belief, such as avoidance and isolation. This provides the person with relief temporarily, but in the long term, it can lead to increased suffering and hopelessness.

- Through "defusing" from internal experiences that are not in line with our values and goals, we can increase our flexibility in action, which can lead to increased fulfillment in the long term (as opposed to immediate and short-term relief). For example:

  A person can have the thought or idea *I am broken* and feel depressed, but still take effective and workable actions toward her values and goals (seeking new relationships, going back to school, applying for jobs, exercising, and so on). It is important to note that through defusing from internal experiences, one is not attempting to change the experience or dispute the content of thoughts. By operating from a mindful or defused perspective, the person is observing her internal experiences and responding by taking valued action, regardless of how she feels or what her mind is telling her.

6. Defusing from the addiction (30 to 40 minutes). The facilitator begins this exercise by passing out the "Defusing from the Addiction" worksheet. Group members are instructed to write their names in the space provided on the

worksheet. Then, group members are asked to draw pictures or write words that represent themselves in the box labeled “Self-Portrait.” For example, one group member drew herself surrounded by her friends and family, while another wrote characteristics that she liked about herself, like her sense of humor. The facilitator then asks each group member to give her addiction a name and write that name in the space provided. In the box labeled “Portrait of Your Addiction,” each group member should draw a picture or write words demonstrating how her addiction might look, if it were separate from her. For example, one group member named her addiction “Queen Cocaine” and drew a stick figure with a crown on its head and a crack pipe in its hands.

It is important to find a balance between humor and seriousness with this exercise, because many clients will dismiss it as “silly” or “childish.” The facilitator can respond with lightness and flexibility while encouraging clients to participate and open up to the exercise.

Next the facilitator writes the following prompts on a whiteboard:

- Places where time is spent
- Emotional health
- Physical health
- Decision-making ability
- Risks taken (healthy or unhealthy)
- Relationships
- Life satisfaction
- Values and goals
- Hobbies and interests

The facilitator then asks group members to write their responses to the whiteboard prompts on the “Defusing from the Addiction” worksheet; in the left column, group members answer the prompts from the perspective of themselves, while in the right column, they answer the prompts from the perspective of their addiction.

This exercise can help group members defuse from their addictions in a way that is both verbal and experiential while highlighting self-chosen values. When they are experiencing emotions, thoughts, or sensations related to using substances, group members can use this exercise or related defusion skills as a way to remain grounded and mindful, rather than act impulsively. For example, someone could say to herself, *Oh, there’s Queen Cocaine again, trying to take me off my path*, as opposed to fusing with whatever intense or uncomfortable internal experience is happening and attempting to rid herself of it, which could lead to relapsing.

Once the group members have filled out the worksheet, the facilitator can help them process the exercise by leading a group discussion. The facilitator should encourage group members to share their answers and reflect on the differences and similarities between themselves and their “addictions.” This discussion can be both emotionally charged and lighthearted, depending on the group and where they are in their recovery. The facilitator should emphasize that there are no wrong answers and encourage open discussion of feelings and responses.

(Break, 10 minutes)

7. Once upon a relapse (20 to 30 minutes). During this exercise, group members are encouraged to take their named addictions and write a potential future-relapse scenario in the form of a story or movie scene. The group facilitator should write the statements and questions below on a dry erase board:

- Title of the book or movie: _____.
- Determine the characters in the scene: _____.
- Who would play the characters? (This is optional and can be done for the sake of fun.)
- Location: _____.

- How would people feel as they were watching or reading it?
- Value-consistent ending to the story or movie versus value-inconsistent ending: _____.

After they have finished the exercise, the facilitator can help the group members process their answers and help them develop plans or actions to prevent the future relapse. This exercise is designed to be both serious and fun (they can choose which actors or actresses they want to play the characters in the scene). In order to wrap up the exercise, the facilitator can encourage the group members to defuse from future thoughts or fantasies of using substances by simply saying things such as "I have seen that movie before. Boring!" "Do I want to play out that story?" "I know how this is going to end! No thanks." Additionally, the facilitator can encourage group members to relate to substance-use thoughts or fantasies as if they were movie trailers that are attractive and engaging but don't translate into enjoyable and fulfilling movies. "Have you ever seen an appealing trailer for a movie and then found that the actual movie sucked?"

8. Debriefing/review (10 to 20 minutes). The debriefing and review section of the session serves multiple purposes:

- To summarize the session process, content, and experience in ways that help group members remember and assimilate important information from the session
- To address any pressing unfinished business
- To elicit feedback from group members regarding the quality of the group, including what would be helpful in future sessions
- To provide focus on what's next: skill practice prior to the next session

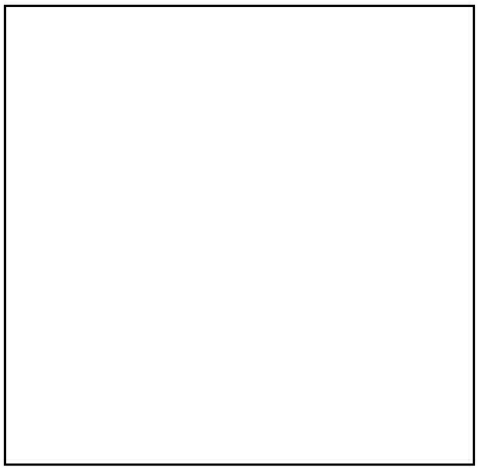
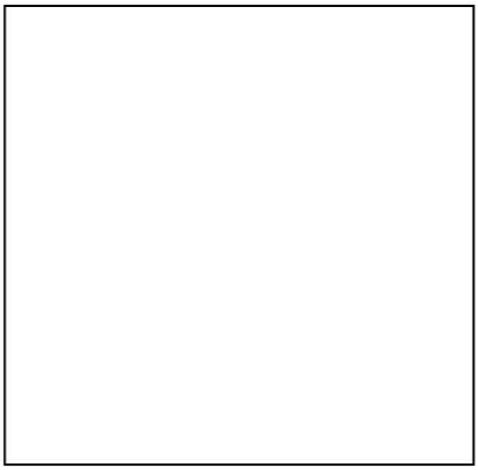
OUTLINE OF DEBRIEFING AND REVIEW:

- Summary statements by clinician–group leader.
- The facilitator writes the following questions on the board and elicits feedback from group members on today's group.
    - "What are you walking away with?"
    - General feedback: "What made sense? What didn't make sense or was not so useful? What do you need in future groups?" (The purpose is to engage clients and elicit specific feedback for tailoring MBS to their specific needs. Additionally, the clinician may tie clients' needs self-assessments to relapse prevention and future topics.)
    - Values and goals: "What steps have you taken toward your values and goals in recent days or weeks?"
    - Random question (favorite movie, book, music, and so on).
- Reminder to practice MBS skills (practice mindfulness and other skills between sessions).

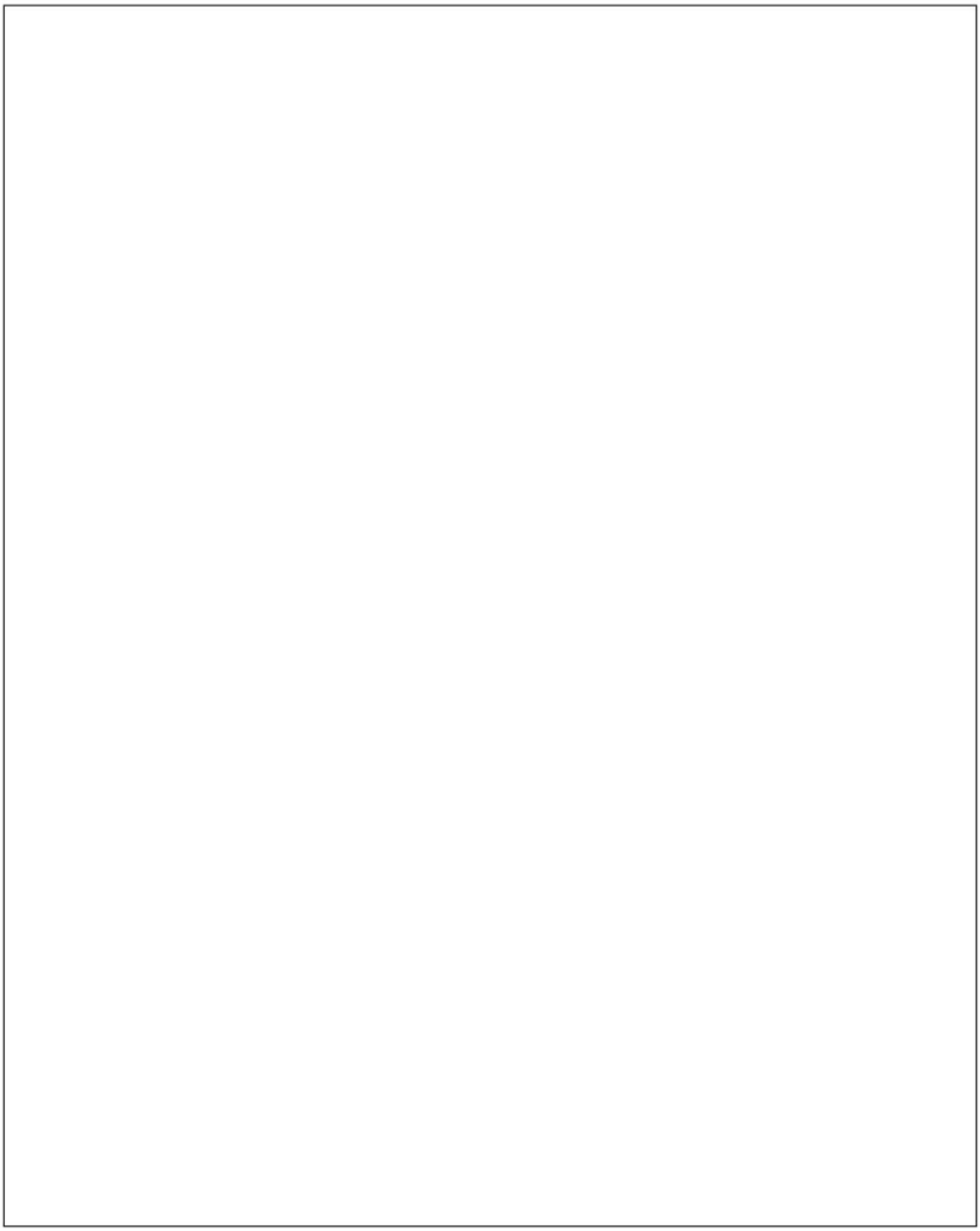
9. Brief closing mindfulness experience (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

10. Topic for next group. The facilitator will inform group members of the topic for the next session: "Value-Based Avoidance": "In the next session, we will further our understanding of (or be introduced to) the 'value-based living' process, that is, living our lives based on what's important to us. Also, we will learn about the concepts of 'Dirty Pain' and 'Clean Pain,' identify past and potential present and future environments and experiences related to substance use, and learn and discuss the concept of value-based avoidance."

Defusing from the Addiction

Your name:	Addiction name:
Self-Portrait 	Portrait of Your Addiction 

Once upon a Relapse…



Session 4: Value-Based Avoidance

SESSION OBJECTIVES

Group members will:

1. Further their understanding of (or be introduced to) the “value-based living” process
2. Learn the concepts “Dirty Pain” and “Clean Pain”

3. Identify past, present, and future (potential) environments and experiences related to substance use
4. Learn the concept “value-based avoidance”
5. Practice and enhance mindfulness skills
6. Further their understanding of comprehensive sobriety planning through the MBS model

SESSION OUTLINE

1. Staff and client introductions and check-in (15 to 45 minutes)
2. Introduction to mindfulness (10 to 15 minutes)
3. Brief review of the MBS model (5 to 10 minutes)
4. a. Value-based living presentation (15 to 20 minutes)
   (Break, 10 minutes)
   b. Value-based living, continued (15 to 20 minutes)
5. “Environments and Experiences” worksheet (30 to 35 minutes)
   (Break, 10 minutes)
6. Value-based avoidance (20 to 30 minutes)
7. Debriefing/review (10 to 20 minutes)
8. Brief closing mindfulness experience (10 to 15 minutes)
9. Topic for next group

SESSION MATERIALS

Session materials, including facilitator guides and client handouts, can be found at the end of the “Session Description.”

FACILITATOR GUIDE:

- Value-Based Living Presentation (see appendix C)

CLIENT HANDOUTS:

- Environments and Experiences Worksheet
- Situation Rating Scale and Action Plan

SESSION DESCRIPTION

1. Staff and client introductions and check-in (two-tiered check-in) (15 to 45 minutes). The two-tiered check-in is designed to allow each person in the group to speak and offer input prior to opening up the floor to larger issues that may need to be processed (such as high-risk situations, goals, successes, and so on).

The facilitator begins the session by asking all group members to answer the first tier of questions. Once they have done so, the facilitator encourages members to address the second tier. This latter tier may or may not involve the participation of the entire group.

INITIAL CHECK-IN:

- First Tier:
  - Name, sobriety date, and something clients did mindfully outside of group
- Second Tier:
  - “Any risky situations? How did you cope?”
  - “Any successes or goals met?”

2. Introduction to mindfulness (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

- Brief mindfulness experience
- Open discussion of experiences
- Questions and comments

3. Brief review of MBS model (5 to 10 minutes). The facilitator provides a brief review of comprehensive sobriety planning by referring to the MBS model. For an overview of the MBS model, the facilitator should refer to appendix B.

Experienced group members may assist in this review process.

4a. Value-based living presentation (15 to 20 minutes). The facilitator should refer to appendix C, “Value-Based Living Presentation,” for instructions.

(Break, 10 minutes)

4b. Value-based living, continued (15 to 20 minutes). After introducing the model (following the above guidelines), the facilitator can then introduce the concepts “Dirty Pain” and “Clean Pain.” Dirty pain can be defined as the extra pain or suffering that is created when one attempts to avoid experiencing discomfort. An example of this would be when an individual continues to use substances in order to avoid withdrawal. Although withdrawal will be uncomfortable and sometimes costly up front, long-term sobriety may mean that the person can live a more fulfilling life and suffer less. Another example is when one avoids social situations due to being anxious and insecure, despite her deeply held desire to have friends and find a significant other. Avoiding social situations can be comforting at first, but the pain created through isolation and avoidance can become far more uncomfortable than working through fear while engaging in social activities.

With that said, clean pain can be defined as the pain that we, as humans, experience as part of living life. Clean pain is normal and is a natural product of working toward values and goals. An example of clean pain is the anxiety we experience prior to asking someone out on a date or the sadness we experience when someone passes away or when we lose a friend. Another example includes the emotional and relationship issues many people have to address and face after achieving an initial period of sobriety. The facilitator can then speak to the group about opening up to clean pain and decreasing the amount of self-inflicted, dirty pain. This can be done through making two columns on a whiteboard and listing examples of both dirty and clean pain to further illustrate the concepts.

This is also an important time to introduce the idea of lapses and perseverance in recovery. A “lapse” is defined as a temporary slip or brief return to substance use after a period of sobriety. Although it does not happen in every case, lapses and relapses are common in recovery. In the case of a lapse after a period of sobriety, a person may experience intense feelings of guilt, shame, and remorse, which can then result in a full-blown relapse. Many individuals create more pain (dirty pain) for themselves by continuing to use substances instead of getting back on track in their efforts to maintain sobriety. If a person can learn to embrace and open up to the clean pain, she may be more likely to avoid a lapse or full-blown relapse.

Next, the facilitator can write the phrases “Short-term discomfort; Long-term fulfillment” and “Short-term relief; Long-term suffering” on the board. Once the group members have had a chance to read the phrases, the facilitator can help them identify decisions in their lives that were difficult initially but led to fulfilling results in the long run. An example would be a person who switches careers by going back to school after working in a certain field for many years. Another example would be when someone makes amends with those she has wronged by working through a 12-step program.

5. "Environments and Experiences" worksheet (30 to 35 minutes). After the previous discussion is concluded, the facilitator introduces the "Environments and Experiences" worksheet by reading the following paragraphs:

> *Life in general can be a risky situation for those attempting to make major lifestyle changes, such as entering recovery. Some stressful or risky situations may be easily navigated, while others may be more difficult and challenging to face. Similarly, some situations may be easily avoided, while others are not and must be faced directly regardless of how long someone has been sober.*
>
> *The next exercise will help you identify experiences and situations in your past, present, and future that are or could be linked with substance use. The exercise will also encourage you to rate each experience and situation as one of three options: "long-term avoidance" (LT), "short-term avoidance" (ST), or "no avoidance" (NA). Avoidance should be determined based on your values, goals, and quality of life, as opposed to being based on avoiding discomfort or seeking instant relief (as discussed in session 3).*

It is important to note that some risky situations may be valued and linked to one's long-term goals. Short-term avoidance of valued (but) risky situations may help a person achieve an initial period of sobriety, after which she may be better equipped to experience the situation, whatever it may be. An example would be a person in recovery who enjoys going to professional sporting events or concerts. These situations, although valued, may need to be avoided in the short term in the service of long-term sobriety. Although it's uncomfortable at first (clean pain, short-term discomfort), the person may be able to go to sporting events and concerts in the future while maintaining sobriety (long-term fulfillment).

Once group members have completed the "Environments and Experiences" worksheet, the facilitator helps them process their answers and discuss issues related to their avoidance ratings (LT, ST, or NA). When the discussion comes to a conclusion, the group facilitator encourages each group member to choose one of her answers that she would like to explore further. The chosen answer will be discussed and explored in the next exercise.

(Break, 10 minutes)

6. Value-based avoidance (20 to 30 minutes). The facilitator distributes and introduces the "Situation Rating Scale and Action Plan" worksheet. It is important that the facilitator guides the group members step-by-step through this exercise in order to avoid confusion and misunderstanding. Initially, the facilitator encourages group members to take the situations they chose from the previous worksheet and describe them on the "Risky Situation" section at the top of page 1 of the worksheet. Then, group members will complete the section titled "How do you expect you might respond in this type of situation?" Next, the facilitator asks group members to rate, on a scale from 0 to 10, how risky the situation is for them. Following this, group members rate the value (or importance) that the situation has for them, also on a scale from 0 to 10. Group members will then indicate a general plan—"Long-Term Avoidance," "Short-Term Avoidance," or "No Avoidance"—by checking the appropriate box:

- *Long-term avoidance* is typically indicated for situations that are high risk and hold little-to-no value.
- *Short-term avoidance* is typically indicated for situations that are high risk and hold moderate-to-high value.
- *No avoidance* (with coping plan) is typically indicated for situations that are high value, or for which the value rating outweighs the risk rating.

After completing the first part of the "Situation Rating Scale and Action Plan" worksheet, the group begins the second. Group members will complete either the first or the second box on the worksheet. Members who chose either "Long-Term Avoidance" or "Short-Term Avoidance" will complete the first box. This box has two parts: "Avoidance strategies" and "Backup coping strategy, if the situation is unexpectedly encountered." Members who chose "No Avoidance" will complete the second box, "Coping Strategies."

After both parts of the worksheet are completed, the facilitator splits the group members into pairs or small groups and has them share their answers with each other (as well as brainstorming, coping, and avoidance strategies). Once the small group discussions have concluded, the facilitator can open the floor for discussion prior to moving into the debriefing and review.

7. Debriefing/review (10 to 20 minutes). The debriefing/review section of the session serves multiple purposes:

- To summarize the session process, content, and experience in ways that help group members remember and assimilate important information from the session
- To address any pressing unfinished business
- To elicit feedback from group members regarding the quality of the group, including what would be helpful in future sessions
- To provide focus on what's next: skill practice prior to the next session

OUTLINE OF DEBRIEFING/REVIEW:

- Summary statements by clinician–group leader.
- The facilitator writes the following questions on the board and elicits feedback from group members on today's group.
    - "What are you walking away with?"
    - General feedback: "What made sense? What didn't make sense or was not so useful? What do you need in future groups?" (The purpose is to engage clients and elicit specific feedback for tailoring MBS to their specific needs. Additionally, the clinician may tie clients' needs self-assessments to relapse prevention and future topics.)
    - Values and goals: "What steps have you taken toward your values and goals in recent days or weeks?"
    - Random question (favorite movies, books, music, and so on).
- MBS skill-practice reminder (practice mindfulness and other skills between sessions).

8. Brief closing mindfulness experience (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

9. Topic for next group. The facilitator will inform group members of the topic for the next session: "Building Recovery Skills, Part 1": "In the next session, we will assess our personal quality of life and develop initial improvement plans, and we will assess recovery needs and develop (or improve) lapse and relapse prevention plans."

List specific people, places, things, events, activities, and so forth.

Environments and Experiences Worksheet

List specific people, places, things, events, activities, and so forth.

Past	Present (potential)	Future (potential)

Situation Rating Scale and Action Plan

Name: _____ Date: _____

Risky Situation (describe): _____

_____

_____

_____

_____

How do you expect you might respond in this type of situation? _____

_____

_____

_____

_____

Situation Risk Rating

1	2	3	4	5	6	7	8	9	10
Little or no risk									Very high risk

Situation Value Rating (How important is it for you to be in this type of situation?)

1	2	3	4	5	6	7	8	9	10
Little or no risk									Very high risk

Plan (check one):

_____ 1. Long-Term Avoidance (situation is high risk and of little-to-no value)

_____ 2. Short-Term Avoidance (situation is high risk and of moderate-to-high value)

_____ 3. No Avoidance (situation is low-to-moderate risk and of moderate-to-high value)

Avoidance Strategies (if you chose option 1 or 2 above):

_____
_____
_____
_____

Backup Coping Strategy (if the situation is unexpectedly encountered):

_____
_____
_____
_____

Coping Strategies (if you chose option 3 above):

_____
_____
_____
_____

Session 5: Building Recovery Skills, Part One

SESSION OBJECTIVES

Group members will:

1. Assess personal quality of life and develop initial improvement plans.
2. Assess sobriety needs and develop (or improve) lapse and relapse prevention plans.
3. Practice and enhance mindfulness skills.
4. Further their understanding of comprehensive sobriety planning through the MBS model.

SESSION OUTLINE

1. Staff and client introductions and check-in (15 to 45 minutes)
2. Introduction to mindfulness (10 to 15 minutes)
3. Brief review of MBS model (5 to 10 minutes)
4. a. Recovery skills self-ratings 1: “Quality of Life” section (10 to 15 minutes)
   (Break, 10 minutes)
   b. Recovery skills self-ratings 1: “Quality of Life” section, continued (20 to 30 minutes)
5. a. Recovery skills self-ratings 1: “Lapse and Relapse Prevention Planning” section (20 to 30 minutes)
   (Break, 10 minutes)

b. Recovery skills self-ratings 1: “Lapse and Relapse Prevention Planning” section, continued (20 to 30 minutes)

6. Debriefing/review (10 to 20 minutes)
7. Brief closing mindfulness experience (10 to 15 minutes)
8. Topic for next group

SESSION MATERIAL

Session materials, including facilitator guides and client handouts, can be found at the end of the “Session Description.”

Client Handout:

- Recovery Skills Self-Ratings 1

SESSION DESCRIPTION

1. Staff and client introductions and check-in (two-tiered check-in) (15 to 45 minutes). The two-tiered check-in is designed to allow each person in the group to speak and offer input prior to opening up the floor to larger issues that may need to be processed (such as high-risk situations, goals, successes, and so on).

The facilitator begins the session by asking all group members to answer the first tier of questions. Once they have done so, the facilitator encourages members to address the second tier. This latter tier may or may not involve the participation of the entire group.

INITIAL CHECK-IN:

- First Tier:
  - Name, sobriety date, and something clients did mindfully outside of group
- Second Tier:
  - “Any risky situations? How did you cope?”
  - “Any successes or goals met?”

2. Introduction to mindfulness (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

- Brief mindfulness experience
- Open discussion of experiences
- Questions and comments

3. Brief review of MBS model (5 to 10 minutes). The facilitator provides a brief review of comprehensive sobriety planning by referring to the MBS model. For an overview of the MBS model, the facilitator should refer to appendix B.

Experienced group members may assist in this review process.

4a. Recovery skills self-ratings 1: “Quality of Life” section (10 to 15 minutes). The main objective of this curriculum (especially in sessions 5 and 6) is to help the group members identify and gain skills that carry the potential to increase their quality of life and chances of staying sober in the long term. The facilitator introduces the “Recovery Skills Self-Ratings 1” worksheet to the group as a tool that can be used to highlight strengths and areas to improve, in relation to life and sobriety-maintenance skills. Then the facilitator has group members complete the worksheet, which will serve as the foundation for the remainder of the session.

Exercise for Recovery Skills Self-Ratings 1: The above exercise should include having the clients conduct two self-ratings: current and presobriety. This will allow them to compare and contrast now versus prerecovery, which may help to enhance self-efficacy. The facilitator then leads a discussion about the differences between their scores and the main areas in which they desire improvement.

The sections of the worksheet "Quality of Life" and "Lapse and Relapse Prevention Planning" should be approached at a pace that is comfortable for the group (with time devoted to each section lasting approximately one hour). When the group reviews the worksheet, it is best to collaborate with group members in choosing three or four items in each section to focus on. Items that are not focused on in this part of the session may be addressed during the last hour of group, if time allows. Outside referrals may be made for things such as dietary advice, fitness counseling, and financial consulting.

(Break, 10 minutes)

4b. Recovery skills self-ratings 1: "Quality of Life" section, continued (20 to 30 minutes). In discussing the worksheet, the facilitator should also pose a series of open-ended questions to the group based on the quality-of-life skills listed in the first section of the checklist. The questions can be in the following format:

- "How does _____ relate to your quality of life and motivation in recovery?"
- "What are some small steps you could take in the near future in order to improve this area of your life?"
- "How would you like to see your diet change, and how would you like to go about doing it?"

  Disclaimer: Facilitators should refrain from giving clients specific dietary advice, and should discourage clients from doing so, as well. Clients should be encouraged to seek the advice of a dietician, nutritionist, or physician for specific dietary recommendations.
- "What types of physical activities would you like to engage in, and how would you like to go about doing them?"
- "How would you like to see your self-care and hygiene improve, and how would you like to go about doing it?"

This discussion can be in open discussion or individualized written format, depending on the size or feel of the group. The facilitator can help group members identify areas of concern and collaborate in developing a healthy recovery plan.

5a. Recovery skills self-ratings 1: "Lapse and Relapse Prevention Planning" section (20 to 30 minutes). Next, the facilitator focuses the group on the "Lapse and Relapse Prevention Planning" section of the worksheet. As a means of introduction, the facilitator may choose to read the following paragraph to the group members:

> *A person who encounters a risky situation could be left feeling overwhelmed and functioning on autopilot. Having sufficient coping skills may improve the person's confidence and ability to act in a manner consistent with personal values and goals. Therefore, it is important for those in recovery to identify risky situations before they encounter them, and learn ways to avoid and cope with them in the future.*

After the introduction, the facilitator assesses group needs and bases the discussion accordingly. Depending on the needs of the group, open-ended questions to pose include:

- "How can you best identify risky situations before you encounter them?"
- "What are some ways in which you have or have not been able to avoid risky situations in recovery?"
- "What coping skills have you found to be effective in dealing with risky situations (situations that are unavoidable or encountered unexpectedly)?"

The facilitator then writes group members' responses on a whiteboard and encourages the group to take notes

on the back of the checklist or on a blank sheet of paper.

Additionally, if needed, the group can identify and role-play drug refusal skills. Drug refusal skills are the behaviors a person can exhibit in order to decline alcohol or drugs when they are offered to her or when they become available. Specific refusal skills may include words to say, body language, tone of voice, and specific actions.

Next, the facilitator has the option of introducing the topic of decision making in recovery. If this is done, the facilitator presents the following information:

- If they are living life on "autopilot," human beings may be more susceptible to developing a narrowed focus and making decisions based on instinct or emotion. Through learning to slow things down, take on new perspectives, and increase self-awareness, a person can make decisions mindfully, based on values and goals as opposed to impulse. (The facilitator can here encourage the group members to brainstorm ways of slowing their lives down, broadening their perspectives, and increasing self-awareness.)

- As indicated above, a person may operate on "autopilot," where she is unaware of the future consequences of her decisions. Decisions that seem innocent or irrelevant up front may produce negative consequences two or three moves ahead, which can place a person at risk for engaging in a behavior that is not in line with her values and goals.

- For example, a person in recovery may go to a party after a long and stressful week without the intention of using, but without an escape or coping plan, either. The person may feel drained and have a desire at an unconscious level to escape or unwind, as well as unconscious urges and cravings, which may become conscious once she is exposed to her drug of choice at the party. Prior to going to the party or after being exposed to her drug of choice, she may fuse with a rationalization or deny the seriousness of the situation (*I'll be fine! Just one won't hurt. I deserve it!*).

Additionally, the facilitator may speak to the group about the "snowball effect" and decision making. If decisions are made without insight or purpose, momentum can build toward relapse or a negative outcome, depending on the situation. The facilitator can elicit examples of decisions the group members have made in the past that led to a series of situations that rapidly escalated and resulted in negative consequences. Next, the facilitator can discuss ways of stopping the ongoing flow of events and moving the momentum toward stability and positive change.

(Break, 10 minutes)

5b. Recovery skills self-ratings 1: "Lapse and Relapse Prevention Planning" section, continued (20 to 30 minutes). In wrapping up the discussion on decision making, the facilitator may introduce group members to purposeful decision-making skills:

- *Playing out the tape:* This process includes thinking two or three moves ahead, considering the consequences beforehand. *What would happen if I did this? What would happen if I did that? It might feel good to use right now, but what are the other consequences? How will I feel a little later today or tomorrow morning?*

- *Compassionate self-talk:* This process includes words a person might say to herself to broaden her perspective or ground herself in her values such as *Slow things down. Be mindful. What do I really need right now? How could I deal with this situation in a way that is in line with how I want to be as a person?*

- Lastly, the facilitator may present to the group the "Lapse Recovery Coping Skills" item on the worksheet. As a way of introduction, the facilitator may read the following paragraph to the group:

  A lapse is defined as a temporary slip or brief return to substance use after a period of sobriety. Although common in recovery, lapses often result in the person feeling guilty, ashamed, or remorseful, and this can result in a full-blown relapse. If a person "gets back on track" sooner rather than later, she increases her chances of regaining and maintaining sobriety.

  The facilitator then asks group members to identify ways they might "get back on track" if they were to ever

experience a lapse. Examples include calling their counselors or sponsors, and returning to their groups or home meetings.

6. Debriefing/review (10 to 20 minutes). The debriefing/review section of the session serves multiple purposes:

- To summarize the session process, content, and experience in ways that help group members remember and assimilate important information from the session
- To address any pressing unfinished business
- To elicit feedback from group members regarding the quality of the group, including what would be helpful in future sessions
- To provide focus on what's next: skill practice prior to the next session

OUTLINE OF DEBRIEFING/REVIEW:

- Summary statements by clinician–group leader.
- The facilitator writes the following questions on the board and elicits feedback from group members on today's group.
    - "What are you walking away with?"
    - General feedback: "What made sense? What didn't make sense or was not so useful? What do you need in future groups?" (The purpose is to engage clients and elicit specific feedback for tailoring MBS to their specific needs. Additionally, the clinician may tie clients' needs self-assessments to relapse prevention and future topics.)
    - Values and goals: "What steps have you taken toward your values and goals in recent days or weeks?"
    - Random question (favorite movie, book, music, and so on).
- MBS skill-practice reminder (practice mindfulness and other skills between sessions).

7. Brief closing mindfulness experience (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

8. Topic for next group. The facilitator will inform group members of the topic for the next session: "Building Recovery Skills, Part Two": "In the next session, we will learn and practice 'urge surfing,' which is a method of coping with urges and cravings, and we will expand mindfulness coping strategies."

Recovery Skills Self-Ratings 1

Rate your coping skills on each item below on a scale of 1 through 5, with 1 being poor or nonexistent and 5 being excellent.

Scale: *1 = poor or nonexistent*

2 = needs improvement

3 = okay, average, normal

4 = good to very good

5 = excellent

DK = don't know

Quality of Life:

PRE POST

_____/_____ Ability to establish and maintain healthy diet

_____/_____ Ability to engage in physical activities or maintain a workout routine

_____/_____ Self-care: dental and other hygiene

_____/_____ Ability to balance finances

_____/_____ Ability to seek, obtain, or maintain employment

_____/_____ Ability to set, work toward, and achieve goals

_____/_____ Engage in and maintain valued relationships with others

Lapse and Relapse Prevention Planning:

PRE POST_____

_____/_____ Ability to identify risky (nonvalued) environments and situations

_____/_____ Ability to avoid risky (nonvalued) environments and situations

_____/_____ Ability to cope, take valued action, and maintain sobriety when in risky or intense situations

_____/_____ Alcohol or drug refusal skills

_____/_____ Identify decisions and behaviors that could lead to lapse/relapse (gradually or rapidly)

_____/_____ Lapse recovery coping skills

Session 6: Building Recovery Skills, Part Two

SESSION OBJECTIVES

Group members will:

1. Learn and practice "urge surfing" as a method of coping with urges and cravings.
2. Assess and expand mindfulness coping strategies.
3. Practice and enhance mindfulness skills.
4. Further their understanding of comprehensive sobriety planning through the MBS model.

SESSION OUTLINE

1. Staff and client introductions and check-in (15 to 45 minutes)
2. Introduction to mindfulness (10 to 15 minutes)
3. Brief review of MBS model (5 to 10 minutes)
4. Introduction to topic: Recovery skills self-ratings 2 (10 to 15 minutes)
   (Break, 10 minutes)
5. Introduction to urge surfing (10 to 15 minutes)
6. a. Recovery skills self-ratings discussion (30 to 45 minutes)
   (Break, 10 minutes)
   b. Recovery skills self-ratings, continued (20 to 30 minutes)
7. Debriefing/review (10 to 20 minutes)
8. Brief closing mindfulness experience (10 to 15 minutes)
9. Topic for next group

SESSION MATERIALS

Session materials, including facilitator guides and client handouts, can be found at the end of the "Session Description."

FACILITATOR GUIDES:

- Urge-Surfing Instructions (see appendix A)
- Mindfulness Instructions (see appendix A)

CLIENT HANDOUTS:

- Recovery Skills Self-Ratings 2
- Coping with Internal Experiences
- Urge Surfing: Take-Home Guide

SESSION DESCRIPTION

1. Staff and client introductions and check-in (two-tiered check-in) (15 to 45 minutes). The two-tiered check-in is designed to allow each person in the group to speak and offer input prior to opening up the floor to larger issues that may need to be processed (such as high-risk situations, goals, successes, and so on).

The facilitator begins the session by asking all group members to answer the first tier of questions. Once all members have done so, the facilitator encourages members to address the second tier. This latter tier may or may not involve the participation of the entire group.

INITIAL CHECK-IN:

- First Tier:
  - Name, sobriety date, and something clients did mindfully outside of group
- Second Tier:
  - "Any risky situations? How did you cope?"

- "Any successes or goals met?"

2. Introduction to mindfulness (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

- Brief mindfulness experience
- Open discussion of experiences
- Questions and comments

3. Brief review of MBS model (5 to 10 minutes). The facilitator provides a brief review of comprehensive sobriety planning by referring to the MBS model. For an overview of the MBS model, the facilitator should refer to appendix B.

Experienced group members may assist in this review process.

4. Introduction to topic: Recovery skills self-ratings 2 (10 to 15 minutes). At this point in the session, the facilitator introduces the "Recovery Skills Self-Ratings 2" worksheet. This tool can be used to identify strengths in coping skills and areas that are in need of improvement. Once this is introduced, the facilitator can have the group members complete the worksheet, which will serve as the foundation for the remainder of the session.

Optional Exercise for Recovery Skills Self-Ratings 2: An optional part of the above exercise might include clients conducting two self-ratings: current and presobriety. This would allow them to compare and contrast now versus prerecovery, and it may help to enhance self-efficacy.

(Break, 10 minutes)

5. Introduction to urge surfing (10 to 15 minutes). The sections of the worksheet ("Coping with Urges and Cravings" and "Expansion and Mindfulness Strategies") should be approached by the facilitator in order and should be taken at a pace that is comfortable to the group (with each lasting approximately one hour). The facilitator can begin by focusing on the "Coping with Urges and Cravings" section of the checklist. The facilitator should begin this section by inquiring about the emotional and physical experiences one goes through when experiencing a craving or an urge to act impulsively. This discussion can focus on both drug and alcohol cravings and other seemingly impulsive human behaviors (for example, acting out through anger, sex, eating, and so on). As a means of introduction, the facilitator may then choose to read the following to the group members:

- *Our internal experiences—emotions, feelings, sensations, thoughts, urges, cravings, and so on—can be compared to a wave that rises and falls.* (On a board or large piece of paper, the facilitator may draw a bell-shaped curve or a wave that peaks and falls.)
- *Once the internal experience begins to rise and peak, many people use strategies to cut off the wave—fighting, avoiding, distracting, giving in, and so on—which often work temporarily but tend to strengthen the wave in the long term.* (The facilitator may then draw a second wave that gets cut off before it peaks, but then shoots right back up shortly after being cut off.)
- *An example of this would be someone who uses her drug of choice when experiencing urges and cravings. The urge or craving may be cut off after the initial use of the substance but will inevitably return once the drug wears off or when the person encounters a risky situation. Many people get caught up in this cycle and often live their lives from distraction to distraction, in service of not feeling or experiencing.*
- *Urge surfing is a skill that helps you to slow things down and to breathe and relax into the urge or craving. Instead of giving in, a person eventually is able to learn that internal experiences such as emotions, urges, and cravings are temporary. They will come and go with little effort from the individual. Despite our desire to control internal experiences, efforts to do so often increase our suffering in the long term. What we actually have control over is how we observe and respond to internal experiences, not the experiences themselves.*

After gauging group members' comprehension, the facilitator distributes the "Coping with Internal Experiences" worksheet. The facilitator then asks for volunteers to read the worksheet out loud, after which there

may be questions and discussion.

6a. Recovery skills self-ratings discussion (30 to 45 minutes). Next, the facilitator leads the group in an urge surfing exercise. (Please refer to appendix A for instructions.)

As indicated in the script, group members are advised against using the most challenging scenario they can think of for this exercise. Instead, group members should choose a scenario that they feel comfortable with and will not be overwhelmed by. However, some may have no reaction at all, and that's okay; they should just treat this as a practice round. Urge surfing activities should not be practiced directly before a break or during the last hour of group due to the strong reaction it may elicit within certain individuals. The exercise itself should be followed by an extended period of open processing (especially if members report intense experiences) and a basic mindfulness meditation. (Please see appendix A for instructions.)

Again, the facilitator should also encourage group members to continue using coping skills that have worked for them in the past (such as keeping busy or distracting themselves), while also learning and practicing strategies, such as urge surfing, that may be more helpful long term.

(Break, 10 minutes)

6b. Recovery skills self-ratings, continued (20 to 30 minutes). The facilitator begins the last hour of group by passing out the "Urge-Surfing Instructions" worksheet (for the clients' own reference and use outside of group) and by assessing for any lingering questions or feelings to process.

Next, the facilitator refers back to the "Recovery Skills Self-Ratings 2" worksheet, focusing on the "Expansion and Mindful Strategies" section. When reviewing this section, it is best to collaborate with the group members in choosing two or three items to focus on. Items that are not focused on in this session may be addressed in individual counseling or in subsequent group sessions.

There are five items in the "Expansion and Mindfulness Strategies" section:

- Understanding and practicing mindfulness
- Using coping imagery
- Using defusion skills with internal experiences (thoughts, memories, sensations, emotions, and so on) and taking valued/flexible action
- Ability to feel emotions without trying to control, change, or fix them
- Reframing

If the group chooses to focus on the first item, "Understanding and practicing mindfulness," the facilitator can refer to appendix A of this book for information and instructions.

If the group chooses to focus on the second item, "Using coping imagery," the facilitator provides a definition: "Use of coping imagery involves picturing an image in your mind that represents strength and stability. You can use this image of strength during moments of weakness or struggle." The facilitator then elicits images of strength from each group member. If group members have difficulty generating images, the facilitator can provide examples: ocean waves (as in urge surfing), mountains, children, superheroes, trains or fast-moving vehicles, and real-life heroes. The facilitator then poses the question: "What would it be like to picture the images you chose when you are experiencing moments of weakness or struggle?"

If the group chooses to focus on the third item, "Using defusion skills with internal experiences and taking valued/flexible action," the facilitator provides the following information:

- *Our internal experiences (thoughts, feelings, and sensations, and so on) are not a problem in and of themselves. With that said, what can be a problem and can increase our suffering are the ways in which we respond to our internal experiences. Becoming "fused" with or overly attached to certain ways of thinking or thought patterns may limit our ability to act flexibly and within our values and goals in certain life situations.*
- *An example would be a person fusing with the following thought or belief:* I'm broken. No one will ever accept me. My life will never change. *These thoughts may also be linked to feelings of hopelessness and*

depression, which the person may also fuse with. In becoming fused with the self-statement and emotions, the person is much more likely to engage in behaviors that strengthen that belief, such as avoidance and isolation. This provides the person with temporary relief, but in the long term it can lead to increased suffering and hopelessness. By "defusing" from internal experiences that are not in line with our values and goals, we can increase our flexibility in action, which can lead to increased fulfillment in the long term.

- *For example, a person can have the thought or idea* I am broken *and feel depressed, but still take effective and workable actions toward her values and goals (seeking new relationships, going back to school, applying for jobs, exercising, and so on). The defusion skill to use with this example would be to encourage the person to think* I am having the thought that I am broken *versus* I am broken. *By operating from a mindful or defused perspective, the person is observing her internal experiences and responding by taking valued action, regardless of how she feels or what her mind is telling her. It is important to note that through defusing from internal experiences, one is not attempting to change the experience or dispute the content of thoughts.*

- If the group chooses to focus on the fourth item, "Ability to feel emotions without trying to control, change, or fix them," the facilitator leads the group members in a discussion related to acceptance and mindfulness in relation to emotions. As a means of explanation and introduction, the facilitator may choose to read the following paragraphs to the group members:

Emotions are not "positive" or "negative." Emotions and feelings are simply just emotions and feelings. By labeling emotions as "positive" and "negative," humans tend to search for ways of increasing or sustaining the "positive" and decreasing or ridding themselves of the "negative." Although the effort that people put into "feeling good" is understandable and may work temporarily, it can be detrimental in the long term in that they will most likely continue to feel the whole spectrum of emotions, despite their best efforts not to. Furthermore, our efforts not to feel "negative" emotions may also increase or sustain the emotion in the long term, resulting in a person who gets angry about being angry, anxious about being anxious, sad about being sad, and so on.

In developing a sense of acceptance and mindfulness, a person can learn to be more observant and aware of emotions and more compassionate toward herself (as opposed to judgmental and self-loathing and self-pitying). Additionally, in recognizing the temporary nature of emotions, a person can "ride out the wave" while continuing to work toward her values and goals. (Topics to highlight during this discussion are mindfulness, self-compassion [practiced in statements such as I am allowed to feel this way], and valued action despite how one is feeling.) *This may include getting out of bed even when you are depressed, going to school even when you are anxious, asking someone on a date even when you are scared of rejection, and calling a support person when you are struggling and want to isolate.*

If the group chooses to focus on the fifth item, "Reframing," the facilitator reviews the concept of "reframing" with the group:

Reframing is a skill that involves developing a productive or healthy perspective on what you previously considered to be a "negative" or "destructive" experience or situation. An example would be treating a lapse as an opportunity for new learning, a mistake that you can learn from rather than a failure. Another example would be viewing an urge or a craving as a temporary experience and a signal to take better care of yourself, rather than a character flaw or something that needs to be feared, fought, or given in to.

7. Debriefing/review (10 to 20 minutes). The debriefing/review section of the session serves multiple purposes:

- To summarize the session process, content, and experience in ways that help group members remember and assimilate important information from the session

- To address any pressing unfinished business

- To elicit feedback from group members regarding the quality of the group, including what would be helpful in future sessions
- To provide focus on what's next: skill practice prior to the next session

OUTLINE OF DEBRIEFING/REVIEW:

- Summary statements by clinician–group leader.
- The facilitator writes the following questions on the board and elicits feedback from group members on today's group:
  - "What are you walking away with?"
  - General feedback: "What made sense? What didn't make sense or was not so useful? What do you need in future groups?" (The purpose is to engage clients and elicit specific feedback so as to tailor MBS to their specific needs. Additionally, the clinician may tie clients' needs self-assessments to relapse prevention and future topics.)
  - Values and goals: "What steps have you taken toward your values and goals in recent days or weeks?"
  - Random question (favorite movie, book, music, and so on).
- MBS skill-practice reminder (practice mindfulness and other skills between sessions).

8. Brief closing mindfulness experience (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

9. Topic for next group. The facilitator will inform group members of the topic for the next session: "Motivation": "In the next session, we will discuss and explore issues related to ambivalence, behavior change, and motivation."

Recovery Skills Self-Ratings 2

Rate your coping skills for each item below on a scale of 1 through 5, with 1 being poor or nonexistent and 5 being excellent.

Scale: *1 = poor or nonexistent*

2 = needs improvement

3 = okay, average, normal

4 = good to very good

5 = excellent

DK = don't know

Coping with Urges and Cravings:

_____ Urge surfing

Expansion and Mindfulness Strategies:

_____ Understanding and practicing mindfulness

_____ Using coping imagery

_____ Using defusion skills with internal experiences (thoughts, memories, sensations, emotions, and so on) and taking valued/flexible action

_____ Ability to feel emotions without trying to control, change, or fix them

_____ Reframing

Coping with Internal Experiences

Internal experiences: Anything that happens inside our minds and bodies (emotions, feelings, sensations, thoughts, urges, cravings, and so on).

Control strategies: Efforts people put forth in an attempt to avoid undesired internal experiences. This may occur through one's behavior or through one's thinking. Behavioral control strategies may involve avoiding otherwise uncomfortable situations, such as declining social events or situations in which the person risks experiencing failure or rejection, or procrastinating. Mental control strategies include processes that serve to avoid undesired internal experiences or decrease their intensity. Examples include daydreaming and mental distraction. Despite our desire to control our internal experiences, our efforts to do so often increase our suffering in the long run. What we have control over is not the experiences themselves, but how we observe and respond to those experiences. Through accepting and opening up to whatever happens inside of our minds and bodies (as opposed to struggling with, fighting, and avoiding, which only provide temporary relief), we can allow ourselves to suffer less and experience a more fulfilling life. This idea relates to the concepts of mindfulness and "urge surfing," the latter being a mindfulness-based technique for coping with urges and cravings to act impulsively. Internal experiences can be compared to an ocean wave, which will rise, peak, and eventually fall. Many people use strategies to cut off the wave (fighting, avoiding, distracting, giving in, and so on), which often works temporarily but strengthens the wave in the long term. An example of this would be someone who uses her drug of choice when experiencing urges and cravings. The urge or craving may be cut off after the initial use of the substance, but will inevitably return once the drug wears off (often growing stronger).

Many people get caught up in this cycle and often live their lives from distraction to distraction in service of not feeling or experiencing. Urge surfing is a strategy that encourages the person to slow things down, breathe, and relax into the craving or urge to act impulsively. Instead of using control strategies, a person eventually is able to learn that internal experiences are temporary and will come and go with little emotional and physical effort.

Urge Surfing: Take-Home Guide

When you are experiencing a craving or an urge to act impulsively:

- Notice how the internal experience is like a wave: it rises, it peaks, and it falls. This pattern continues. Stay with the experience. Observe the waves. Even though you are not reacting, the cravings and urges fall; they subside. They may rise again and subside again. You are like a surfer riding the waves. You may enjoy the freedom of observing and not needing to react.
- You may notice thoughts, emotions, or physical sensations that come…and go.
- Experience what a craving or urge is while making the choice to be mindful rather than to react. Some cravings and urges are more intense than others. Some are like small waves, while others are more like ocean or tidal waves.
- Notice that you can be present and not react, that you can experience cravings and urges without reacting.
- (After a period of time, do the following.) Open your eyes if they were closed, and bring your attention back to the room.

Session 7: Motivation

SESSION OBJECTIVES

Group members will:

1. Openly discuss and explore issues related to ambivalence, behavior change, and motivation.
2. Practice and enhance mindfulness skills.
3. Further their understanding of comprehensive sobriety planning through the MBS model.

SESSION OUTLINE

1. Staff and client introductions and check-in (15 to 45 minutes)
2. Introduction to mindfulness (10 to 15 minutes)
3. Brief review of MBS model (5 to 10 minutes)
4. Motivation presentation and discussion (10 to 15 minutes)
   (Break, 10 minutes)
5. Partner exercise and discussion (20 to 30 minutes)
6. Barriers to change (15 to 20 minutes)
   (Break, 10 minutes)
7. Miracle question (20 to 30 minutes)
8. Debriefing/review (10 to 20 minutes)
9. Brief closing mindfulness experience (10 to 15 minutes)
10. Topic for next group

SESSION MATERIALS

Session materials can be found at the end of the "Session Description."

CLIENT HANDOUTS:

- Rulers: Importance, Confidence, and Commitment
- Barriers to Change
- Miracle Question

SESSION DESCRIPTION

1. Staff and client introductions and check-in (two-tiered check-in) (15 to 45 minutes). The two-tiered check-in is designed to allow each person in the group to speak and offer input prior to opening up the floor to larger issues that may need to be processed (such as high-risk situations, goals, successes, and so on).

The facilitator begins the session by asking all group members to answer the first tier of questions. Once they have done so, the facilitator encourages members to address the second tier. This latter tier may or may not involve the participation of the entire group.

INITIAL CHECK-IN:

- First Tier:
  - Name, sobriety date, and something clients did mindfully outside of group
- Second Tier:
  - "Any risky situations? How did you cope?"
  - "Any successes or goals met?"

2. Introduction to mindfulness (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

- Brief mindfulness experience
- Open discussion of experiences
- Questions and comments

3. Brief review of MBS model (5 to 10 minutes). The facilitator provides a brief review of comprehensive sobriety planning by referring to the MBS model. For an overview of the MBS model, the facilitator should refer to appendix B.

Experienced group members may assist in this review process.

4. Motivation presentation and discussion (10 to 15 minutes). In this session, we focus on the struggle that people encounter in sustaining behavior changes. We introduce three exercises that group members can use to identify both their motivation and potential pitfalls along the way that can lead to a lapse or even a relapse. We suggest that the facilitator open the session by writing the following quote, attributed to Mark Twain, on a whiteboard and eliciting responses from the group members. Mark Twain reportedly identified the difference between quitting a habit and maintaining the change as follows:

Giving up smoking is the easiest thing in the world. I know because I've done it thousands of times.

The facilitator then transitions to the next exercise by having each group member identify an area of her life or a behavior she would like to change (but hasn't yet). The identified change can be giving up alcohol or drugs (including nicotine), or it can be another behavioral change, such as increasing daily exercise. Additional examples include returning to school, changing, and maintaining a diet. The behavioral changes identified will be used in the partner exercise, which will be conducted after the first break.

(Break, 10 minutes)

5. Partner exercise and discussion (20 to 30 minutes). Next, the facilitator engages the group members in a partner exercise, which incorporates a technique used in motivational interviewing referred to as the "readiness ruler." The readiness ruler encourages individuals to explore three elements of the behavior change process: importance, confidence, and commitment. Through exploring each of these elements, individuals are encouraged to look at their proximity to making the change, potential barriers, and possible solutions to moving forward (in incremental steps). Additionally, this process can help individuals work through ambivalence toward change, by helping them to openly explore the change process.

The facilitator begins the exercise by having the clients divide into pairs. Depending on the group dynamics, pairs can be determined by client choice or through pairing up with the client to her right (or left), by gender, or by crossing gender. The facilitator can then pass out the "Rulers: Importance, Confidence, and Commitment" worksheet and have the group members write down at the top of the worksheet the behavior change they identified at the end of the previous hour. Once they have written down the behavior change, the partners should be instructed to exchange worksheets and begin the exercise.

The group members then interview their partners using the "Rulers: Importance, Confidence, and Commitment" worksheet in order to guide the interview and keep track of their partner's responses. Once the initial "interview" is complete, the partners will then switch roles so that each person will have an opportunity to be the

interviewer and the interviewee.

Once group members have completed their interviews of one another, each pair will have an opportunity to present to the group. Going from pair to pair, each client will introduce her partner and read the results of the interview. When everyone has had a chance to share, the facilitator will help the group members explore things they learned during the exercise (about themselves, others, behavior changes, and so on). In the spirit of MI, this exercise can bring a gentle and nonjudgmental perspective into the process as more information comes to light about what is happening in the change process.

6. Barriers to change (15 to 20 minutes). Next, the facilitator passes out the "Barriers to Change" worksheet. The group members should then be encouraged to list the internal (inside of them) and external (outside of them) factors in their lives that have hindered them from making life changes. Once they have listed and labeled them as internal or external, the group members can brainstorm (by themselves or with a partner or a small group, which can be determined by the facilitator) small steps they can take in order to overcome each of the barriers. Once the worksheet is complete, the group facilitator encourages each group member to share her answers with the larger group.

(Break, 10 minutes)

7. Miracle question (20 to 30 minutes). The facilitator begins the last hour of group by distributing the "Miracle Question" worksheet. The "Miracle Question" exercise is a natural progression from the "Barriers to Change" exercise in that it encourages the group members to identify what their lives would be like if the barriers to change were removed or overcome. Prior to having the group members complete the worksheet, the facilitator assesses for comprehension and answers questions. Once the worksheet is complete, the group facilitator encourages each group member to share her answers with the larger group.

8. Debriefing/review (10 to 20 minutes). The debriefing/review section of the session serves multiple purposes:

- To summarize the session process, content, and experience in ways that help group members remember and assimilate important information from the session
- To address any pressing unfinished business
- To elicit feedback from group members regarding the quality of the group, including what would be helpful in future sessions
- To provide focus on what's next: skill practice prior to the next session

OUTLINE OF DEBRIEFING/REVIEW:

- Summary statements by clinician–group leader.
- The facilitator writes the following questions on the board and elicits feedback from group members on today's group.
    - "What are you walking away with?"
    - General feedback: "What made sense? What didn't make sense or was not so useful? What do you need in future groups?" (The purpose is to engage clients and elicit specific feedback for tailoring MBS to their specific needs. Additionally, the clinician may tie clients' needs self-assessments may to relapse prevention and future topics.)
    - Values and goals: "What steps have you taken toward your values and goals in recent days or weeks?"
    - Random question (favorite movie, book, music, and so on).
- MBS skill-practice reminder (practice mindfulness and other skills between sessions).

9. Brief closing mindfulness experience (10 to 15 minutes). The facilitator should refer to appendix A for the script

and instructions.

10. Topic for next group. The facilitator will inform group members of the topic for the next session: "Re-Mindfulness": "In the next session, we will explore and understand lapse and relapse as a process, and learn and discuss the concept of 're-mindfulness,' that is, returning to a mindful state after becoming distracted."

Rulers: Importance, Confidence, and Commitment

Please rate yourself on each in the following areas: importance, confidence, and commitment.

Importance

1. On a scale of 0 to 10, with 0 being "not important at all" and 10 being "very highly important," how important is it to you to make the change?
2. Your rating: _____
3. Why is it (your rating) and not 0? Please list all the reasons you can think of (you may use the back of the page if necessary):

   _____

   _____

Confidence

1. On a scale from 0 to 10, with 0 being "not confident at all" and 10 being "very highly confident," how confident are you that you will make the change?
2. Your rating: _____
3. Why is it (your rating) and not 0? Please list all the reasons you can think of (you may use the back of the page if necessary):

   _____

   _____

Commitment

1. On a scale from 0 to 10, with 0 being "not committed at all" and 10 being "very highly committed," how committed are you to making the change?
2. Your rating: _____
3. Why is it (your rating) and not 0? Please list all the reasons you can think of (you may use the back of the page if necessary):

   _____

   _____

Barriers to Change

What are some of the internal (inside of you) and external (outside of you) factors that hinder you from making changes in your life?

Label each "I" for internal or "E" for external:

_____

_____

_____
_____
_____
_____
_____

Brainstorm small steps you can take in order to begin overcoming each of the barriers:

_____
_____
_____
_____
_____
_____
_____

Miracle Question

If you were to wake up tomorrow with your life 100 percent on track with the way you want it to be and all barriers to change removed, what would it look like? Consider things such as relationships, work, school, fun, and family. Be as realistic as possible.

_____
_____
_____
_____
_____
_____
_____

What is a small step that you are willing to take in the next twenty-four hours in order to move yourself or your life closer to the way you want it to be?

_____
_____
_____
_____
_____
_____
_____

Session 8: "Re-Mindfulness"

SESSION OBJECTIVES

Group members will:

1. Explore and understand lapse and relapse as a process.
2. Learn the concept of "re-mindfulness" practice and enhance mindfulness skills.
3. Further their understanding of comprehensive sobriety planning through the MBS model.

SESSION OUTLINE

1. Staff and client introductions and check-in (15 to 45 minutes)
2. Introduction to mindfulness (10 to 15 minutes)
3. Brief review of MBS model (5 to 10 minutes)
4. Autopilot (10 to 15 minutes)
5. *(Break, 10 minutes)* Relapse road (45 to 60 minutes)
6. *(Break, 10 minutes)* Re-mindfulness card exercise (20 to 30 minutes)
7. Debriefing/review (10 to 20 minutes)
8. Brief closing mindfulness experience (10 to 15 minutes)
9. Topic for next group

SESSION MATERIALS

Session materials, including facilitator guide and client handouts, can be found at the end of the "Session Description."

FACILITATOR GUIDE:

- Re-Mindfulness Card Examples

CLIENT HANDOUTS:

- Relapse Road
- High-Risk Events and Scenarios (Experience in Situation) (appendix D)
- Three-by-five-inch blank (unlined) index cards for the Re-Mindfulness Card exercise

SESSION DESCRIPTION

1. Staff and client introductions and check-in (two-tiered check-in) (15 to 45 minutes). The two-tiered check-in is designed to allow each person in the group to speak and offer input prior to opening up the floor to larger issues that may need to be processed (such as high-risk situations, goals, successes, and so on).

The facilitator begins the session by asking all group members to answer the first tier of questions. Once they have done so, the facilitator encourages members to address the second tier. This latter tier may or may not involve the participation of the entire group.

INITIAL CHECK-IN:

- First Tier:
  - Name, sobriety date, and something clients did mindfully outside of group
- Second Tier:
  - "Any risky situations? How did you cope?"
  - "Any successes or goals met?"

2. Introduction to mindfulness (10 to 15 minutes). The facilitator should refer to appendix A for the script and

instructions.

- Brief mindfulness experience
- Open discussion of experiences
- Questions and comments

3. Brief review of MBS model (5 to 10 minutes). The facilitator provides a brief review of comprehensive sobriety planning by referring to the MBS model. For an overview of the MBS model, the facilitator should refer to appendix B.

Experienced group members may assist in this review process.

4. Autopilot (10 to 15 minutes). The facilitator introduces the concept of autopilot by reading the following metaphor to the group:

> *For many people, life can be like riding in a car on autopilot. However, living life on autopilot is a bit different in that it involves more than just the speed at which a person's "car" is traveling. Life on autopilot can feel as if the "car" were programmed to go down only certain roads and toward certain destinations, with similar outcomes each time. Despite our ability to make choices and take valued paths toward our goals, we get in the "car" day after day and find ourselves driving toward situations and experiences that feel familiar and comfortable, but are not necessarily places we want to be in the long term.*

Once the facilitator has read the metaphor to the group, she helps the group members to process their reactions by asking the following questions:

- "Does this metaphor make sense in your life? If so, how?"
- "How fulfilling was your quality of life when you were on autopilot?"
- "How did it feel when you first realized that you had been on autopilot?"
- "What might you do to prompt yourself to awaken from autopilot?"

(Break, 10 minutes)

5. Relapse road (45 to 60 minutes). Next, the facilitator engages the group members in a discussion about the lapse and relapse process. It is important to note that some individuals may reject the concept of a "lapse" due to seeing it as a reservation or "permission to use one more time." If any of the group members respond in this way, the facilitator should validate their position and encourage them to use terms that work for them.

The facilitator reads the following paragraphs to the group as a means of introduction:

> *We are now going to discuss a road that many people travel down when trying to make a behavioral change, especially when it comes to substance use. For some individuals, lapses and relapses can be part of the recovery process. A "lapse" is defined as a limited or single use of a substance after a period of sobriety, without being a full-blown relapse. "Relapse" is defined as returning to the previous pattern of use prior to becoming sober. The moments prior to or after a lapse or relapse are often challenging and confusing times.*
>
> *With that said, the lapse or relapse process can begin prior to someone actually using a substance. This can be referred to as engaging in "prelapse decisions and behaviors," that is, unconscious or conscious decisions and behaviors that lead down a path toward high-risk situations and potential lapses or relapses. If prelapse behaviors are addressed early on, a person may be able to avoid a lapse and subsequent relapse, and return to a valued path.*

The facilitator then asks group members to provide examples of their own prelapse decisions and behaviors:

"What are some examples of prelapse behaviors you have experienced or know of that could lead toward lapse or relapse?"

If group members are unable to provide any examples, the facilitator provides the following:

- After a stressful week at work, a person in recovery skips a session with her counselor and goes to a family celebration at a place where alcohol is being served.
- Someone drives by the old neighborhood "for memory's sake" and happens to see one of her old friends who is still using.

Next, the facilitator distributes two worksheets:

- Relapse Road
- High-Risk Events and Scenarios (Experience in Situation) (appendix D)

Then the facilitator explains the MBS model on the High-Risk Events and Scenarios worksheet by reading the following:

Risk of lapse or relapse increases according to the person's experiences in particular situations. Typically, people experience situations like math problems, such as 1 + 1 = 2. For example, a person in recovery goes to a wedding where alcohol is available and people are indulging. In the past, she had also indulged in alcohol at weddings. Since she (1) is at a wedding where alcohol is available (+ 1), the response is that she drinks (= 2).

Other examples include:

- *Feeling anxious (experience) at a party where you have access to marijuana (situation), so you indulge (response)*
- *Thinking about a time when things were better (experience) while you are alone and still in contact with old friends who are still using (situation), so you call them to score some drugs (response)*
- *Feeling lonely (experience) while not having a support network to call (situation), so you drive to the local bar (response)*

The facilitator asks group members to complete the "Relapse Road" worksheet. First, they should complete the "Experience in Situation" box at the top of the worksheet. In this box, they are asked to write information about a prior lapse or about a possible future high-risk scenario. They should describe both the risky situation and their experience (or anticipated experience) in the situation. Group members may refer to the "High-Risk Events and Scenarios" worksheet for promptings.

Next, group members are asked to write, in the "Prelapse Behavior" boxes, prelapse behaviors they have engaged in or might engage in. The behaviors may or may not be directly linked to the high-risk situation they had just described. Some group members may recall the prelapse behavior that occurred just before a lapse and then the behavior before that, and so on, working their way back in time. Others may list the behaviors in no particular order. Either approach is fine.

The facilitator may also walk through the "Relapse Road" worksheet with the group by providing the following example.

A person in recovery has a stressful week and is invited to a party that her coworkers are hosting on a Friday night. Prelapse behaviors might include staying up too late the night before, remaining at the party despite wanting to leave, and not bringing a sober friend to the party. The person might feel anxious (experience) while at the party (situation), and the outcome could be that she smokes marijuana.

The facilitator encourages group members to share their answers in an open-group discussion.

Next, the facilitator revisits the MBS model on High-Risk Events and Scenarios, emphasizing alternative ("as

opposed to") situations. The facilitator may read to the group:

> *In becoming aware of autopilot or programmed responses, a person can develop a transcendent or broader perspective, which can help her experience a situation differently and respond in a value-consistent manner.*
>
> *Examples include:*

- *Feeling happy (experience) at a wedding where there is an open bar and drinking is encouraged (situation) and drinking (response)*
  - *As opposed to feeling happy (experience) at a wedding where there is an open bar (situation) and you had arranged to be accompanied by a sober support (response)*
- *Feeling anxious (experience) at a party where you have access to marijuana (situation) and choosing to use (response)*
  - *As opposed to feeling anxious (experience) at the party (situation) and leaving or refusing the marijuana that's offered (response)*
- *Remembering a time when things were better (experience) while alone (situation) and making contact with old friends who are still using (response)*
  - *As opposed to remembering a time when things were better (experience) while currently in healthy relations with friends and family (situation), after having repaired those relationships (response)*
- *Feeling lonely (experience) while not having a support network to call (situation) and driving to a local bar (response)*
  - *As opposed to feeling lonely (experience) while having a support network to call (situation), after having built a peer support network (response)*

The facilitator then leads group discussion about how someone can experience similar things in different situations and with different outcomes.

Group members may point out that the above exercise makes a simple matter appear more complex and that the "take-home" message is just that they need to respond differently to life challenges. In response, the facilitator can affirm their insight and remind them that when someone is used to operating on "autopilot," that person might fail to see alternative perspectives and options.

(Break, 10 minutes)

6. Re-mindfulness card exercise (20 to 30 minutes). The facilitator begins this part of the session by introducing the concept of "re-mindfulness."

The facilitator writes the definition of "re-mindfulness" on the board and then reads it aloud:

> *"Re-mindfulness" means gently returning yourself to a mindful state after becoming distracted or going on autopilot.*

The facilitator then reads the following metaphor aloud:

> *When you find yourself traveling in a nonvalued direction, you can use the re-mindful process to get back on track. It's like putting on the brakes or pulling a U-turn and saying to yourself,* I've been down this road before; this is a route that needs to be abandoned.

The final exercise in this session is the "re-mindfulness card." Group members can use the card as a tool when they recognize that they are drifting away from sobriety. The re-mindfulness card can be kept in a wallet or purse for quick access. The facilitator begins the exercise by passing out blank three-by-five-inch index cards to the group.

The facilitator then writes the “re-mindful card” format on the whiteboard, beginning with the front side and taking one item at a time. The facilitator explains each item as she goes, asking group members to enter their personal information throughout the process:

1. Stop: *The first thing you can do when you find yourself on autopilot and heading in a nonvalued direction (such as toward a relapse) is to physically stop what you’re doing. Examples include sitting down on a chair or pulling over to the side of the road (if you’re driving).*

2. Aware: *Become aware of your feelings, thoughts, tension, and urges, mindfully observing thoughts and bringing attention to the present moment.*

3. Breathe: *Focus on your breathing to ground yourself.*

4. Respond: *Don’t react. Respond to the situation in a value-consistent manner.*

5. Re-Mindful: *This is a space for you to write something meaningful, such as a quote. For example:*

    - *“The only way out is through” (Robert Frost).*

    - *“If you are not moving away from a drink, you are moving closer to it. Life isn’t about finding yourself. Life is about creating yourself” (George Bernard Shaw).*

    - *“Although the world is full of suffering, it is full also of the overcoming of it” (Helen Keller).*

6. Re-New Commitment (reasons to commit to recovery): *Please list your reasons for committing to sobriety. Some examples are family, health, job, education, and peace of mind.*

Now, the facilitator will walk group members through items on the back of the card:

- Sober supports: Group members are asked to list sober supports and their phone numbers.

- Healthy activities/support meetings: Group members are asked to list healthy activities they can engage in, including peer support meetings.

If time allows, group members should review their cards with the larger group.

Re-Mindfulness Card (Front)

Re-Mindfulness Card

STOP

Aware Breathe Respond

Re-MINDFUL:

Re-New Commitment (reasons to commit to recovery):

Re-Mindfulness Card (Back)

Re-Mindful Resources

Sober Supports:

Healthy Activities/Support Meetings:

Re-Mindfulness Card Examples

Re-Mindfulness Card (Front)

Re-Mindfulness Card

STOP

Aware Breathe Respond

Re-MINDFUL:

The only way out is through.

Re-New Commitment (reasons to commit to recovery):

My family, my job, and me.

Re-Mindfulness Card (Back)

Re-Mindful Resources

Sober Supports:

Sandra Spouse: 555-123-4567

Nick Sponsor: 555-987-6543

Phil Goode: 555-918-2736

Healthy Activities/Support Meetings:

AA meeting

Clean garage

Play with kid

7. Debriefing/review (10 to 20 minutes). The debriefing/review section of the session serves multiple purposes:

- To summarize the session process, content, and experience in ways that help group members remember and

assimilate important information from the session

- To address any pressing unfinished business
- To elicit feedback from group members regarding the quality of the group, including what would be helpful in future sessions
- To provide focus on what's next: skill practice prior to the next session

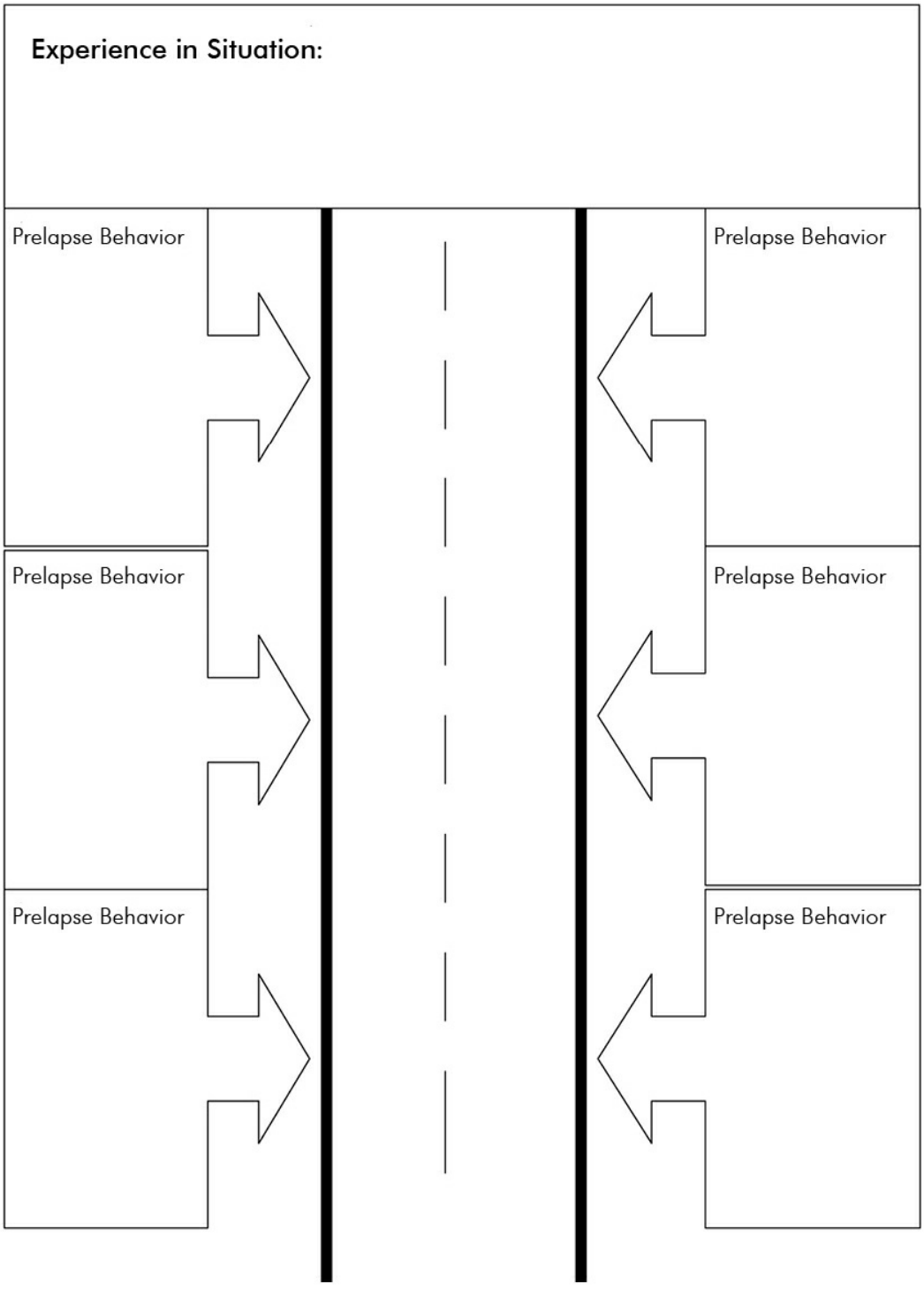
OUTLINE OF DEBRIEFING/REVIEW:

- Summary statements by clinician–group leader.
- The facilitator writes the following questions on the board and elicits feedback from group members on today's group:
  - "What are you walking away with?"
  - General feedback: "What made sense? What didn't make sense or was not so useful? What do you need in future groups?" (The purpose is to engage clients and elicit specific feedback for tailoring MBS to their specific needs. Additionally, the clinician may tie clients' needs self-assessments to relapse prevention and future topics.)
  - Values and goals: "What steps have you taken toward your values and goals in recent days and weeks?"
  - Random question (favorite movies, books, music, and so on).
- MBS skill-practice reminder (practice mindfulness and other skills between sessions).

8. Brief closing mindfulness experience (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

9. Topic for next group. The facilitator will inform group members of the topic for the next session: "Quality of Life: Value-Based Living": "In the next session, we will further our understanding of (or be introduced to) the 'value-based living' process, examine important life areas, and develop a plan to improve our quality of life."

Relapse Road



Session 9: Quality of Life: Value-Based Living

SESSION OBJECTIVES

Group members will:

1. Further their understanding of (or be introduced to) the "value-based living" process
2. Examine important life areas and develop a plan to improve their quality of life
3. Practice and enhance mindfulness skills
4. Further their understanding of comprehensive sobriety planning through the MBS model

SESSION OUTLINE

1. Staff and client introductions and check-in (15 to 45 minutes)
2. Introduction to mindfulness (10 to 15 minutes)
3. Brief review of MBS model (5 to 10 minutes)
4. Value-based living presentation (15 to 20 minutes)
   (Break, 10 minutes)
5. "Important Points: Value-Based Living" exercise (15 to 20 minutes)
6. "Value-Based Living" exercise (30 to 40 minutes)
   (Break, 10 minutes)
7. Moving forward (20 to 30 minutes)
8. Debriefing/review (10 to 20 minutes)
9. Brief closing mindfulness experience (10 to 15 minutes)
10. Topic for next group

SESSION MATERIALS

Session materials, including facilitator guide and client handouts, can be found at the end of the "Session Description."

FACILITATOR GUIDE:

- Value-Based Living Presentation (see appendix C)

CLIENT HANDOUTS:

- Important Points: Value-Based Living
- Value-Based Living
- Moving Forward

SESSION DESCRIPTION

1. Staff and client introductions and check-in (two-tiered check-in) (15 to 45 minutes). The two-tiered check-in is designed to allow each person in the group to speak and offer input prior to opening up the floor to larger issues that may need to be processed (such as high-risk situations, goals, successes, and so on).

The facilitator begins the session by asking all group members to answer the first tier of questions. Once they have done so, the facilitator encourages members to address the second tier. This latter tier may or may not involve

the participation of the entire group.

INITIAL CHECK-IN:

- First Tier:
  - Name, sobriety date, skill practice (such as, something clients did mindfully outside of group)
- Second Tier:
  - "Any risky situations? How did you cope?"
  - "Any successes or goals met?"

2. Introduction to mindfulness (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

- Brief mindfulness experience
- Open discussion of experiences
- Questions and comments

3. Brief review of MBS model (5 to 10 minutes). The facilitator provides a brief review of comprehensive sobriety planning by referring to the MBS model. For an overview of the MBS model, the facilitator should refer to appendix B.

Experienced group members may assist in this review process.

4. Value-based living presentation (15 to 20 minutes). See appendix C, "Value-Based Living Presentation," for instructions for this section of the session.

(Break, 10 minutes)

5. "Important Points: Value-Based Living" exercise (15 to 20 minutes). The facilitator begins this part of the session by distributing the "Important Points: Value-Based Living" worksheet. In explaining the worksheet to the group members, the facilitator initially highlights the differences between the value-based living categories of "Current Status" and "Valued Status." The facilitator does this by writing the following definitions on the board and asking group members to write the definitions on their worksheets in the "Current Status" and "Valued Status" sections:

- *Current Status:* What your quality of life looks like right now (satisfaction, fulfillment, enjoyment, and so on)
- *Valued Status:* What you would like your quality of life to look like

The facilitator then provides an example: "A group member may note that she currently lives at her parent's home (current status) but would prefer to be living in her own apartment (valued status)."

Next, the facilitator explores the following key concepts with the group members by writing them on the board. Group members are encouraged to write down the key points in the "Ideas to Keep in Mind" section of the worksheet. The facilitator can read the following points out loud in order to explain the worksheet:

1. *"Current Status" is a snapshot of your quality of life at the moment. It's not something to be ashamed of or overwhelmed by.*
2. *"Valued Status" provides direction.*

   We aren't talking about perfection. (No one is perfect. It may be unrealistic to aim for 100 percent life satisfaction.) Most people's lives fall somewhere between "Current Status" and "Valued Status." Through bringing to light the differences between "Current Status" and "Valued Status," we can begin exploring

and setting goals to increase the quality of our lives.

3. *Quality of life is multidimensional. It involves many life areas, including "financial," "work," "social/friends," "physical/health," "personal development," "family/relationships," "living environment," "emotional," "community/volunteer," and "spiritual." These different areas are not isolated concerns; they can influence one another.*

   For example:

   1. *If you don't focus on your personal development, your relationships may be affected.*

   2. *If you neglect your physical health, your emotional health may suffer.*

The facilitator then asks group members to provide examples.

6. "Value-Based Living" exercise (30 to 40 minutes). The facilitator distributes the "Value-Based Living" worksheet to the group members. Under the column "Current Status," group members are asked to write their personal current status for each life area. Following that, under the column "Valued Status," group members are asked to write their personal valued status. Some examples might be a person who values relationships but is single and spending too much time watching TV alone. Another might be a person who values physical health and is following a routine of healthy diet and exercise. As these examples reflect, the "Current Status" and "Valued Status" may or may not be consistent with one another. Blank boxes are included for group members who would like to add categories that are not listed on the sheet.

As the group members are completing the worksheet, the facilitator walks around the room and is available for questions and suggestions. After group members complete their worksheets, the facilitator asks them to meet in pairs or small groups. In the small groups, members are asked to discuss their answers and provide each other with supportive feedback. Once the small group discussions are completed, the facilitator brings the discussion to the larger group and encourages each group member to share one or two categories prior to the break.

Example Large Group Dialogue

E*xample Scenario:* Client 1 is married with two children, ages ten and thirteen. She has identified that she values her family, which is why she works long hours. Her workdays frequently last twelve hours plus weekend time. She comes home every night and has "a couple of drinks." Her family is tired of never seeing her. When she is home, they report that she is always drinking and, while not passed out, is "kind of out of it." She is not available to them.

Client 1. I love my family, but in my field, I have to work long hours to be able to afford the house and our lifestyle. I am doing it for them.

Clinician. Your family is important to you; otherwise why would you bother working so hard? But it is stressful.

Client 1. That's it exactly.

Clinician. I am hearing you considering "Work" and "Finances" as one area; is that correct?

Client 1. To me it's one and the same.

Clinician. Okay, so on your worksheet "Financial" area, in the "Current Status" column, your current status is "employed full-time, long hours, satisfying work." Do you want to add anything else to that?

Client 1. No. I would always like more money, but I can't complain.

Clinician. For the "Family" area, I am hearing how much you love your family, so how would you define "Current Status" and "Valued Status"?

Client 1. My wife and kids complain that I'm never around. "Current Status" is strained, to be honest. And "Valued Status" would be, I guess, to spend more time as a family. I've always wanted to have a game night or at least eat one dinner together.

Clinician. Is it okay if I give you some feedback?

Client 1. Sure.

Clinician. Okay, so, in the next exercise, called "Moving Forward," we're going to talk about goals related to quality of life. In this coming exercise, you might consider setting "family," "family meals," or "family night" as a goal. How does that sound to you?

Client 1. Sounds great.

(Break, 10 minutes)

7. Moving forward (20 to 30 minutes). The facilitator begins this part of the session by distributing the worksheet "Moving Forward." Group members are asked to select "life areas" from the previous worksheet ("Value-Based Living") for which they would like to set goals. The group members are encouraged to formulate goals that are specific, measurable, practical, and accomplishable and that they can commit to. As the group members are completing the worksheet, the facilitator walks around and is available for questions and suggestions. Once the "Moving Forward" worksheet is completed, group members are encouraged to share their answers with the larger group.

8. Debriefing/review (10 to 20 minutes). The debriefing/review section of the session serves multiple purposes:

- To summarize the session process, content, and experience in ways that help group members remember and assimilate important information from the session
- To address any pressing unfinished business
- To elicit feedback from group members regarding the quality of the group, including what would be helpful in future sessions
- To provide focus on what's next: skill practice prior to the next session

OUTLINE OF DEBRIEFING/REVIEW:

- Summary statements by the clinician–group leader.
- The facilitator writes the following questions on the board and elicits feedback from group members on today's group.
    - "What are you walking away with?"
    - General feedback: "What made sense? What didn't make sense or was not so useful? What do you need in future groups?" (The purpose is to engage clients and elicit specific feedback for tailoring MBS to their specific needs. Additionally, the clinician may tie clients' needs self-assessments to relapse prevention and future topics.)
    - Values and goals: "What steps have you taken toward your values and goals in recent days or weeks?"
    - Random question (favorite movie, book, music, and so on).
- MBS skill-practice reminder (practice mindfulness and other skills between sessions).

9. Brief closing mindfulness experience (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

10. Topic for next group. The facilitator will inform group members of the topic for the next session: "Relationships": "In the next session, we will identify and process the impact that relationships have on our well-being and recovery, learn and identify helpful versus unhelpful communication skills and interpersonal behaviors, and establish and identify values and goals related to relationships."

Important Points: Value-Based Living

Current Status:

Valued Status:

Ideas to Keep in Mind

1.

2.

3.

Value-Based Living

Life Area	Current Status	Valued Status
Financial		
Work		
Social/Friends		
Physical/Health		
Personal Development		
Family/Relationship		

Value-Based Living (cont.)

Life Area	Current Status	Valued Status
Living Environment		
Emotional		
Community/Volunteer		
Spiritual		
Additional Life Area:		
Additional Life Area:		

Moving Forward

Life Area	Steps I Can Take (that are specific and measurable, and practical and accomplishable, and that I am committed to)

Session 10: Relationships

SESSION OBJECTIVES

Group members will:

1. Identify and process the impact that relationships have on their well-being and recovery
2. Learn and identify helpful versus unhelpful communication skills and interpersonal behaviors

3. Establish and identify values and goals related to relationships
4. Practice and enhance mindfulness skills
5. Further their understanding of comprehensive sobriety planning through the MBS model

SESSION OUTLINE

1. Staff and client introductions and check-in (15 to 45 minutes)
2. Introduction to mindfulness (10 to 15 minutes)
3. Brief review of MBS model (5 to 10 minutes)
4. Introduction to topic: Family, friends, and others (10 to 15 minutes)
   (Break, 10 minutes)
5. Communication skills (50 to 60 minutes)
   (Break, 10 minutes)
6. Partner exercise: "Relationships in Recovery" (20 to 30 minutes)
7. Debriefing/review (10 to 20 minutes)
8. Brief closing mindfulness experience (10 to 15 minutes)
9. Topic for next group

SESSION MATERIALS

Session materials, including facilitator guide and client handouts, can be found at the end of the "Session Description."

FACILITATOR GUIDE:

- Thumbs-Up, Thumbs-Down Answer Key (Facilitator Guide)

CLIENT HANDOUTS:

- Concentric Circles (untitled)
- Thumbs-Up, Thumbs-Down (untitled)
- Relationships in Recovery

SESSION DESCRIPTION

1. Staff and client introductions and check-in (two-tiered check-in) (15 to 45 minutes). The two-tiered check-in is designed to allow each person in the group to speak and offer input prior to opening up the floor to larger issues that may need to be processed (such as high-risk situations, goals, successes, and so on).

The facilitator begins the session by asking all group members to answer the first tier of questions. Once they have done so, the facilitator encourages members to address the second tier. This latter tier may or may not involve the participation of the entire group.

INITIAL CHECK-IN:

- First Tier:

- Name, sobriety date, and something clients did mindfully outside of group

- Second Tier:
  - "Any risky situations? How did you cope?"
  - "Any successes or goals met?"

2. Introduction to mindfulness (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

- Brief mindfulness experience
- Open discussion of experiences
- Questions and comments

3. Brief review of MBS model (5 to 10 minutes). The facilitator provides a brief review of comprehensive sobriety planning by referring to the MBS model. For an overview of the MBS model, the facilitator should refer to appendix B.

Experienced group members may assist in this review process.

4. Introduction to topic: Family, friends, and others (10 to 15 minutes). The facilitator begins this exercise by distributing the "Concentric Circles" worksheet. The facilitator explains to the group members that the inner circle represents the "self" and that the outer circles represent the degrees of closeness to the self.

After explaining the "Concentric Circles" worksheet, the facilitator asks group members to identify the people who are closest to them and to write the names of those people in the circle next to the "self" circle. This should not be limited to only "positive" influences or relationships. In the remaining (outer) two circles, group members identify people in their lives who are less close to them but who are influential (either in positive or negative ways). Examples may include employers, coworkers, teachers, legal parties, old friends who are still using, dealers, and other acquaintances. It is important to note that individuals will vary in their answers. For example, some may place their families on the inner circle, while others may place them on the outer.

The facilitator can then lead a discussion related to degrees of closeness and the impact that relationships have on one's mind-set and well-being. Specific facilitation questions are included below:

1. *How do the people in your outer circle negatively influence your life? How do the people in your outer circle positively affect your life?*
2. *How do the people in your inner circle negatively affect your life? How do the people in your inner circle positively affect your life?*
3. *Identify unhealthy people who are in your inner circle but might be better off in your outer circle.*
4. *Identify healthy people who are in your outer circle whom you might benefit from having in your inner circle.*

(Break, 10 minutes)

5. Communication skills (50 to 60 minutes). The facilitator begins this section of the session by writing "Helpful" on one side of the whiteboard and "Unhelpful" on the other (setting up two columns). Next, the facilitator provides a brief lecture and leads a group discussion based on the helpful and unhelpful communication behaviors included below:

COMMUNICATION BEHAVIORS: NOT HELPFUL AND HELPFUL

- Not Helpful
  - Advice giving

- Lecturing
- Excessive questioning, for example, asking “why?” repeatedly
- Storytelling
- Bringing up the past
- Blaming and shaming
- Sarcasm
- Body language (closed, aggressive, or defensive)

- Helpful
  - “I” statements
  - Listening
  - Validation
  - Open-ended questions
  - Affirmations
  - Positive reinforcement
  - Body language (open and inviting, and making eye contact)
  - Warmth, empathy, genuineness

As each behavior is discussed, the facilitator should write the behavior on the board under the corresponding column.

Following the discussion, the facilitator passes out the “Thumbs Up, Thumbs Down” worksheet. The facilitator introduces the exercise by reading the following:

> *During this next exercise, I will read a series of fourteen statements, each of which represents a different helpful or unhelpful communication behavior.*
>
> *As I read each statement, you will reference the communication behaviors written on the board, choose one, and then write it in the appropriate column on your worksheet.*
>
> *The “Thumbs Up” column represents helpful communication behaviors, while the “Thumbs Down” column represents unhelpful communication behaviors.*

With the communication behaviors still written on the board, the facilitator reads, one at a time, the statements on the “Thumbs Up, Thumbs Down Answer Key (Facilitator Guide).” Group members are asked to choose which type of behavior (“helpful” or unhelpful”) each statement represents and which column (“Thumbs Up” or “Thumbs Down”) it should be placed under. (“Thumbs Up” represents “helpful” and “Thumbs Down” represents “unhelpful”).

The group members should follow along and write their answers in the spaces provided. (It is important to note that this exercise can be subjective and that there can be multiple answers to some of the statements, which should be allowed and encouraged.) After the group members have rated each of the fourteen statements, the facilitator should review and explore their responses and reactions to each statement (one at a time).

Next, the facilitator leads the group members in a discussion about self-advocacy skills. During the discussion, the facilitator asks group members to think about ways they can improve personal relationships. Group members are then encouraged to explore ways in which they can obtain their relationship needs and wants through the use of the

helpful communication skills discussed in this exercise.

(Break, 10 minutes)

6. Partner exercise: "Relationships in Recovery" (20 to 30 minutes). The facilitator begins this exercise by distributing the "Relationships in Recovery" worksheet. Once group members complete the worksheet, they are asked to split up into groups of two to four individuals in order to discuss their answers. After the small groups are finished discussing, the facilitator leads a discussion with the entire group related to their answers and relationship values and goals.

7. Debriefing/review (10 to 20 minutes). The debriefing/review section of the session serves multiple purposes:

- To summarize the session process, content, and experience in ways that help group members remember and assimilate important information from the session
- To address any pressing unfinished business
- To elicit feedback from group members regarding the quality of the group, including what would be helpful in future sessions
- To provide focus on what's next: skill practice prior to the next session

OUTLINE OF DEBRIEFING/REVIEW:

- Summary statements by the clinician–group leader.
- The facilitator writes the following questions on the board and elicits feedback from group members on today's group:
  - "What are you walking away with?"
  - General feedback: "What made sense? What didn't make sense or was not so useful? What do you need in future groups?" (The purpose is to engage clients and elicit specific feedback so as to tailor MBS to their specific needs. Additionally, the clinician may tie clients' needs self-assessments to relapse prevention and future topics.)
  - Values and goals: "What steps have you taken toward your values and goals in recent days or weeks?"
  - Random question (favorite movie, book, music, and so on).
- MBS skill-practice reminder (practice mindfulness and other skills between sessions).

8. Brief closing mindfulness experience (10 to 15 minutes). The facilitator should refer to appendix A for the script and instructions.

9. Topic for next group. The facilitator will inform group members of the topic for the next session: "Exploring Values": "In the next session (Session 1), we will identify values and things in our lives that are important to us."

Thumbs Up, Thumbs Down Answer Key (Facilitator Guide)

Note: Some quotes may be interpreted in multiple ways. Group members can repeat answers and have multiple answers in one box.

1. "You should really do that, because it worked for me. It will definitely help you. Trust me, I know best."

   Answer. Advice giving (unhelpful; thumbs down)

2. "You have a month sober. That is great! You have been working really hard."

Answer. Affirmation (helpful; thumbs up)

3. “You should know better; you are an adult. Adults are supposed to be responsible and follow through with what they are asked to do. You need to grow up and start acting like a mature adult. When are you going to learn?”

   Answer. Lecturing (unhelpful; thumbs down)

4. “Where were you last night? Why didn’t you call? Why were you hanging out with those people again? I thought you said you were going to cut them off? Are you ever going to learn?”

   Answer. Excessive questioning (unhelpful; thumbs down)

5. “I can understand where you are coming from. I would be hurt too if that happened to me.”

   Answer. Warmth, empathy, genuineness (helpful; thumbs up)

6. “I was in your situation last year. I was with Gary at the time, and we were hanging out at his apartment. It was winter, so the weather was awful; we were staying inside, because it was cold and we didn’t want to go anywhere in the snow. We were eating pizza and got a phone call from Steve, and he had heard from Jen that there was…”

   Answer. Storytelling (unhelpful; thumbs down)

7. “What was that like for you when you were going through those difficult times?”

   Answer. Open-ended question (helpful; thumbs up)

8. “You always do this! You are never going to change! I have bailed you out so many times. Remember last year when…”

   Answer. Bringing up the past (unhelpful; thumbs down)

9. “You are such a loser. All of our financial problems are your fault. I should just kick you out. You are a sorry excuse for a partner.”

   Answer. Blaming and shaming (unhelpful; thumbs down)

10. “You deserve a break. You have been working long hours, and that is draining. How can I help?”

    Answer. Validation/affirmation (helpful; thumbs up)

11. “Oh! So you have been sober for a whole month. What is that—two days longer than last time? Good job. Psshhh.”

    Answer. Sarcasm (unhelpful; thumbs down)

12. “I get upset when things don’t work out. I would like to come up with a plan for us to move forward with this situation.”

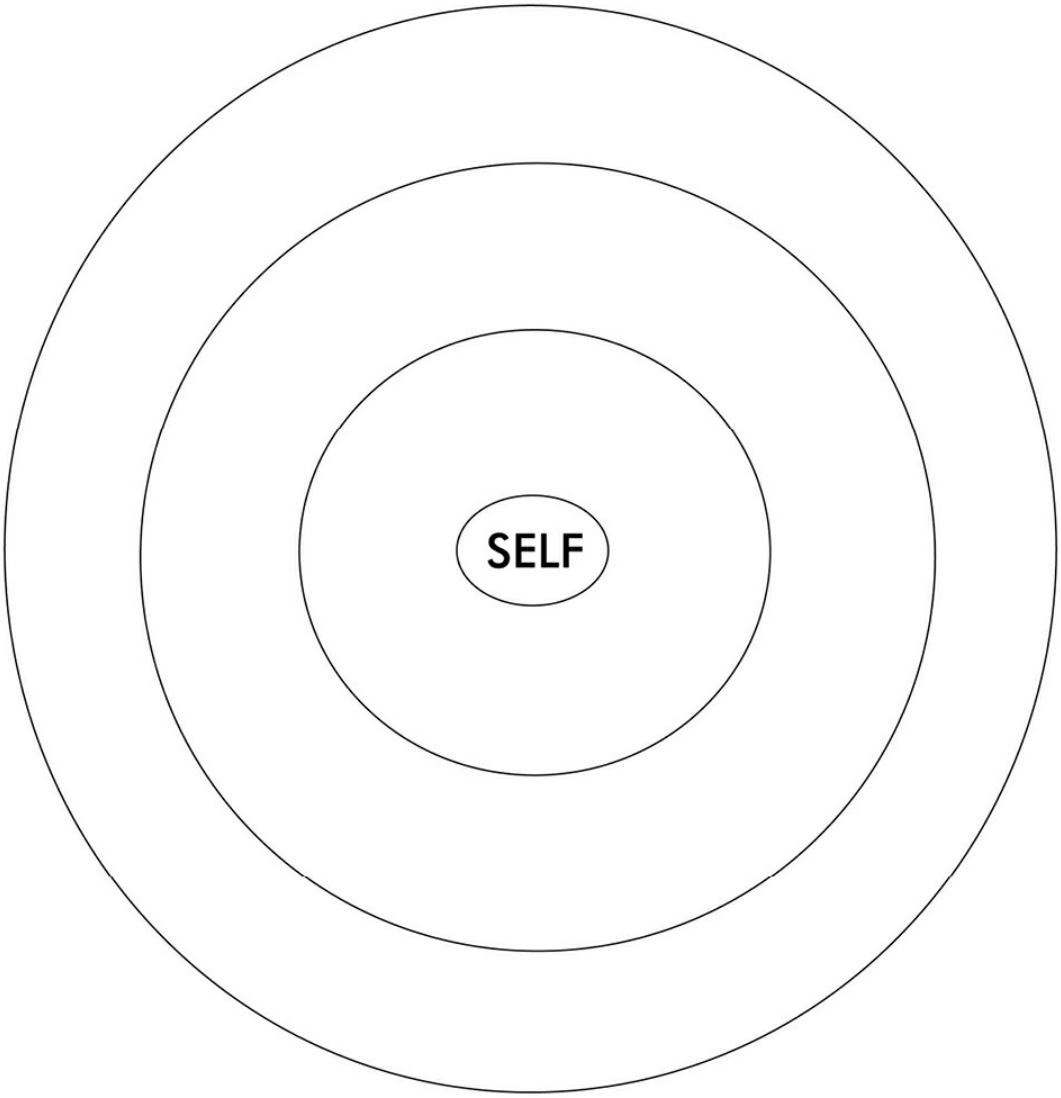
    Answer. “I” statements (helpful; thumbs up)

13. “Okay, so what I heard you say was that you want me to be on time more often and to follow through with my responsibilities. You would also like to spend more time doing fun things with me, like going to the movies and out for dinner. Did I hear you right?”

    Answer. Listening (helpful; thumbs up)

14. “I really enjoyed when you took time to talk to me about your day and then asked me about mine. It felt good.”

    Answer. Positive reinforcement (helpful; thumbs up)


SELF

	
1.	1.
2.	2.
3.	3.
4.	4.
5.	5.
6.	6.
7.	7.
8.	8.
9.	9.
10.	10.
11.	11.
12.	12.
13.	13.
14.	14.

Relationships in Recovery

1. What types of relationships do you want to seek out? (This includes significant others, your children, parents, relatives, friends, neighbors, classmates, and all your other social contacts.)
2. How do you want to treat the people you are establishing relationships with (values within relationships)?
3. How do you want to be treated by those individuals (values and respect related to your sense of “self”)?
4. What sorts of activities do you want to engage in within these relationships?

Chapter 5

Mindfulness-Based Sobriety: Residential Treatment Curriculum

Courage is not the absence of despair; it is, rather, the capacity to move ahead in spite of despair.

—Rollo May

The mindfulness-based sobriety (MBS) residential curriculum focuses on the person's need for the residential level of care and what needs to happen (planning, skill development, motivation enhancement, and so on) to prepare for the next (lower) level of care. Accordingly, factors that contribute to the person's need for residential treatment may be regarded as barriers to discharge; these factors preclude success in a lower level of care, such as a partial-hospitalization program (PHP) or an intensive outpatient program (IOP).

We recommend that MBS groups be a core of the residential treatment array of services. The groups can meet three to seven times per week. This model is designed for relatively brief residential stays. However, this should not present problems for individuals whose clinical needs require longer stays. Further participation in this curriculum will allow for additional review of principles, assessment of sobriety challenges, relapse prevention planning, personal reflection, motivation enhancement, and recovery-skill practice.

Session Setting and Materials

Room Arrangement: For MBS sessions, a circular or semicircular arrangement of chairs is recommended, and the presence of a table, which may be experienced as an interpersonal barrier, is discouraged.

Facilitator Guides and Client Handouts: In order to conduct MBS sessions, the clinician will need facilitator guides and client handouts. They are located in two places: the guides and handouts that are used in multiple sessions can be found in appendices A, B, C, and D, while the guides and handouts that are specific to individual session topics are located after the session descriptions. Additionally, facilitator guides and client handouts can be downloaded directly from www.newharbinger.com/28531. See the back of the book for more information.

Whiteboard/Flip Chart and Clipboards: During most sessions, the facilitator will write information on a whiteboard or flip chart. Additionally, clients will need a surface to write on, for which clipboards may suffice.

MBS in Residential Treatment Session Content

While there is a rotation of twelve group topics, the role of the "topic of the day" is less pronounced in the residential group session than in the IOP model. Each group session is three hours in length and consists of three sections. Typically, there is a brief break (five to ten minutes) between each section. Acceptable modifications of the MBS format of three-hour, three-section sessions are outlined below.

Section one begins with a two-tiered check-in. During the first tier, clients are asked to share their names, sobriety dates, and something they have done mindfully in the past twenty-four to forty-eight hours. During the second tier, group members are encouraged to share what they want to get out of treatment or what they need from that day's group session. Following that, they are encouraged to share any immediate insights or concerns they might have, including both challenges and successes. This is followed by mindfulness practice, after which group members share their experiences and discuss how mindfulness might improve quality of life. Next, the facilitator provides a brief overview of the MBS model. The overview serves multiple purposes. For new and continuing clients, the overview provides an overarching context within which the topic of the day can make sense. It provides or reinforces the "big picture," and reminds or prompts clients to be attuned to potential blind spots.

Section two begins with an experiential exercise related to things in group members' lives that are important to them. In this exercise, group members start by reflecting on things that are important to them. After a few minutes,

the facilitator instructs group members to write or draw on a blank piece of paper whatever comes to mind. After they are done writing or drawing, the facilitator encourages group members to share their answers with their peers. This provides group members the opportunity to creatively and independently identify and express things in their lives that are important to them.

During the next part of this section, group members are asked to explore the impact that substance use has had on important areas of their lives. Additionally, actions and strategies to improve those life areas are identified in service of preparing group members to be discharged from residential treatment. While this core component of the MBS residential model is a regular focus in MBS residential groups, an individual's perspective will evolve throughout the course of treatment as he or she gains insight, and as situations, plans, and skills develop. Detailed instructions on how to facilitate this section of the group are included in the section "MBS in Residential Treatment Session Outline." The second section of the session concludes with a review of the previous session's topic.

The third section of the session begins with the topic of the day. There are twelve topics in all. The topic of the day is intended to provide focus on a particular subject matter or skill set within the broader MBS model. The third section and session conclude with a debriefing, review, and mindfulness practice.

Modifications for MBS Residential Split Sessions

Modifications can be made to the MBS residential format of three-hour, three-section sessions. One option is to retain the three-section format while extending the break periods. This approach can assume a range of variations. For example, the first section might run in the morning, the second in the afternoon, and the third in the evening. Another type of modification is to provide two 90-minute sections with an extended break, such that there is a morning session and an afternoon session. When providing MBS sessions in two 90-minute sections, the midpoint would usually fall after the "Important Things" reflection exercise (outlined below), although clinical judgment should permit flexibility.

MBS in Residential Treatment Session Outline

The following session outline, along with the topic of the day description and session materials, provides the facilitator with the necessary MBS structure for any particular group session.

PART 1: 45 TO 60 MINUTES

Check-in (20 to 25 minutes)

- Staff and client introductions and check-in (two-tiered check-in)
    - Initial check-in: Name, sobriety date, and something clients have done mindfully in the past 24 to 48 hours
    - Second-tier check-in:
        - What each client wants to get out of treatment or from this group session
        - Immediate insights or concerns that group members may have, including both challenges and successes

Mindfulness practice and discussion (20 to 25 minutes)

- Sitting meditation (5 to 15 minutes)
    - The facilitator may use materials from appendix A for the mindfulness part of the session.
- Open exploration of experience (5 to 15 minutes)
    - Name and experience description (encourage each participant to describe his experience in an

objective, nonjudgmental manner).

- The facilitator may use materials from appendix A for the mindfulness part of the session.

Brief review of MBS model (see appendix B) (5 to 10 minutes)

- Mindfulness and quality of life: Definition and discussion of practical applications
  - Defining mindfulness, acceptance, and commitment and exploring how they influence a person's quality of life.
    - *Mindfulness:* Being present in the immediate ("here and now") situation and observing, nonjudgmentally and without psychological attachment, avoidance, or reaction.
    - *Acceptance*: To acknowledge the realities of a situation while not fighting "what is." Acceptance is a nonjudgmental view that allows clients to move forward on a valued path toward their self-selected goals. Acceptance is not approval; it is acknowledging one's experience.
    - *Commitment:* In MBS, commitment means taking action in service of one's values and goals, regardless of internal experiences (one's thoughts, feelings, cravings, sensations, and so on).

Break (5 to 10 minutes)

PART 2: 45 TO 60 MINUTES

"Important Things" reflection exercise (15 to 20 minutes)

- Facilitator does one of the following:
  - Writes on the board, "What's important to you?"
  - States to client group: "Now I want you to close your eyes, take a few minutes, and see what comes to mind when I ask you 'What's really important for you in your life?'" (2 to 3 minutes of silence)
- Next, the facilitator instructs the group members to write or draw on a blank piece of paper whatever came to mind during the exercise.
  - After group members are done writing or drawing, the facilitator encourages them to share their answers with their peers.

Life domain exploration (20 to 25 minutes)

- On the board, the facilitator writes the following examples of key domains that might influence a person's quality of life: "Financial," "Work," "Social/Friends," "Physical/Health," "Personal Development," "Family/Relationship," "Living Environment," "Emotional," "Community/Volunteer," "Spiritual."
- Next, the facilitator asks group members whether anything has been missed, and then adds members' responses to the list on the board.
- The facilitator distributes the "Life Domain Exploration" client worksheet. Depending on time, the facilitator gives each client one or two worksheets.
  - Clients pick one or two of the domains that are written on the board (one domain per worksheet) and work through each domain using the worksheet.
  - Group members are told that they can choose a new domain or domains—ones that they haven't

addressed in prior sessions—or they can continue with a domain or domains that they find especially important to them.

- The facilitator then processes and reviews the worksheet with the clients.
    - Clients are encouraged to share at least one answer each.
    - Feedback and suggestions can be provided to the client with the client's permission and should be delivered in a tentative matter.

Review of previous topic (10 to 15 minutes)

Break (5 to 10 minutes)

PART 3: 45 TO 60 MINUTES

Topic of the day (30 to 40 minutes): See "Residential Group Topic Rotation" later in this section, and refer to the topic descriptions, facilitator guides, and client handouts that follow.

Debriefing/review (15 to 20 minutes)

- Summary statements by the clinician–group leader.
- Clinician elicits feedback from clients on that day's group.
    - "What are you walking away with?"
    - General feedback: "What made sense? What didn't make sense or was not so useful? What do you need in future groups?" (The purpose is to engage clients and elicit specific feedback for tailoring MBS to their specific needs. Additionally, the clinician may tie clients' needs self-assessments to relapse prevention and future topics.)
    - Values and goals: "What steps have you taken toward your values and goals in recent days or weeks?"
    - Random question (favorite movies, books, music, and so on). For example, "If you could have lunch with two people—dead or alive, real or imaginary—who would they be and what would you ask them?"
- Skill-practice reminder (practice between sessions).
- Other announcements (if any).
- Brief closing mindfulness experience (5 to 10 minutes).

Life Domain Exploration

Life Domain:

How was this area of your life affected by substance use?

What would need to change in this area in order for you to succeed outside of residential treatment?

How could you go about making those changes?

Residential-Group Topic Rotation:

1. Relapse Prevention Plan 1
2. Relapse Prevention Plan 2
3. Relapse Prevention Plan 3
4. Recovery Environment
5. Role-Playing and Drug Refusal Skills, Part One
6. Scheduling and Creating a Routine
7. Recovery Skills and Lapse/Relapse Traps, Part One
8. Spirituality
9. Coping with Emotions and Urges
10. Role-Playing and Drug Refusal Skills, Part Two
11. Recovery Skills and Lapse/Relapse Traps, Part Two
12. Value-Based Living

The following pages are organized by session topics. They contain session descriptions, facilitator guides, and client handouts.

Sessions 1, 2, and 3: Relapse Prevention Plan

Due to the importance and comprehensive nature of the topic "Relapse Prevention Plan," it spans three consecutive sessions. While the overall approach is presented and all three handouts are usually distributed in session 1, sessions 2 and 3 allow for continued processing to enhance awareness and strengthen sobriety strategies. It should also be noted that in addition to these three sessions, relapse prevention (sobriety) planning occurs throughout MBS sessions.

TOPIC OBJECTIVES

Group members will:

- Gain understanding about lapse and relapse processes, including the definitions and the differences
- Understand how lapses and relapses may occur along the path to recovery and that they do not reflect personal failures
- Learn that lapses and relapses are processes, not isolated events
- Identify their individual situations that pose a high risk for relapse
- Develop relapse prevention plans that include strategies to avoid or cope with high-risk situations
- Identify and develop skills to cope with high-risk situations and triggers

INSTRUCTIONS

The facilitator begins this section by explaining the objectives, as outlined above. Next, the facilitator introduces and explains the concepts of lapse and relapse to the group members.

- *Lapse:* This is also known as "slip." A limited use of a substance that doesn't necessarily lead to a full-blown relapse, that is, returning to the previous pattern of addictive behavior.
- *Relapse:* A return to the previous (full-blown) pattern of addictive behavior.

The facilitator then engages in a dialogue with the group members about the lapse and relapse processes (a series of decisions and events that lead to a person's initial use after abstaining). This process can also be understood as someone engaging in "prelapse" decisions and behaviors. The facilitator should also normalize lapse and relapse, and introduce these concepts as processes that sometimes occur during the course of recovery. This can be done by asking the following open-ended questions:

- "How can a lapse or relapse be a process and not just an event?"
- "Can you think of examples from your own experiences where a series of decisions and events led to your lapsing or relapsing?"

Once the concepts are processed and understood, the facilitator distributes the client handouts:

- "High-Risk Events and Scenarios (Experience in Situation)" information sheet (appendix D)
- "Situation Rating Scale and Action Plan" worksheet (located immediately following this session description)
- "Relapse Prevention Plan Worksheet" (located immediately following this session description)

The facilitator explains the MBS model on the "High-Risk Events and Scenarios" sheet.

The facilitator reads the following paragraph to the group as a means of introduction:

Risk of lapse or relapse increases according to the person's experiences in particular situations. Typically people experience situations like math problems, such as 1 + 1 = 2. For example, a person in recovery goes to a wedding where alcohol is available and people are indulging. In the past, he also had indulged in alcohol at weddings, and since the person (1) is at a wedding where alcohol is available (+ 1), the outcome is that he drinks (= 2). In becoming aware of autopilot or programmed responses, a person can develop a transcendent or broader perspective, which can help him experience a situation differently and respond in a value-consistent manner.

The facilitator may give some examples of experience in situation:

- Feeling anxious (experience) at a party where you have access to marijuana (situation) and choosing to use
  - As opposed to feeling anxious (experience) at the party (situation) and leaving or refusing the marijuana that's offered
- Feeling happy (experience) at a wedding where there is an open bar and drinking is encouraged (situation)
  - As opposed to feeling happy (experience) at a wedding where there is an open bar and you are accompanied by a sober support (situation)
- Thinking about a time when things were better (experience) while you are alone and still in contact with old friends who are still using (situation)
  - As opposed to thinking about a time when things were better (experience), after having repaired your relationships with healthy friends and family (situation)

- Feeling lonely (experience) while not having a support network to call (situation)
    - As opposed to feeling lonely (experience) while having a peer support network (situation)

The facilitator then leads group discussion about how someone can experience similar things in different situations and with different outcomes.

The facilitator distributes and introduces the "Situation Rating Scale and Action Plan" worksheet. It is important that the facilitator guides the group members step-by-step through this exercise in order to avoid confusion and misunderstanding. Initially, the facilitator encourages the group members to take the situations they chose from the previous worksheet and describe them on the "Risky Situation" section at the top of page 1 (front) of the worksheet. Then, group members will complete the section titled "How do you expect you might respond in this type of situation?" Next, the facilitator asks group members to rate, on a scale from 0 to 10, how risky the situation is for them. Following this, group members rate the value (or importance) that the situation has for them, also on a scale from 0 to 10. Group members will then indicate a general plan—"Long-Term Avoidance," "Short-Term Avoidance," or "No Avoidance"—by checking the appropriate box:

- *Long-term avoidance* is typically indicated for situations that are of high risk and little-to-no value.
- *Short-term avoidance* is typically indicated for situations that are of high risk and moderate-to-high value.
- *No Avoidance* (with coping plan) is typically indicated for situations that are of high value or have a value rating that outweighs the risk rating.

After completing the first part of the "Situation Rating Scale and Action Plan" worksheet, the group begins the second. On page 2, group members complete either the top or bottom box of the worksheet. Members who chose either "Long-Term Avoidance" or "Short-Term Avoidance" will complete the top box. This box has two parts: "Avoidance Strategies" and "Backup Coping Strategy (if the situation is unexpectedly encountered)." Members who chose "No Avoidance" will complete the bottom box, "Coping Strategies."

After both parts of the worksheet are completed, the facilitator splits the group members into pairs or small groups and has them share their answers (as well as brainstorming, coping, and avoidance strategies) with each other. Once the small group discussions have concluded, the facilitator can open the floor for discussion prior to moving into the debriefing/review.

The facilitator then passes out and reviews with the clients the "Relapse Prevention Plan Worksheet."

- "High-risk situations are situations in which one may experience greater temptations to use."
- Clients are asked to enter their personal "high-risk situations" in the first column of the "Relapse Prevention Plan Worksheet."
- Next, remaining columns in the "Relapse Prevention Plan Worksheet" are completed. Note:
    - Situations rated "high risk" on the "Situation Rating Scale" are generally avoided.
    - Regarding the "Likely Ways of Encountering" column on the "Relapse Prevention Plan Worksheet," the facilitator gives an example, such as going back to the old "neighborhood" where one might encounter a dealer or old user buddies.
    - For situations rated "high value" on the "Situation Rating Scale," coping strategies are generally a focus.
    - Situations that are "high risk" *and* "high value" are more nuanced:
        - They may be temporarily avoided until such time as the person has developed sufficient coping skills and confidence.
        - When not avoided, coping strategies may include the person's being accompanied by a sober support or developing detailed strategies that are practiced through role-playing.

- Topics of sections B, C, and D are referenced, because “Continuing Recovery Plans,” “Positive Supports,” and “Quality of Life Enhancement” are important in relapse prevention planning. These sections may be completed in this session or subsequent to this session.

- The facilitator emphasizes that one’s relapse prevention plan is a “living,” active resource, one that should be reviewed and revised through ongoing experience.

Upon completion of the “Relapse Prevention Plan Worksheet,” the facilitator leads an open discussion about each box in the worksheet and the various relapse scenarios captured by the group members. The facilitator encourages the group members to focus on one situation at a time, starting with the situation on the front of the worksheet.

Note: At the beginning of sessions 2 and 3, the facilitator reviews the MBS relapse prevention (sobriety) planning model (as shown above) and answers any questions. After the review, the open discussion (immediately above) resumes. Accordingly, sessions 2 and 3 are largely interactive processes in the service of enhancing awareness and strengthening strategies.

SESSION MATERIALS

Client Handouts:

- High-Risk Events and Scenarios (Experience in Situation) (appendix D)
- Situation Rating Scale and Action Plan
- Relapse Prevention Plan Worksheet

Situation Rating Scale and Action Plan

Name: _____ Date: _____

Risky Situation (describe): _____

_____

_____

_____

_____

How do you expect you might respond in this type of situation? _____

_____

_____

_____

_____

Situation Risk Rating

1	2	3	4	5	6	7	8	9	10
Little or no risk									Very high risk

Situation Value Rating (How important is it for you to be in this type of situation?)

1	2	3	4	5	6	7	8	9	10
Little or no importance									Very important

Plan (check one):

_____ 1. Long-Term Avoidance (situation is high risk and of little-to-no value)

_____ 2. Short-Term Avoidance (situation is high risk and of moderate-to-high value)

_____ 3. No Avoidance (situation is low-to-moderate risk and of moderate-to-high value)

Avoidance Strategies (if you chose option 1 or 2 above):

_____

_____

_____

_____

Backup Coping Strategy (if the situation is unexpectedly encountered):

_____

_____

_____

_____

Coping Strategies (if you chose option 3 above):

_____

_____

_____

_____

Relapse Prevention Plan Worksheet

Client: _____ Date: _____

A. High-Risk Situations

High-Risk Situation	Likely Ways of Encountering	How to Avoid	Escape Strategies	Coping Strategies
1.				
2.				
3.				
4.				

B. Continuing Recovery Plans: Planning for Transition to Lower Level of Care (Co-Occurring Mental Health Problems; Peer Fellowships, such as AA, NA, and so on)

_____

_____
_____
_____
_____
_____

C. Positive Supports: Friends and Family

_____
_____
_____
_____
_____
_____

D. Quality of Life Enhancement: Occupy Time Doing Enjoyable, Healthy Things (such as Leisure, Exercise, Diet, Meditation)

_____
_____
_____
_____
_____

Session 4: Recovery Environment

For this topic, “recovery environment” refers to people, places, activities, and things.

TOPIC OBJECTIVES

Group members will:

- Identify characteristics of the environment that are conducive to recovery and characteristics of the environment that are not conductive to recovery.
- Identify qualities of the environment that give them energy and strength, as opposed to qualities that take away energy (“drain”) them.
- Identity specific examples in their personal lives of healthy changes they would like to make.

INSTRUCTIONS

The facilitator begins this section by explaining the objectives, as outlined above.

Next, the facilitator distributes the worksheets to the group members. The facilitator should instruct the group members to complete one page at a time (there are three pages in all, two for the first worksheet and one for the second). The facilitator should pause for discussion after each page.

Once each page has been completed and discussed, the facilitator assesses for comprehension and further questions.

SESSION MATERIALS

Client Handouts:

- Recovery Environment
- Improving Recovery Environment

Recovery Environment

Current Environment (Outside of Residential Treatment Setting)
People whom I tend to spend my time with:

Qualities of people that give me energy or strength:

Qualities of people that drain my energy or strength:

Places where I tend to spend my time:

Qualities of environments that give me energy or strength:

Qualities of environments that drain my energy or strength:

Activities and things in my life that I spend time with or engage in:

Qualities that give me energy or strength:

Qualities that drain my energy or strength:

Recovery Environment:

Qualities of people whom I would like to have in my life:

Qualities of the environments I would like to spend my time in:

Qualities of activities I would like to engage in:

Improving Recovery Environment

Where and how can I meet people I would like to have in my life?

Where do I want to spend my time (specific places)?

What do I want to spend my time doing?

Session 5: Role-Playing and Drug Refusal Skills, Part One

TOPIC OBJECTIVE

Group members will improve drug refusal skills.

INSTRUCTIONS

The facilitator begins this section by explaining the objective, as outlined above.

Next, the facilitator orients the group to the topic of "Role-Playing and Drug Refusal Skills." Drug refusal skills include the ability to assertively and effectively refuse drugs.

Note: Role-playing and drug refusal skills are addressed in two separate sessions (session 5 and session 10), with different skills discussed and practiced in each session.

The facilitator distributes the "Role-Play: Drug Refusal Skills" client handout and instructs the group members to write down each skill on the handout and to take notes in the designated areas.

The facilitator then writes the following skills on the whiteboard:

- Skill 1: Be firm, matching your tone to your words.
- Skill 2: Make eye contact with the person.
- Skill 3: Use body posture to convey conviction (stand tall, in a closed versus open stance).
- Skill 4: Be honest with the person. Examples include:
    - "I cannot afford to use; I have too much at stake."
    - "I am on medication, and I can't drink."
- Skill 5: Let the person know that there are consequences. Examples include:
    - "My (loved ones, partner, significant other, or children will leave; I can't take that chance."
    - "I will lose my _____ (job, driver's license, or house)."
    - "I will go to jail if I get another DUI."
- Skill 6: If an encounter with the person is unexpected, keep the conversation to a minimum or avoid physical contact with the person altogether.

Once each skill is discussed and understood, the facilitator helps the group members practice the skills:

- Initially, the facilitator asks for a volunteer in order to provide a practice example of a role-play. The facilitator and the group member act out a brief scenario, during which one of them refuses to consume drugs or alcohol.
    - The facilitator can provide an example or ask the group for one. Examples include:
        - An old friend who is still using stops by and pulls out some drugs or alcohol for the two of you.
        - You run into your old drug dealer.

        - Your friends get in your car and pull out a blunt and light up.

- After the practice scenario, the facilitator divides the group members into pairs. The facilitator has each pair identify who will be first to play the role of the "using person" and who will play the role of the "person practicing refusal skills." Group members are given five minutes or so to practice skills 1, 2, and 3 together. Prior to beginning the first scenario, the facilitator encourages group members not to push too hard and to back off if someone becomes upset or too uncomfortable.
- Once the first round is completed, the facilitator has the pairs switch roles and repeat.
- The group members then repeat the above process for skills 4, 5, and 6.
- Each pair is then encouraged to play out a scenario or two (depending on time) in front of the larger group.
- The group members' experiences are processed and discussed at the end of the skills practice. Questions for discussion might include:
    - "How did it feel to play the role of a using friend?"
    - "How did it feel to practice the refusal skills?"
    - "Which skill did you find most helpful and why?"
    - "Which skill did you find least helpful and why?"

SESSION MATERIAL

Client Handout:

- Role-Play: Drug Refusal Skills

Role-Play: Drug Refusal Skills

Today's Practice Skills:

1. _____

Notes:

2. _____

Notes:

3. _____

Notes:

4. _____

Notes:

5. _____

Notes:

6. _____

Notes:

Session 6: Scheduling and Creating a Routine

TOPIC OBJECTIVES

Group members will:

- Learn how to schedule and create a routine.
- Practice creating a schedule that is balanced and conducive to valued living.
- Develop a tentative schedule for daily life that they can use once they leave residential treatment.

INSTRUCTIONS

The facilitator begins this section by explaining the objectives, as outlined above.

Next, the facilitator passes out the "Pretreatment Weekly Schedule" client handout and encourages group members to re-create (as best they can) their daily and weekly schedules prior to entering treatment.

Once group members have re-created their pretreatment schedules, the facilitator helps them identify gaps and high-risk times of day (group members can highlight, circle, or check off high-risk time periods and activities).

The facilitator then passes out the "Posttreatment Weekly Schedule" handout and leads a discussion on how to create a balanced routine by eliciting from the group members activities they would like to insert into their posttreatment schedules (or activities that they want to spend more time engaging in).

Next the facilitator encourages group members to create their potential posttreatment schedules.

Once this is completed, the group members are encouraged to share their schedules and highlight changes from the pretreatment to posttreatment schedules.

SESSION MATERIALS

Client Handouts:

- Pretreatment Weekly Schedule
- Posttreatment Weekly Schedule

Pretreatment Weekly Schedule

	Morning	Afternoon	Evening
Mon.			
Tue.			
Wed.			
Thur.			
Fri.			
Sat.			
Sun.			

Posttreatment Weekly Schedule

	Morning	Afternoon	Evening
Mon.			
Tue.			
Wed.			
Thur.			
Fri.			
Sat.			
Sun.			

Session 7: Recovery Skills and Lapse/Relapse Traps, Part One

TOPIC OBJECTIVE

Group members will learn about and identify traps commonly encountered by people in recovery that may lead them to lapse or relapse.

INSTRUCTIONS

The facilitator begins this section by explaining the objective, as outlined above.

Next, the facilitator passes out the “Lapse/Relapse Trap: Decision Making” worksheet and asks for a volunteer to read the paragraph at the top of the worksheet.

Once the paragraph is read aloud by the group member, the facilitator provides the group with the following example:

Intellectual	Put off school	Save money Less stress More free time	Hard time finding work Complacency Stagnation Relapse

After the example is discussed and understood, the facilitator encourages group members to complete the worksheet. Once this is completed, each member shares his answers with the group.

Next, the facilitator distributes the “Lapse/Relapse Trap: Identifying Common Thinking Patterns” worksheet.

On the whiteboard, the facilitator also writes definitions of fusion, defusion, and autopilot.

- *Fusion:* The act of becoming one with or attached to a thought, belief, or emotion
- *Defusion:* Taking an observational, detached perspective in relation to a thought, belief, or emotion
- *Autopilot:* Unmindful, habitual, routine behavior

Lastly, the facilitator encourages group members to read the instructions and complete the worksheet. Additionally, the facilitator briefly speaks to group members about using mindfulness and awareness as coping skills in recovery. Through being aware of thinking patterns, individuals can learn to identify, let go of, and not act on thoughts that are not in line with their values and goals.

SESSION MATERIALS

Client Handouts:

- Lapse/Relapse Trap: Decision Making
- Lapse/Relapse Trap: Identifying Common Thinking Patterns

Lapse/Relapse Trap: Decision Making

By living life in a mindful manner, you can make decisions from a point of clarity. However, when you make decisions on autopilot, you can take actions that may seem harmless at first but lead to negative consequences in the long run. Below, for each category, list decisions that you have made in the past or could make in the future that resulted in short-term benefits and long-term negative consequences.

	Decision →	Short-Term Benefits →	Long-Term Consequences
Financial			
Work			
Social			
Physical Health/ Wellness			
Personal Development			
Emotional			
Spirituality			

Lapse/Relapse Trap: Identifying Common Thinking Patterns

When an individual is on autopilot and not being mindful, he or she may be more likely to fuse with and act on thoughts that will result in nonvalued behavior (for example, acting impulsively or without thinking). Furthermore, individuals who are on autopilot may forget that they have the choice to defuse from nonvalued thinking patterns and impulses such as rationalization, denial, justification, and positive expectation.

A defusion technique that may enhance one's ability to be mindful is "Naming Your Addiction." In the space below, provide names for and quotes from your addiction. Afterward, circle or write in the blank spaces the matching thinking patterns. *For example:* Name, "Mr. Addiction"; Quote, "I can have just one!"; circle "Rationalization" or "Denial"; a person can say, "Oh, there's Mr. Addiction again. He is so in denial. He loves those rationalizations."

Name	Quote	Thinking Pattern
		Rationalization Denial Justification Positive Expectation
		Rationalization Denial Justification Positive Expectation
		Rationalization Denial Justification Positive Expectation
		Rationalization Denial Justification Positive Expectation

Session 8: Spirituality

TOPIC OBJECTIVES

Group members will:

- Clarify what spirituality is to them and the importance of spiritual matters in their lives.
- Identify and potentially address barriers to spiritual practice (although this may not happen and is certainly not mandatory).

INSTRUCTIONS

The facilitator begins this section by explaining the objectives, as outlined above.

The topic section of the "Spirituality" session should also begin with the facilitator encouraging the group members to practice openness, acceptance, and patience with both themselves and others. Some may struggle with or have differing opinions related to spirituality and religious practices, and it is important to establish an open and accepting milieu from the beginning of group through the end.

Next, the facilitator distributes the "Spirituality" worksheet and assists each group member in developing an individual definition of spirituality. The facilitator can do this by having the group members answer questions 1 and 2 on the worksheet ("Personal definition of spirituality" and "Background and orientation").

After group members discuss their initial answers, the facilitator instructs them to complete the remaining questions on the worksheet, including goals, barriers, and ways to overcome specific barriers, as well as tips for establishing and maintaining a spiritual practice.

The facilitator encourages group members to rank the importance of each goal, their confidence in completing the goal, and their motivation to work toward it (on a scale from 0 to 10, in which 0 means low and 10 means high).

Once this is completed (some may need help if they struggle with the concept of spirituality), the group can share and discuss their answers in an open and relaxed manner.

SESSION MATERIAL

Client Handout:

- Spirituality

Spirituality

Personal definition of spirituality:

Background and orientation:
My sense of spirituality comes from…

Goals:
1.
Importance:
Confidence:
Motivation:

2.
Importance:
Confidence:
Motivation:

3.
Importance:
Confidence:
Motivation:
Anticipated environmental and personal constraints or barriers:

Possible ways to overcome environmental and personal constraints or barriers:

Helpful tips for engaging in and maintaining a spiritual practice:

Session 9: Coping with Emotions and Urges

TOPIC OBJECTIVE

Group members will learn and practice “urge surfing” as a method of coping with urges and cravings.

INSTRUCTIONS

Important Note: This topic section may take longer than the others to facilitate. Those who are facilitating this topic should plan for an extra 20 to 30 minutes for practice, processing, and discussion.

The facilitator begins this section by explaining the “Topic Objective” as outlined above.

Next, the facilitator passes out the “Coping with Cravings: Confidence Ruler” and asks group members to complete it.

On this worksheet, group members are asked to rate their “craving level” and “confidence in coping without using” on scales of 0 to 10. (Specific instructions are provided on the worksheet.)

The optional “Follow-Up Questions” on page 2 of the worksheet are designed to help group members increase their confidence in coping without using, identify current strengths and resources, and explore further actions that they might take to enhance confidence and sobriety.

Next, the facilitator introduces the topic of “Coping with Internal Experiences.” The facilitator can introduce this part of the session by directly reading the following script or by independently introducing the concepts within:

> *Through labeling what happens inside our minds and bodies (emotions, feelings, sensations, thoughts, urges, cravings, and so on) as “internal experiences,” we avoid labeling them as negative or positive.*
>
> *Through accepting and opening up to our internal experiences (as opposed to struggling with, fighting, and avoiding them, which only provides temporary relief), it is believed that we will suffer less and experience a more fulfilling life. This idea also relates to the concept of “urge surfing,” which will be discussed and practiced later in the session.*

After introducing the topic, the facilitator should gauge comprehension and inquire about the internal experiences that the group members have struggled with over the course of their lives. The facilitator then inquires about strategies that the group members have used to cope with internal experiences in the past, and the long-term effectiveness (or in most cases ineffectiveness, such as the impact of alcohol and drug use and other control methods on internal experiences) of these strategies.

It is important for the facilitator to be empathetic, to normalize efforts to control emotions, and to avoid power struggles during this discussion, because it may elicit strong reactions from certain group members. The facilitator should also encourage group members to continue using coping skills that have worked for them in the past (such as keeping busy and distracting themselves) while they learn and practice strategies that may be more helpful in the long term.

The facilitator then inquires about the emotional and physical experiences one goes through when experiencing a craving or an urge to act impulsively. This discussion can focus on both drug and alcohol cravings and other seemingly impulsive human behaviors (such as acting out through anger, sex, eating, and so on). The facilitator then likens internal experiencing (emotionality, feelings, sensations, thoughts, urges, cravings, and so on) to a wave. (It is helpful to draw, on the whiteboard, a bell-shaped curve or a wave that peaks and falls.) The facilitator then reads the following script or independently introduces the concepts within:

> *Once the internal experience begins to rise and peak, many individuals use strategies to cut off the wave (fighting, avoiding, distracting, giving in, and so on) that often work temporarily but strengthen the wave in the long term* (the facilitator can then draw a second wave that gets cut off before it peaks but then shoots right back up shortly after being cut off). *An example of this would be someone who uses his drug of choice when experiencing urges and cravings. The urge or craving may be cut off after the initial use of the substance but will inevitably return once the drug wears off.*
>
> *Many individuals get caught up in this cycle and often live their lives from distraction to distraction, in service of not feeling or experiencing. Urge surfing is a strategy that encourages the individual to slow things down, breathe, and relax into the craving or urge to act impulsively. Instead of using control strategies, one eventually is able to learn that internal experiences are temporary and will come and go with little effort from the individual. Despite our desire to control our internal experiences, our efforts to do so often increase our suffering in the long term. What we have control over is how we observe and*

respond to internal experiences, not the experiences themselves.

The facilitator then gauges comprehension and leads the group members in an urge surfing exercise. (See appendix A, "Mindfulness and Urge Surfing," for a script and further explanation.) As indicated in the script, group members are encouraged to avoid using the most challenging scenario they can think of for the exercise. Instead, group members should choose a scenario that they feel comfortable with and will not be overwhelmed by. Urge surfing exercises should not be practiced directly before a break due to the strong reaction it may elicit within certain individuals. The exercise itself should be followed by an extended period of open processing (especially if members report intense experiences) and a restorative meditation.

Lastly, the facilitator passes out the "Coping with Cravings: Confidence Ruler" and asks group members to complete it again. Results on this worksheet will serve as a postmeasure when they are compared with results on the same tool used at the beginning of the session topic.

Prior to ending the session, the facilitator distributes and reviews the "Coping with Internal Experiences" and "Urge-Surfing Instructions" worksheets with the group members.

SESSION MATERIALS

Client Handouts:

- Coping with Cravings: Confidence Ruler
- Coping with Internal Experiences
- Urge-Surfing Instructions

Coping with Cravings: Confidence Ruler

Name: _____ Date: _____

Please rate yourself on:

- Alcohol or other drug (substance) craving level
- Confidence in coping with cravings without using

If you are experiencing cravings for more than one substance, please rate for each substance.

Specify substance: _____

Craving Level

On a scale from 0 to 10, with 0 being "minimal craving" and 10 being "maximum craving," please rate your level of craving to use over the past three days.

Minimum Cravings										Maximum Cravings
0	1	2	3	4	5	6	7	8	9	10

Confidence

On a scale from 0 to 10, with 0 being "not confident at all" and 10 being "very highly confident," how confident are you that you can cope with your cravings without using?

Minimum Confidence										Maximum Confidence
0	1	2	3	4	5	6	7	8	9	10

Coping with Cravings Confidence Ruler: Optional Follow-Up Questions

Why are you at a rating of _____ (stated level of confidence) and not 0?

_____

_____

_____

_____

What has helped you to reach this level of confidence in coping without using?

_____

_____

_____

_____

What would it take for you to increase your confidence level?

_____

_____

_____

_____

Coping with Internal Experiences

Internal Experiences: Anything that happens inside our minds and bodies (emotions, feelings, sensations, thoughts, urges, cravings, and so on).

Control Strategies: Efforts people put forth to try to avoid undesired internal experiences. This may occur through one's behavior or through one's thinking.

Behavioral control strategies may involve avoiding uncomfortable situations, such as social events or situations in which the person risks experiencing failure or rejection, or procrastinating. Mental control strategies include processes that allow us to avoid undesired internal experiences or decrease their intensity. Examples include daydreaming and mental distraction. Despite our desire to control our internal experiences, our efforts to do so often increase our suffering in the long run. What we have control over is how we observe and respond to those experiences, not the experiences themselves. Through accepting and opening up to whatever happens inside of our minds and bodies (as opposed to struggling with, fighting, and avoiding them, which provide only temporary relief), we can allow ourselves to suffer less and experience a more fulfilling life. This idea relates to the concepts of mindfulness and "urge surfing," the latter being a mindfulness-based technique for coping with urges and cravings to act impulsively. Internal experiences can be compared to an ocean wave, which will rise, peak, and eventually fall. Many individuals use strategies to cut off the wave (fighting, avoiding, distracting, giving in, and so on), which often work temporarily but strengthen the wave in the long term. An example of this would be someone who uses his drug of choice when experiencing urges and cravings. The urge or craving may be cut off after the initial use of the substance, but will inevitably return once the drug wears off (often it will grow stronger).

Many individuals get caught up in this cycle and often live their lives from distraction to distraction, in service of not feeling or experiencing. Urge surfing is a strategy that encourages the individual to slow things down, breathe, and relax into the craving or urge to act impulsively. Instead of using control strategies, one eventually is able to learn that internal experiences are temporary and will come and go with little emotional and physical effort.

Urge-Surfing Instructions

When you are experiencing a craving or an urge to act impulsively:

- Notice how the internal experience is like a wave: it rises, it peaks, and it falls. This pattern continues. Stay with the experience. Observe the waves. Even though you are not reacting, the cravings and urges fall; they subside. They may rise again and subside again. You are like a surfer riding the waves. You may enjoy the freedom of observing and not needing to react.
- You may notice thoughts, emotions, or physical sensations that come…and go.
- Experience what a craving or urge is while making the choice to be mindful, rather than reacting. Some cravings and urges are more intense than others. Some are like small waves, while others are more like ocean or tidal waves.
- Notice that you can be present and not react, that you can experience cravings and urges without reacting.
- After the designated period of time, open your eyes, if they were closed, and bring your attention back into the room.

Session 10: Role-Playing and Drug Refusal Skills, Part Two

TOPIC OBJECTIVE

Group members will improve drug refusal skills.

INSTRUCTIONS

The facilitator begins this section by explaining the "Topic Objective," as outlined above.

Next, the facilitator orients the group to the topic of "Role-Playing and Drug Refusal Skills." This topic is broken into two separate sessions with different skills discussed and practiced during each group session.

The facilitator then writes the following skills on a whiteboard:

- Skill Set 1: Keeping Valued Relationships
  - Suggesting Alternatives
    - "Let's watch a movie."
    - "Let's go play video games."
    - "Let's go play basketball."
    - "You mentioned your mom needs some help; let's go help her out."
  - Using Humor
    - "I would get caught, so I will have to move in with you when my wife kicks me out for using."
    - "No thanks; the last time I used just once, it lasted a year."
  - Asking for Help
    - "It's really important to me to stop using; can you help me by not doing it around me?"
    - "Your support would really mean a lot; I gave up drugs, not you."
- Skill Set 2: Ending Unwanted Relationships

- Be brief or rude, but not cruel; burn the bridge without provoking violence or confrontation.
- Be insistent (increase intensity if the person is persistent).
- Leave the situation.

The facilitator then distributes the "Drug Refusal Skills 2" worksheet. The group members are then encouraged to write the designated skills on their worksheets.

Once this is discussed and understood, the facilitator helps the group members practice the skills:

- Initially, the facilitator should ask for a volunteer in order to provide a practice example of a role-play. The facilitator and the group member can act out a brief scenario, during which one of them refuses to consume drugs or alcohol.
- The facilitator can provide an example or ask the group for one. Examples include:
  - An old friend who is still using stops by and pulls out some drugs or alcohol for the two of you.
  - You run into your old drug dealer.
  - Your friends get in your car, pull out a blunt, and light up.
- After the practice scenario, the facilitator divides the group members into pairs and asks each pair to identify who will be first to play the role of the "using person" and the role of the "person who uses refusal skills." Give them five minutes or so to practice skill set 1 (see above). Prior to beginning the first scenario, the facilitator should encourage the group members to not push too hard and to back off if someone becomes upset or too uncomfortable.
- Once the first round is completed, the facilitator should have the pairs switch roles and repeat.
- The facilitator should then repeat the above process for skill set 2 (see above).
- Each pair should then be encouraged to play out a scenario or two (depending on time) in front of the larger group.

SESSION MATERIAL

Client Handout:

- Drug Refusal Skills 2

Drug Refusal Skills – 2

Today's Practice Skills:

1. Keeping valued relationships:

_____
_____
_____
_____
_____
_____

2. Ending unwanted relationships:

_____
_____
_____
_____
_____
_____

Notes

_____
_____
_____
_____

Session 11: Recovery Skills and Lapse/Relapse Traps, Part Two

TOPIC OBJECTIVE

Group members will identify how procrastination and complacency can negatively affect recovery.

INSTRUCTIONS

The facilitator begins this section by explaining the "Topic Objective," as outlined above.

Next, the facilitator briefly reviews the information and concepts contained within the initial "Recovery Skills and Lapse/Relapse Traps" session (although it is an open-group format, the clients will benefit from a brief intro to the concepts prior to or after reviewing them for the first time).

Once the initial session is reviewed and understood, the facilitator passes out the "Relapse Trap: Complacency and Procrastination" worksheet. The facilitator then asks for a volunteer to read "Important Point 1."

After "Important Point 1" is read out loud and understood, the facilitator leads the group in a discussion based on the following questions:

- "How has motivation or a lack of motivation affected your life in the past?"
- "How do you see motivation or a lack of motivation affecting your life in the future?"
- "What are some ways you could overcome issues with motivation and procrastination?"

This process should be repeated for the remaining points on the worksheet. The following questions can be used for point numbers 2 and 3:

- Important Point 2
    - "How did fear or avoidance affect your life and ability to accomplish goals in the past? In what ways may fear and avoidance affect your life in the future?"
    - "What are some ways you could overcome issues related to fear and avoidance?"
- Important Point 3
    - "How did coping-skill procrastination affect your life in the past?"
    - "How do you see coping-skill procrastination affecting your life in the future?"
    - "What specific skills do you have or anticipate having a hard time practicing on a regular basis?"
    - "What are some ways you could overcome issues with coping-skill procrastination?"

SESSION MATERIAL

Client Handout:

- Relapse Trap: Complacency and Procrastination

Relapse Trap: Complacency and Procrastination

Things to consider:

- Important Point 1: Motivation
- Important Point 2: Living vs. Existing
- Important Point 3: Coping-Skill Procrastination (using skills only when in a crisis or a high-risk situation)

Important Point 1: Motivation

Another common trap that people may find themselves falling victim to is the "I will do it when I feel like it" trap. Motivation may or may not come to you initially, which requires you to take valued action first and let your feelings, thoughts, and sense of desire and fulfillment catch up to you.

Important Point 2: Living vs. Existing

Many individuals (both inside and out of recovery) do not take the time to define how they want their lives to be (values and goals) and take the actions to get there. In recovery, individuals who fall victim to this trap are often referred to as "dry drunks" or "dry users." This type of outlook or lifestyle can be referred to as "basic existence." If you are breathing—your heart is beating and your brain is working—you can exist while being totally dissatisfied and unfulfilled in life.

Living life requires us to take healthy risks and face fears in order to live in a way that is in line with our values and goals (for example, seeking relationships while facing rejection, seeking success and accomplishments while risking failure and embarrassment, and so on). The more we base our lives on avoidance, procrastination, and complacency, the more we risk living a very unfulfilling life (and in the case of addiction, experiencing lapse or relapse).

Important Point 3: Coping-Skill Procrastination (using skills only when in a crisis or a high-risk situation)

Like any skill in life, coping skills require repeated practice in order to determine what works and what does not. With that said, it is important to practice and use coping skills as needed and to be mindful of them on a daily or regular basis. If you wait to use them in a crisis or in high-risk situations, they are less likely to work or seem like viable options. This often results in the individual abandoning effective coping skills and losing self-esteem and worth due to feeling as if he or she has failed in some way.

Session 12: Value-Based Living

TOPIC OBJECTIVE

Group members will learn a value-based versus avoidance-based model for living.

INSTRUCTIONS

The facilitator begins this section by explaining the objectives, as outlined above.

The facilitator then provides the "Value-Based Living Presentation" (see appendix C).

SESSION MATERIALS

Facilitator Guide:

- Value-Based Living Presentation (see appendix C)

Client Handout:

- Value-Based Living Flowchart (refer to appendix C)

APPENDICES

Appendix A

Mindfulness and Urge Surfing

Mindfulness is "moment to moment nonjudgmental awareness, cultivated by paying attention" (Kabat-Zinn, 2007).

Mindfulness is a process of being in the present moment. One simply observes—is aware of—what is going on, using all the senses (taste, touch, smell, hearing, and vision). Mindfulness is not thinking, deliberating, conceptualizing, strategizing, or multitasking. Mindfulness is experiencing what is in the here and now without judgment. While relaxation sometimes occurs during mindfulness practice, it does not always occur, and it is not a primary purpose of the practice. Enhancing awareness *is* a primary intent of mindfulness.

Although mindfulness techniques were discovered and practiced in both Eastern and Western religions, as used in this curriculum mindfulness is a psychological technique and does not require any particular religious beliefs.

Incorporating mindfulness practice into everyday living may help the person to heighten awareness, reduce painful attachments, be less judgmental, and reduce impulsivity. Research provides evidence that mindfulness practice is effective in reducing substance abuse and substance relapse and in reducing anxiety and depression.

Mindfulness can be either a formal practice, as in meditation, or an informal practice in everyday life. In this curriculum, mindfulness meditation is practiced in every session, while informal practice is modeled and coached for everyday living.

Mindfulness Instructions

The clinician will:

- Orient clients to what mindfulness meditation is.
- Decide how long the mindfulness session will be, usually between five and fifteen minutes in a therapy session. (A simple kitchen timer may be used to allow the facilitator to model the practice along with the group.)

INSTRUCTIONS TO CLIENTS:

1. *Sit comfortably in a chair or on a mat or pillow with your back relatively straight. If sitting is uncomfortable, you may lie down. Your hands may be placed flat on your lap, palms up or down.*
2. *You may either close your eyes or keep them open with a "soft gaze." A soft gaze should be directed at an object that will not be "thought provoking," such as a place on the floor, the wall, or the table in front of you. Eye contact with others should be avoided.*
3. *Take a moment to find a comfortable position.*
4. *Mindfulness is being present in the immediate ("here and now") situation, observing nonjudgmentally and without psychological attachment, avoidance, or reaction.*
5. *Mindfulness practice begins with focusing attention on your breathing.*
6. *Let your breathing "do its own thing." Feel the breath enter through your nostrils with each inhalation and exit with each exhalation. Don't try to control it. It may or may not become faster or slower than it normally is; that's okay. Just let your breathing take its own natural course.*
7. *Focus on your breathing. You may focus on a specific breathing sensation, such as the nostrils as the air passes through or your abdomen rising and falling.*

8. *Your focus will likely drift away from your breathing. You may notice sensations from your body and in your environment: sounds, temperature, the feeling of sitting on the chair, and so on. You may notice thoughts or emotions coming and going. This is normal.*

9. *When you notice something other than your breathing, just acknowledge that and gently return to your practice. Do not become attached to it and do not try to "push" it away; just let it be and refocus on your breathing. This is the process of mindfulness.*

10. *The silent practice will begin now and will continue for* (designated number of) *minutes.*

11. (After one or two minutes of silence, the facilitator may gently remind group members of the following.) *If your mind has wandered, this is normal. Just acknowledge it and bring your attention back to your breathing.*

12. (When the designated period of time comes to an end, the clinician instructs group members as follows.) *Open your eyes, if they were closed, and bring your attention back into the room.*

13. (The facilitator will ask group members to share their experiences, encouraging them to remain open and nonjudgmental.) *Practice is just "practice." There is no such thing as "perfect." Regardless of what your mind does, continue to gently bring your attention back to the present.* (The facilitator may encourage inquiry by asking open questions, sometimes followed by reflections, as follows.)

    1. *What was the experience like for you?*

    2. *When your mind drifted, how did you become aware of this? What did you do to bring it back to focusing on your breathing?*

    3. *What other kinds of challenges did you face?*

    4. *Did you notice any smells, tastes, sounds, or bodily sensations? Did it keep your attention, or were you able to let it go and refocus on your breathing?*

14. (The facilitator informs clients as follows.) *Mindfulness takes practice and effort. It is a skill to be developed over time. In a way, mindfulness practice can be compared to weight lifting; each person starts with a different set of weights and works to increase strength.*

15. (The facilitator may convey the following to clients.) *Key concepts of mindfulness include being less attached to judgments and to thoughts and feelings while remaining focused on the here and now.*

The above script is an example of a particular mindfulness exercise wherein the facilitator is asking the person to be in the present moment by focusing on breathing. Other mindfulness techniques might ask the person to focus on particular sensations, sounds, or other experiences. Additionally, mindfulness can be integrated into everyday life activities. One example is that of washing dishes. Objects of a person's awareness may include the feeling of soap and water on the skin, the feeling of the item as it is being washed, the coolness of the air, and so on, as opposed to judgments about how he hates doing dishes. Common assignments for group members to practice mindfulness skills in everyday life are:

- Washing dishes
- Vacuuming or sweeping the floor
- Making a bed
- Taking a shower
- Exercising
- Eating

- Lying in bed
- Walking

It is recommended that clinicians routinely practice mindfulness for their own benefit and to increase the effectiveness of their work with others.

Urge Surfing: An Application of Mindfulness

Being mindful—being aware of the present without reacting—may be especially useful when the here and now includes experiencing cravings and urges to act in a specific way, particularly to indulge in behavior that is anticipated to provide immediate gratification or relief, even though doing so may be counter to one's well-being in the intermediate or long term. Cravings and urges tend to follow a pattern of entering consciousness, increasing, peaking, and finally subsiding, at least until the next one comes. Alan Marlatt coined the term "urge surfing" (Marlatt et al., 2002) to describe this process. Being mindful of this process moment to moment is akin to "riding a wave." While riding the wave, the person has no control over the wave itself. What she does have control over is how she responds to it. With openness and awareness, one can observe and not react or be controlled by the intensity of the waves.

Urge-Surfing Instructions in This Curriculum

Decide how long the urge surfing session will be, usually between fifteen and twenty minutes.

1. *Sit comfortably in a chair or on a mat or pillow with your back relatively straight. If sitting is not comfortable, you may lie down.*
2. *Close your eyes. Focus your attention for a few minutes on your breathing.*
3. *Now, think of a type of situation in which you might feel an urge to react impulsively, such as with substance use or an angry outburst. Select a situation that is challenging but not one that is overwhelming. The purpose of the exercise is to experience and observe the urge to react while not reacting, to have a gentle and curious approach. You may select a type of situation you have experienced in the past, one in which you experienced cravings or urges to do something that is inconsistent with your values and goals.*
4. *It is important to note that some of you may not experience an actual urge or a craving during this exercise, which is not uncommon. Please attempt to follow the exercise as best you can and use it as a "practice round" for real-life experiences.*
5. *Imagine that you are in that situation and that you do not engage in the reactive behavior.*
6. *If in the course of this exercise, the situation begins to feel overwhelming, pick a less-challenging situation. If the situation is overwhelming, you may open your eyes for a while, move around in your seat, and focus on your immediate sensations, such as the feeling of your body sitting in the chair or your feet on the floor, sounds in the room, and so on. You may close your eyes again as you feel more comfortable.*
7. *Now, the exercise will begin.*
8. *In your mind's eye, picture the challenging situation. I'll give you a minute or two of silence while you imagine the situation.* (The facilitator allows the group members to have a minute or two to picture the situation.)
9. *You may notice thoughts, emotions, or physical sensations that come…and go.*
10. *If cravings or urges occur, just observe them.*
11. *Notice how the cravings and urges are like waves: they rise, they crest, and they fall. This pattern continues. Stay with the experience. Observe the waves. Even though you are not reacting, the cravings and*

urges fall; they subside. They may rise again and subside again. You are like a surfer riding the waves. You may enjoy the freedom of observing while not needing to react.

12. *Some cravings and urges are more intense than others. Some are like small waves, while others are more like ocean or tidal waves.*

13. *Notice that you can be present and not react, that you can experience cravings and urges and not react.*

14. (After the designated period of time, give the following instruction.) *Now, you may let go of the scenario and return your attention to breathing.*

15. (After another minute or two, give the following instruction.) *Open your eyes, if they were closed, and bring your attention back into the room.*

16. (Counselor processes the group's experiences with them.) *What was your experience during the urge surfing exercise? What kinds of emotions did you experience? How did you respond to the emotions? What kinds of thoughts did you experience? How did you respond to those thoughts? What kinds of sensations did you experience? How did you respond to them?*

Appendix B

Overview of the MBS Model and Principles

BRIEF REVIEW OF MBS MODEL (TEN TO FIFTEEN MINUTES)

- MBS is a process of *awareness enhancement* through mindfulness and self-reflection.
- MBS helps individuals explore and clarify what is important to them (their values) and helps them develop value-based goals so that they can get to where they really want to go!
- MBS integrates aspects of acceptance and commitment therapy (ACT), motivational interviewing, and relapse prevention therapy.
    - ACT is an evolution of CBT (cognitive behavioral therapy) and involves accepting "what is" and committing to value-informed living.
        - Describe the experiential avoidance model: Acceptance is a natural, healthy alternative to experiential avoidance.
    - The facilitator may define acceptance and commitment:
        - *Acceptance:* To acknowledge the realities of a situation while not fighting "what is." Acceptance is a nonjudgmental view that allows clients to move forward on a valued path toward their self-selected goals. Acceptance is not approval; it is acknowledging one's experience.
        - *Commitment:* In MBS, commitment means taking action in service of one's values and goals, regardless of internal experiences (one's thoughts, feelings, cravings, sensations, and so on).
    - MBS is informed by motivational interviewing, a collaborative approach that respects the person's autonomy: we work together toward a common purpose. Change is not imposed; it comes from within.
    - MBS also integrates relapse prevention therapy. We explore types of situations that commonly lead to relapse, and we develop strategies to avoid or cope with them.
- MBS involves skill development:
    - Using mindfulness to experience "what is" but not react
    - Reflecting on values and responding but not reacting
    - Urge surfing, a way of experiencing cravings and urges to use, while not reacting
    - Using sobriety skills in social situations, such as "drug refusal" skills.
- To the extent that individuals are aware of their situations and to the extent that their goals and actions are grounded in personal values, they are less likely to engage in self-defeating behaviors, that is, behaviors that diminish their sense of well-being, self-worth, and personal meaning. In MBS, sobriety is approached in the service of value-based living.

Appendix C

Value-Based Living Presentation

This appendix provides guidance for group leaders in describing the value-based living model and accompanying flowcharts. It is presented here and is used in IOP sessions 3, 4, and 9, as well as residential session 12.

The facilitator begins this process by walking the group members through the "Survival, Avoidance, and Value-Based Living Flowcharts (Facilitator's Guide)" (at the end of this appendix). The facilitator starts off by drawing "Figure 1: Early Humans—Survival" flowchart on a dry erase board (leaving room for the "Figure 2: Current Humans—Avoidance" flowchart to be drawn below it). In explaining the chart, the facilitator initially engages the group members in a discussion about the survival needs of early humans (life-threatening situations or events). This can be done by reading the following paragraph:

> *Much of early human life revolved around survival (as opposed to the value-based living of current humans). It is believed that much of their time was taken up by addressing concerns such as finding food, shelter, water, and protection. The fight, flight, or freeze instinct is believed to have developed out of our need to take quick action in service of survival.* (Pause to ask the group the following question, as a way to engage them in the presentation and to add a bit of humor.) *If we encountered a lion and stopped to think about how to respond, what do you think would happen? We would quickly become food for the lion! However, if on encountering the lion, we felt frightened (anxiety or discomfort) and instinctually (without thinking) ran away from it, killed it, or played dead (fight, flight, or freeze), we were able to survive (situation resolved plus relief).*

The facilitator pauses at this point, assesses for comprehension, and answers any questions the group might have. Next, the facilitator introduces the "Figure 2: Current Humans—Avoidance" flowchart, which is in the "Survival, Avoidance, Value-Based Living Flowcharts (Facilitator's Guide)" at the end of this appendix. The "Figure 1: Early Humans—Survival" flowchart should be left on the board in order to compare and contrast the models. Once figure 2 has been drawn on the board, the facilitator reads the following paragraphs to the group:

> *As humans developed, our lives became more complex and involved. Our fulfillment in life is now based less on survival (we now have grocery stores, fast-food restaurants, varying degrees of shelter and transportation, and so on) and more on values and goals (what we want our lives to be about). However, values and goals are often linked with emotions in current humans, like anxiety and discomfort, that were at one time linked with life-threatening situations in early humans. Examples of such emotions include vulnerability, uncertainty, fear, anxiety, and so on.*
>
> *With that said, our "survival" instincts (experiential avoidance) have now seemingly expanded to valued non–life-threatening situations, which can eventually result in issues that are life limiting and threatening. In our efforts (experiential avoidance) to avoid experiencing the aforementioned emotions, we tend to seek out ways of achieving instant relief or comfort, which can be seen as an extension of early human-survival behaviors, such as running back to the cave or killing the threat. Unfortunately, the more we avoid valued things or experiences that make us feel uncomfortable, the further away we get from our values and goals, which can often result in our feeling worse than when we started (situation unresolved plus discomfort). This can then result in someone's becoming trapped in the "avoidance" cycle.*
>
> *An example of this would be someone struggling with an addiction to heroin. He may overdose several times or lose all sources of support and still continue to use. Heroin provides instant and short-term relief from discomfort, whereas living a valued life increases a person's anxiety and discomfort initially but can eventually lead to fulfillment. Although it is understandable why someone would consciously or instinctually seek short-term relief, it can have devastating consequences in the long term. Another example would be someone who values socializing and relationships, but is anxious and fearful in social situations. If that person avoids social situations, he may experience an initial or short-term sense of relief. However, if he avoids them long term, he may feel even more anxious and depressed due*

to not having any friends or valued relationships.

The facilitator pauses at this point, assesses for comprehension, and answers any questions the group might have. The facilitator can then summarize and link the models by reading the following paragraphs to the group:

The instinct to solve problems or take quick action helped us to develop "bigger and better" strategies and tools for surviving and thriving as early humans. This approach evolved to include the desire to control as much of our environment as possible (building shelters, domesticating animals, controlling internal climate, developing means for protection, and so on). Again, as part of this process, we learned how to act quickly and often without thinking (impulse and instant relief or "gratification") when faced with life-threatening situations. However, as humans developed the ability to think and solve problems at a more complex level, the instinct to control has seemingly expanded to include things that happen inside of us as well (emotions, thoughts, memories, sensations, and so on), which can amplify our suffering in the long term.

Yet another example of this would be the understandable but ultimately futile human pursuit of feeling comfortable and happy most, if not all, of the time. In our instinctual search for bigger and better ways to control our internal experiences, we have discovered strategies that are highly effective in the short term (experiential avoidance, alcohol and drugs, television, sex, food, and so on). Unfortunately, if overused or relied on as a way to cope with life, the aforementioned strategies carry the potential to severely limit our lives in the long term (distance from values, addiction, isolation, missed opportunities, decreased health and wellness) due to their temporary nature.

Life now requires us to feel and move toward (not away from, as in the case of the lion) certain difficult experiences. Examples include applying for jobs (while risking rejection), asking someone on a date (while risking embarrassment and rejection), expressing one's true feelings (while risking being vulnerable), and experiencing an alcohol and drug craving (without acting on it).

Pose the following questions to the group at this point:

What happens when we fuse with our instincts and treat our internal experiences (thoughts, feelings, sensations, and so on) in the same way as we address external threats or discomforts, that is, to eliminate or fix them? For instance, what if we try to eliminate our fear of rejection by avoiding close relationships? Moving away from others emotionally may provide some immediate relief from anxiety, but what are the longer-term consequences? Are we more or less fulfilled with our lives?

Prior to moving on, the facilitator assesses for comprehension and answers any questions the group might have. The facilitator can then draw and introduce the "Figure 3: Current Humans—Value-Based Living" flowchart:

In order to remain in contact with your values, you have to be willing to feel a range of emotions. Although a person may still feel compelled to seek instant relief from uncomfortable feelings (experiential avoidance leads to relief), if he is able to remain mindful and take valued action (valued living action), he is then able to connect with his values and experience a sense of fulfillment. The "valued living" chart can be viewed as an overall approach to life that is ongoing and fluid. Although no one is perfect and some may have more resources than others, one can approach life mindfully and take effective actions in service of his values and goals (as opposed to remaining in one's comfort zone or feeling good all the time). One can be fulfilled in life while experiencing any number of emotions.

The facilitator pauses at this point, assesses for comprehension, and answers any questions the group might have. The facilitator then walks through the flowcharts with the examples included above or elicits examples from the group members' lives.

Survival, Avoidance, and Value-Based Living Flowcharts (Facilitator Guide)

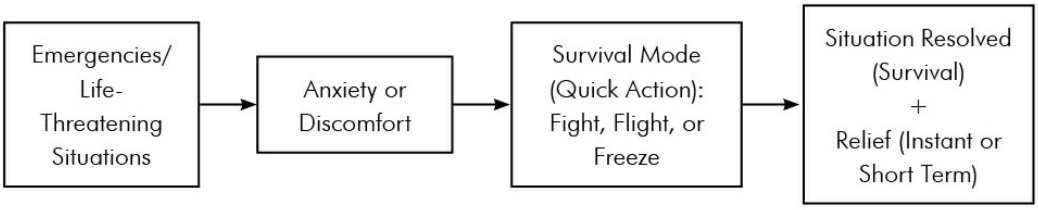


Figure 1: Early Humans—Survival

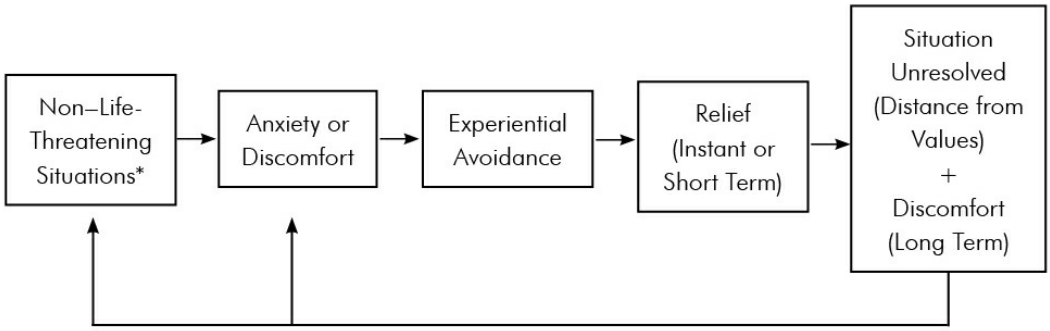


Figure 2: Current Humans—Avoidance

* Over time, the avoidance cycle increasingly limits life satisfaction and may endanger one's health.

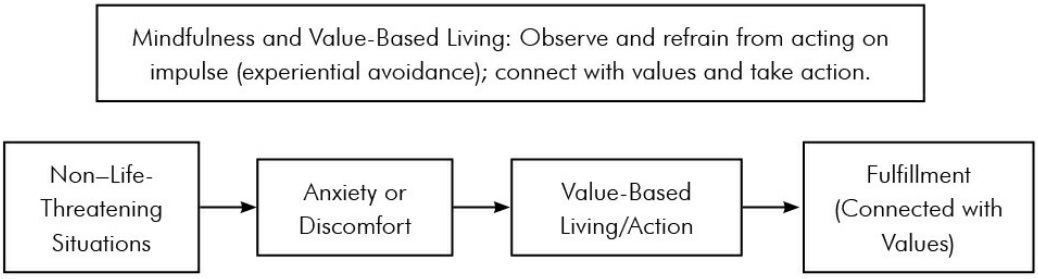


Figure 3: Current Humans—Value-Based Living

Appendix D

High-Risk Events and Scenarios (Experience in Situation)

EXPERIENCES:

- Emotions*:* Undesired or desired emotions, such as joy, sorrow, hope, despair, courage, anger, frustration, desire, aversion, guilt, shame, love, hate, sadness, loneliness, fear, happiness
- Thoughts, including self-critical or judgmental, those that glamorize substance use, worry or rumination, confidence related, other beliefs
- Sensations:
  - Physical discomfort accompanying withdrawal from drugs or alcohol, including cravings and urges
  - Pain: Chronic or acute
  - Physical sensations one might associate with an oncoming panic attack
  - Other sensations
- Memories:
  - Trauma related
  - Pleasant memories, such as those associated with prior use
- Visualizations, including imagination and forms of fantasizing
- Behavioral tendencies:
  - Impulses and compulsions
  - Other behavioral tendencies
- Experience of intent (that is, intending to do something):
  - Intending to get rid of or decrease unwanted feelings
  - Intending to test one's ability to drink or use drugs in a controlled manner
  - Intending to self-reward through substance use (*I deserve this*)
  - Other intent

SITUATIONS:

- Social:
  - Conflicts
  - Peer pressure
  - Interactions, including "medicating" social anxiety and enhancing intimacy

    - Celebrations and entertainment, such as weddings, sporting events, graduations, music concerts
    - Isolation
    - Other social settings
- *Places*, such as locations where alcohol or other drugs are prevalent or can be obtained
- *Time related*, such as holiday, anniversary, time of year, stage of life, and other time-related factors
- *Other situations*

Appendix E

Glossary of Terms

A*cceptance:* To acknowledge the realities of a situation while not fighting "what is." Acceptance is a nonjudgmental view that allows people to move forward on a valued path toward their self-selected goals. Acceptance is not approval; it is acknowledging one's experience.

Acceptance and commitment therapy: A third-wave CBT therapy. ACT (pronounced "act") utilizes mindfulness, which is acceptance along with commitment and behavior changes.

ASAM PPC-2R: American Society of Addiction Medicine Patient Placement Criteria for the Treatment of Substance-Related Disorders, 2nd edition, revised consists of criteria for assessing level of care need and appropriateness for individuals with substance use disorders. Levels of care include early intervention; outpatient treatment; intensive outpatient treatment; partial hospitalization; low, medium, or high intensity residential treatment; medically monitored intensive inpatient treatment; or medically managed intensive inpatient treatment. ASAM assessments are made across six dimensions: acute intoxication and/or withdrawal; biomedical conditions and complications; emotional, behavioral, or cognitive conditions and complications; readiness to change; relapse, continued use, or continued problem potential; and recovery environment.

Attachment: A term used in Zen Buddhism that refers to psychological adherence to objects of experience, such as desires, thoughts, and emotions; comparable to acceptance and commitment therapy term "fusion," a psychological process in which a person becomes fused with internal experiences (a thought, feeling, sensation, and so on).

Autopilot: Unmindful, habitual, routine behavior.

Awareness: The act of being fully present in the current moment, observing one's senses of sight, sound, taste, touch, hearing, and experience (internal and external).

Closed-group format: Group therapy in which all clients begin and complete the group at the same time. Closed-group treatments are generally sequential in that session content builds from knowledge and skills developed in prior sessions.

Cognitive distortion: A term frequently used in second-generation cognitive behavioral therapy (CBT) that essentially implies that a particular belief is an inaccurate representation of reality. This term and the concept it represents are not used in ACT or MBS.

Cognitive restructuring: A term commonly used in second-generation cognitive behavioral therapy (CBT) that refers to interventions that attempt to eliminate or change cognitions or thoughts. Such attempts may include pointing out irrationalities, offering alternative cognitions, or "thought stopping" and "thought replacement" techniques. This term and the concept it represents are not used in ACT or MBS, both of which hold that attempting to stop or replace thoughts may have iatrogenic effects—that is, it may actually draw further focus to the thoughts one is trying to eliminate.

Commitment: In MBS, taking action in service of one's values and goals, regardless of internal experiences (one's thoughts, feelings, cravings, sensations, and so on).

Continuum of care: Refers to the integrated differing levels of care across substance abuse treatment. This includes residential, partial hospitalization, intensive outpatient, and aftercare treatment levels.

Coping imagery: A therapeutic technique in which the person visualizes an image or source of strength and stability. Examples include a mountain, a phoenix rising, an inspirational figure, and one's children. This image can be utilized during moments of weakness or struggle.

Craving: In substance-use disorder (addiction) treatment, a strong desire to use a substance.

Defusing from the addiction: The act of separating from one's addiction. The client identifies by stating, "I have an

addiction" versus "I am an addict."

Defusion: Taking an observational, detached perspective in relation to a thought, belief, or emotion.

Denial: A defense mechanism used to refute what is real or uncomfortable.

Discrepancy: In motivational interviewing, the inner tension that one experiences when confronted with competing motivations. The competing motivations include, on the one hand, those that relate to personal values and value-oriented goals and, on the other, the desire for immediate relief or gratification, such as through substance use.

Domain: Context or areas of a client's life in which self-selected values are applied. Some examples are family, work, religion, and so on.

Drug refusal skills: An individual's abilities to cope with opportunities to use substances without using; typically drug refusal skills involve declining to accept and use drugs when offered by another person. Drug refusal skills can be role-played within treatment to practice and develop new skills. Some examples are to politely say no, firmly say no, walk away, carry a nonalcoholic beverage, or educate friends and family that the client is in recovery and cannot have any alcohol or substances.

Experiential avoidance: Used in ACT, a term that refers to the process of avoiding one's internal experiences that are uncomfortable. This avoidance gives short-term relief at the cost of meaningful living.

Fusion: The act of becoming one with or attached to a thought, belief, or emotion.

Intensive outpatient treatment (IOP): A level of care in addiction treatment in which the person attends treatment services a minimum of nine hours per week and no more than nineteen hours per week.

Internal experiences: A term used to describe private mental processes, including thoughts, feelings, and sensations.

Justification: A defense mechanism used by a client to allow the client to take an action that is otherwise unacceptable or inconsistent with self-defined values.

Lapse: Also known as "slip." A limited use of substance without necessarily leading to full-blown relapse or return to a previous pattern of addictive behavior.

Lapse trap: A situation or decisional path leading toward a situation that poses a high risk for the individual to use substances.

Lifestyle balance: A relapse prevention therapy (RPT) term referring to potentially healthy aspects of life—such as diet, exercise, recreation, spirituality—that enhance life quality and lessen risks for substance relapse. In RPT, lifestyle balance is also called "global issues."

Mindfulness (or mindfulness meditation): Being present in the immediate ("here and now") situation, observing nonjudgmentally and without psychological attachment, avoidance, or reaction.

Motivational interviewing: "Motivational interviewing is a collaborative, goal-oriented style of communication with particular attention to the language of change. It is designed to strengthen personal motivation for commitment to a specific goal by eliciting and exploring the person's own reasons for change within an atmosphere of acceptance and compassion" (Miller & Rollnick, 2013, p. 29).

Open-group format: A group-therapy format that allows clients to join an ongoing group at any particular session. This is contrasted to a closed-group approach, in which all clients in a therapy group begin and complete at the same time.

Openness: In MBS, a nondefensive, accepting, and flexible approach to one's experience.

Person-centered guiding: In motivational interviewing, use of Rogerian person-centered skills (such as reflections, open questions, and positive regard) while guiding a collaborative counseling process by focusing on potentially high-yield therapeutic content.

Positive expectations: In addiction, the expectation that using a substance will have favorable, desired effects.

Prelapse behavior: Behavior that can lead down a path toward a high-risk situation and potential lapse or relapse.

Rationalization: A defense mechanism used by a client to explain why a behavior (that may be otherwise unacceptable) is effective.

Recovery environment: The individual's surroundings and life circumstances that might affect sobriety potential—including residence, work, and other places—while the client is working on stabilizing a clean and sober lifestyle.

Recovery skills: A broad array of behavioral interventions designed to assist a client in attaining and maintaining a sober lifestyle.

Recovery skills self-rating: A client self-assessment that identifies a client's current skills; it can be used to identify strengths and weak areas in which to work and add new skills.

Reframing: Interventions that attempt to provide or initiate development of alternative perspectives or "frames" in reference of one's beliefs—that is, a productive or healthy perspective on what one previously considered to be a "negative" or "destructive" experience or situation.

Refusal skills: See "Drug refusal skills."

Relapse: A return to the previous pattern of addictive behavior.

Relapse prevention plan: A strategy a person uses to prevent returning to a previous pattern of addictive behavior. This may include a written outline of plans and skills the client will use to support a sober lifestyle.

Relapse prevention therapy (RPT): A specific and comprehensive cognitive behavioral therapy approach to relapse prevention developed by the late Dr. Alan Marlatt. RPT helps the person to identify high risk for relapse situations; assists in developing strategies to avoid, escape, and cope with high-risk situations; and promotes lifestyle balance to provide healthy alternatives that are inconsistent with substance use.

Re-mindfulness: Gently returning oneself to a mindful state after becoming distracted or going on autopilot.

Residential treatment: A particular level of care in addiction treatment in a twenty-four-hour residential setting—involves a higher level of care than intensive outpatient treatment, and a lower level of care than medically managed intensive inpatient treatment.

Role-playing: Individuals acting out life scenarios in service of developing skills or alternative perspectives.

Second-generation cognitive behavioral therapy (CBT): Please refer to appendix F.

Self-agency: One's sense of autonomy; a person's sense of being the agent of his own decisions and actions.

Situated freedom: A term used in contemporary existential therapy. Simone de Beauvoir coined the term "situated freedom," referring to the existential given that one always has freedom and that one's freedom always exists in the context of his life situation. One task at hand in existential therapy is to ascertain one's context or situation in terms of both the givens (the situation) and the options (one's freedom). In addiction treatment, a similar concept is sometimes expressed in the serenity prayer as "things I cannot change" and "things I can."

Situation rating scale: An MBS measure of situations in regard to two dimensions: personal value and relapse risk. This measure informs strategies to balance value-based living with relapse risk.

Slip: Also known as "lapse," use of substance without necessarily leading to full-blown relapse, that is, returning to a previous pattern of addictive behavior.

Sobriety: Awareness and acceptance of one's present experience and commitment to value-consistent living.

Sobriety challenges: Risks to achieving and maintaining sobriety.

Sobriety planning: Strategic considerations toward achieving sobriety.

Spirit: In motivational interviewing (MI), spirit—partnership, acceptance, compassion, and evocation—underlies and informs all aspects of MI practice and is prioritized over technique.

Spirituality: In mindfulness-based sobriety, spirituality is a sense of experiencing connection with something greater than oneself. Some individuals may regard their spirituality in religious contexts.

Stages of change: Introduced by Prochaska and DiClemente (1984) in their Transtheoretical Model of Change, stages of change (or readiness to change) levels include:

- *Precontemplation:* The person is not considering making a change.
- *Contemplation:* The person is considering making a change but has not decided to do so.
- *Preparation:* The person has decided to make a change and is preparing an approach.
- *Action:* The person is actively addressing his problem.
- *Maintenance:* The person has achieved and is now in the process of maintaining the accomplishment.

Therapeutic frame: The overarching structural, conceptual model within which therapeutic processes take place.

Therapeutic relationship: A therapist–client relationship characterized by qualities that promote therapeutic change. In the humanistic tradition, a therapeutic relationship is characterized by empathy, genuineness, and unconditional positive regard. In contextual behavioral science, the therapeutic relationship can be considered as a relationship that promotes the client's transition toward value-based living.

Third-generation cognitive behavioral therapy (CBT): Please refer to appendix F.

Urge: Contrasted with "craving," which is a strong desire for something, an urge is a compulsion or motivation to do something, such as to act on a craving.

Urge surfing: Coined by Alan Marlatt, this term refers to the use of mindfulness practice in the presence of addiction-related behavioral urges, such as urges to use substances. Marlatt noticed that urges rise, crest, and then subside in "wavelike" patterns. Being present with and observant of urges, while not reacting, is like "riding waves." One learns that urges are temporary and that they will subside even without use of substances.

Value-based avoidance: Intentionally avoiding a situation that poses a high risk for substance relapse. For situations that have personal value but also pose a high risk for relapse, avoidance may be temporary as the individual develops increased capacity to be present and cope.

Value-based living: Living in fidelity with one's personal values. A person can ask the questions, *What's important in my life?* and *How do I want to be as a person?*

Valued action: Behaving in a way that is in line with one's values.

Appendix F

Generations of Cognitive Behavioral Therapy

Behavior therapy is in its third generation, yet all three evolutions are actively practiced. In the first generation, referred to simply as "behavior therapy," mental processes were considered a "black box" that cannot be observed and therefore need not be taken into account in research, assessment, or therapy. Behavior therapy consists of two general types: classical and operant. In classical conditioning (Pavlov, 1927), behaviors are assessed in regard to preceding stimuli. Unconditioned stimuli yield biological responses from the animal—for example, a dog salivates when food is presented. Conditioned stimuli are events that, on their own, yield no particular response, yet when paired with an unconditioned stimulus—typically over a period of time—learning occurs. The animal begins to respond to the conditioned stimulus in the same way that it responds to the unconditioned stimulus. Thus, after repeated pairings of a bell sound with presentation of food, Pavlov's dogs learned to salivate when the bell was rung, even in the absence of food. In operant therapy, according to B. F. Skinner (1971), consequences of behaviors are considered in regard to their reinforcement or punishment value. Reinforcers increase targeted behaviors, while punishers decrease them. Negative reinforcement increased behaviors by removing aversive stimuli.

Behavior therapy principles are currently practiced in a number of ways, including:

- Behavioral assessments that analyze the effects of antecedents and consequences on behavior.
- Behavioral management programs that incentivize behaviors that are targeted for increase (such as prosocial behavior) and award points (that can be exchanged for commodities) when these behaviors are observed.
- Incentivizing or rewarding attendance at group-therapy sessions through food and other commodities.

The general principle in current behavioral therapy is to ignore behaviors that one wants to extinguish (that is, behaviors one wants to decrease or stop) unless they are a threat to safety (Kazdin, 2008). The risk of paying attention to unwanted behaviors is that the attention may act as a reinforcer, leading to an increase. As the saying goes, "Negative attention is better than no attention." At the same time that unwanted behaviors are ignored, behaviors targeted for increase are reinforced (rewarded). This practice can occur with children at home, when a parent ignores tantrums while giving attention and praise when the child is playing cooperatively or doing homework. This practice may also occur in treatment milieus, where behavioral management programs focus on "catching" clients doing something "good" (such as prosocial) and rewarding them with attention, praise, or points that can be exchanged for desired commodities.

Second-generation cognitive behavioral therapy (CBT) differs from its predecessor, behavior therapy, by "opening the black box"—that is, it introduced cognitions into behavior therapy. Initially spearheaded by the work of Albert Ellis (Ellis & Harper, 1961), pleasant and unpleasant emotions, as well as adaptive and maladaptive behaviors, are considered to be the product of thoughts (or cognitions). In a nutshell, rational thoughts lead to neutral or pleasant feelings plus adaptive behaviors. Irrational thoughts lead to unpleasant feelings or maladaptive behaviors. In treatment programs, irrational thoughts are sometimes coined "stinking thinking." The treatment objective is to eliminate or decrease irrational thoughts and to replace them with rational thoughts. Ellis's approach is called "rational emotive therapy" (RET) (Ellis & Harper, 1961) or "rational emotive behavioral therapy" (REBT) (Ellis, 2001). Other related approaches in second-generation CBT are also characterized by the attempt to intentionally and directly change one's maladaptive cognitions.

Third-generation CBT, including acceptance and commitment therapy (ACT), is the most current evolution. Unlike second-generation CBT, the third-generation approach does not recommend eliminating or replacing "problematic" thoughts. In fact, third-generation CBT warns that attempting to change thoughts may have the opposite effect: it may *increase* one's focus on the unwanted thoughts. In the third generation, instead of attempting to "fix" what is believed to be "broken," individuals are encouraged to be aware of and to accept that which is present, including cognitions. The person learns the relativity of thoughts: to differentiate thoughts from reality. One learns to defuse from—that is, become less attached to—thoughts and feelings. One simply "observes" cognitions in

a nonjudgmental, nonattached, and nonreactive way while accepting one's experience. In third-generation CBT, motivation for action is grounded in a commitment to personal values.

Appendix G

Recommended Readings

Appendix Outline

Literature is related to research on:

- Acceptance and commitment therapy
- Motivational interviewing
- Relapse prevention therapy
- Empathy and therapeutic alliance
- Mindfulness

Acceptance and Commitment Therapy

Hayes, S. C. (2005). *Get out of your mind and into your life: The new acceptance and commitment therapy.* With S. Smith. Oakland, CA: New Harbinger Publications.

Hayes, S. C., Strosahl, K. D., & Wilson, K. G. (2012). *Acceptance and commitment therapy: The process and practice of mindful change* (2nd ed.). New York: Guilford Press.

Luoma, J. B., Hayes, S. C., & Walser, R. D. (2007). *Learning ACT: An acceptance and commitment therapy skills-training manual for therapists*. Oakland, CA: New Harbinger Publications.

Wilson, K. G., & DuFrene, T. (2012). *The wisdom to know the difference*. Oakland, CA: New Harbinger Publications.

Motivational Interviewing

Arkowitz, H., Westra, H. A., Miller, W. R., & Rollnick, S. (Eds.). (2008). *Motivational interviewing in the treatment of psychological problems*. New York: Guilford Press.

Miller, W. R., & Rollnick, S. (2013). *Motivational interviewing: Helping people change* (3rd ed.). New York: Guilford Press.

Rosengren, D. B. (2009). *Building motivational interviewing skills: A practitioner workbook*. New York: Guilford Press.

Wagner, C. C., & Ingersoll, K. S. (2013). *Motivational interviewing in groups*. New York: Guilford Press.

Relapse Prevention Therapy

Bowen, S., Chawla, N., & Marlatt, G. A. (2011). *Mindfulness-based relapse prevention for addictive behaviors: A clinician's guide*. New York: Guilford Press.

Larimer, M. E., Palmer, R. S., & Marlatt, G. A. (1999). Relapse prevention: An overview of Marlatt's cognitive-behavioral model. *Alcohol Research and Health*, *23*, 151–160.

Marlatt, G. A., & Donavan, D. M. (Eds.). (2005). *Relapse prevention: Maintenance strategies in the treatment of addictive behaviors* (2nd ed.). New York: Guilford Press.

Marlatt, G. A., Parks, G. A., & Witkiewitz, K. (2002). Clinical guidelines for implementing relapse prevention therapy: A guideline developed for the behavioral health recovery management project. Seattle: University of Washington, Addictive Behaviors Research Center.

Empathy and Therapeutic Alliance

Norcross, J. C. (Ed.). (2011). *Psychotherapy relationships that work* (2nd ed.). New York: Oxford University Press.

Norcross, J. C., Hogan, T. P., & Koocher, G. P. (2008). *Clinician's guide to evidence-based practices: Mental health and the addictions.* New York: Oxford University Press.

Rogers, C. R. (1951). *Client-centered therapy*. With chapters contributed by E. Dorfman, T. Gordon, & N. Hobbs. Boston: Houghton Mifflin.

Rogers, C. R. (1980). *A way of being*. Boston: Houghton Mifflin.

Mindfulness

Kabat-Zinn, J. (2005). *Wherever you go, there you are: Mindfulness meditation in everyday life.* New York: Hyperion.

Kabat-Zinn, J. (2012). *Mindfulness for beginners: Reclaiming the present moment—and your life*. Boulder, CO: Sounds True.

Nhat Hanh, T. (1987). *The miracle of mindfulness! A manual of meditation* (M. Ho, Trans.; V.-D. Mai, Illustrator). Boston: Beacon Press.

Suzuki, S. (1968). *Zen mind, beginner's mind: Informal talks on Zen meditation and practice.* Boston: Shambhala Publications.

Appendix H

Research Support

This appendix will list publications that provide or reference research support for contributing models and practices:

- Acceptance and commitment therapy and other contextual behavioral approaches
- Motivational interviewing
- Relapse prevention therapy
- Empathy and therapeutic alliance
- Mindfulness

Acceptance and Commitment Therapy and Other Contextual Behavioral Approaches

Below please find a list of articles on efficacy research related to acceptance and commitment therapy and other contextual behavioral and third-generation cognitive behavioral therapies. More can be found on the Association for Contextual Behavioral Science (ACBS) website: www.contextualscience.org.

Abramowitz, J. S., Tolin, D. F., & Street, G. P. (2001). Paradoxical effects of thought suppression: A meta-analysis of controlled studies. *Clinical Psychology Review, 21*, 683–703.

Bach, P., & Hayes, S. C. (2002). The use of acceptance and commitment therapy to prevent the rehospitalization of psychotic patients: A randomized controlled trial. *Journal of Consulting and Clinical Psychology, 70*, 1129–1139.

Blackledge, J. T., Ciarrochi, J., & Deane, F. P. (Eds.). (2009). *Acceptance and commitment therapy: Contemporary theory, research, and practice*. Bowen Hills, Queensland, Australia: Australian Academic Press.

Bohlmeijer, E. T., Fledderus, M., Rokx, T. A., & Pieterse, M. E. (2011). Efficacy of an early intervention based on acceptance and commitment therapy for adults with depressive symptomatology: Evaluation in a randomized controlled trial. *Behaviour Research and Therapy, 49*, 62–67.

Brinkborg, H., Michanek, J., Hesser, H., & Berglund, G. (2011). Acceptance and commitment therapy for the treatment of stress among social workers: A randomized controlled trial. *Behaviour Research and Therapy, 49*, 389–398.

Campbell-Sills, L., Barlow, D. H., Brown, T. A., & Hofmann, S. G. (2006). Effects of suppression and acceptance on emotional responses of individuals with anxiety and mood disorders. *Behaviour Research and Therapy, 44*, 1251–1263.

Eifert, G. H., & Heffner, M. (2003). The effects of acceptance versus control contexts on avoidance of panic-related symptoms. *Journal of Behavior Therapy and Experimental Psychiatry, 34*, 293–312.

Flaxman, P. E., & Bond, F. W. (2010). A randomized worksite comparison of acceptance and commitment therapy and stress inoculation training. *Behavior Research and Therapy, 48*, 816–820.

Forman, E. M., Herbert, J. D., Moitra, E., Yeomans, P. D., & Geller, P. A. (2007). A randomized controlled effectiveness trial of acceptance and commitment therapy and cognitive therapy for anxiety and depression.

Behavior Modification, 31, 772–799.

Forman, E. M., Hoffman, K. L., McGrath, K. B., Herbert, J. D., Brandsma, L. L., & Lowe, M. R. (2007). A comparison of acceptance- and control-based strategies for coping with food cravings: An analog study. *Behaviour Research and Therapy, 45*, 2372–2386.

Gregg, J. A., Callaghan, G. M., Hayes, S. C., & Glenn-Lawson, J. L. (2007). Improving diabetes self-management through acceptance, mindfulness, and values: A randomized controlled trial. *Journal of Consulting and Clinical Psychology, 75*, 336–343.

Hayes, S. C., Luoma, J. B., Bond, F. W., Masuda, A., & Lillis, J. (2006). Acceptance and commitment therapy: Model, processes, and outcomes. *Behaviour Research and Therapy, 44*, 1–25.

Hayes, S. C., Wilson, K. G., Gifford, E. V., Bissett, R., Piasecki, M., Batten, S. V., et al. (2004). A preliminary trial of twelve-step facilitation and acceptance and commitment therapy with polysubstance-abusing methadone-maintained opiate addicts. *Behavior Therapy, 35*, 667–688.

Hinton, M. J., & Gaynor, S. T. (2010). Cognitive defusion for psychological distress, dysphoria, and low self-esteem: A randomized technique evaluation trial of vocalizing strategies. *International Journal of Behavioral and Consultation Therapy, 6*, 164–185.

Johnston, M., Foster, M., Shennan, J. Starkey, N. J., & Johnson, A. (2010). The effectiveness of an acceptance and commitment therapy self-help intervention for chronic pain. *Clinical Journal of Pain, 26*, 393–402.

Kishita, N., Ohtsuki, T., & Muto, T. (2012). Experimental analysis of the nature [of] cognitive defusion: Effects of contextual control over transformation of stimulus function established by topographical features of equivalence class members. *Japanese Journal of Behavior Therapy, 38*, 105–116.

Kishita, N., & Shimada, H. (2011). Effects of acceptance-based coping on task performance and subjective stress. *Journal of Behavior Therapy and Experimental Psychiatry, 42*, 6–12.

Levitt, J. T., Brown, T. A., Orsillo, S. M., & Barlow, D. H. (2004). The effects of acceptance versus suppression of emotion on subjective and psychophysiological response to carbon dioxide challenge in patients with panic disorder. *Behavior Therapy, 35*, 747–766.

Luciano, C., Molina, F., Gutierrez-Martinez, O., Barnes-Holmes, D., Valdivia-Salas, S., Cabello, F., et al. (2010). The impact of acceptance-based versus avoidance-based protocols on discomfort. *Behavior Modification, 34*, 94–119.

McCracken, L. M., MacKichan, F., & Eccleston, C. (2007). Contextual cognitive-behavioral therapy for severely disabled chronic pain sufferers: Effectiveness and clinically significant change. *European Journal of Pain, 11*, 314–322.

Merwin, R. M., Rosenthal, M. Z., & Coffey, K. A. (2009). Experiential avoidance mediates the relationship between sexual victimization and psychological symptoms: Replicating findings with an ethnically diverse sample. *Cognitive Therapy and Research, 33*, 537–542.

Powers, M. B., Zum Vorde Sive Vörding, M. B., & Emmelkamp, P. M. (2009). Acceptance and commitment therapy: A meta-analytic review. *Psychotherapy and Psychosomatics, 78*, 73–80.

Roemer, L., Orsillo, S. M., & Salters-Pedneault, K. (2008). Efficacy of an acceptance-based behavior therapy for generalized anxiety disorder: Evaluation in a randomized controlled trial. *Journal of Consulting and Clinical Psychology, 76*, 1083–1089.

Twohig, M. P., Hayes, S. C., Plumb, J. C., Pruitt, L. D., Collins, A. B., Hazlett-Stevens, H., et al. (2010). A randomized clinical trial of acceptance and commitment therapy versus progressive relaxation training for obsessive compulsive disorder. *Journal of Consulting and Clinical Psychology, 78*, 705–716.

Varra, A. A., Hayes, S. C., Roget, N., & Fisher, G. (2008). A randomized control trial examining the effect of acceptance and commitment training on clinician willingness to use evidence-based pharmacotherapy. *Journal of Consulting and Clinical Psychology, 76*, 449–458.

Woods, D. W., Wetterneck, C. T., & Flessner, C. A. (2006). A controlled evaluation of acceptance and commitment therapy plus habit reversal for trichotillomania. *Behaviour Research and Therapy, 44*, 639–656.

Motivational Interviewing

On the *Motivational Interviewing* website (www.motivationalinterview.org) are more than seven hundred research-related articles on motivational interviewing. Below is a limited list of these articles:

Adamson, S. J., & Sellman, J. D. (2008). Five-year outcomes of alcohol-dependent persons treated with motivational enhancement. *Journal of Studies on Alcohol and Drugs, 69*, 589–593.

Babor, T. F. (2004). Brief treatments for cannabis dependence: Findings from a randomized multisite trial. *Journal of Consulting and Clinical Psychology, 72*, 455–466.

Bager, P., & Vilstrup, H. (2010). Post-discharge brief intervention increases the frequency of alcohol abstinence: A randomized trial. *Journal of Addictions Nursing, 21*, 37–41.

Berman, A. H., Forsberg, L., Durbeej, N., Kallmen, H., & Hermansson, U. (2010). Single-session motivational interviewing for drug detoxification inpatients: Effects on self-efficacy, stages of change and substance use. *Substance Use and Misuse, 45*, 384–402.

Brown, J. M., & Miller, W. R. (1993). Impact of motivational interviewing on participation and outcome in residential alcoholism treatment. *Psychology of Addictive Behaviors, 7*, 211–218.

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About Gateway Foundation

With more than forty years of treatment experience, Gateway Foundation is the largest provider of substance abuse treatment in Illinois. With eleven centers located throughout the state, Gateway is a recognized leader in the use of evidence-based practices and integrated treatment of substance abuse and co-occurring mental health problems. Every year, Gateway helps thousands of individuals successfully complete treatment and gives renewed hope to those who care about them. Gateway offers outpatient and residential programs for adults and adolescents. You can find out more at recovergateway.org, or by calling the 24-hour helpline at 877-505-HOPE (4673).

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